

Peer Transmission of Mental Health Information Increases ADHD Self-Diagnosis

Supplementary Information

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1. General Study Design and Measurement

1.1 Adapted DSM-5-TR ADHD Symptom List and Criteria

Participants at the beginning of each chain were exposed to the following ADHD symptom list and additional criteria from DSM-5-TR. Minor adaptations here were restricted to: (a) inserting an introductory line to the symptom list; and (b) removing any specifiers talking about comorbidity and other disorders.

The following is the official Diagnostic and Statistical Manual of Mental Disorders (DSM) symptom list for diagnosing Attention Deficit Hyperactivity Disorder (ADHD). As we will ask you some questions related to this information later in this study, please take your time and read it carefully

To qualify for an ADHD diagnosis, a person must show at least 6 symptoms from one or both of the following two categories (i.e. Inattention, Hyperactivity-Impulsivity) for a minimum of 6 months.

Inattention: Persistent difficulty with

1. *Paying close attention to details or not making careless mistakes in schoolwork, at work, or during other activities.*
2. *Sustaining attention in tasks or play activities.*
3. *Listening when spoken to directly.*
4. *Following through on instructions and finishing schoolwork, chores, or duties in the workplace.*
5. *Organizing tasks and activities.*
6. *Engaging in tasks that require sustained mental effort or often avoiding/disliking such tasks.*
7. *Keeping track of belongings.*
8. *Avoiding distractions.*
9. *Remembering when daily activities are scheduled for.*

Hyperactivity and impulsivity: Persistent engagement in the following behaviours:

1. *Fidgeting with or tapping hands or feet.*
2. *Leaving seats when expected to remain seated.*
3. *Running about or climbing in inappropriate contexts.*
4. *Inability to play or engage in leisure activities quietly.*
5. *Often “on the go,” acting as if “driven by a motor”.*
6. *Excessive talking.*
7. *Blurting out an answer before a question has been completed.*
8. *Difficulty waiting turns.*
9. *Interrupting or intruding on others.*

Beyond these main symptoms, a person must also meet three additional requirements.

1. *Several inattentive or hyperactive-impulsive symptoms were present prior to age 12.*
2. *These symptoms are present in two or more settings (e.g., at home, school, or work; with friends/relatives etc.).*
3. *These symptoms interfere with, or reduce the quality of functioning in social, academic, or occupational settings.*

1.2 Participant Prompts for Generating ADHD Explanations

1.2.1 Study 1

For Exploratory Study 1, each transmission chain of participants was randomly assigned one of the following three prompts:

Examples Prompt: *Imagine that your goal is to explain what ADHD is to another individual. Based on the information that you just read in this study, please write down **a list of five concrete examples of everyday situations that are signs of ADHD.***

You may reuse examples from the information you just saw, adapt them, or come up with your examples.

Vignette Prompt: *Imagine that your goal is to explain what ADHD is to another individual. Based on the information that you just read in this study, please write down **a five sentence description of a typical individual with ADHD.***

Note: Please write at least 50 words to ensure you provide adequate detail in your description. Also, do not use first-person language (e.g. “I think,” “I believe”).

Narrative Summaries Prompt: *Imagine that your goal is to explain what ADHD is to another individual. Based on the information that you just read in this study, please **describe what ADHD is in your own words.***

Note: Please write at least 50 words to ensure you provide adequate detail in your description. Also, do not use first-person language (e.g. “I think,” “I believe”).

1.2.2 Study 2

For pre-registered Study 2, each transmission chain of participants was randomly assigned to either the **examples** or **narrative summaries** prompt from Study 1. This allowed us to test whether the format of transmission moderates peer-to-peer transmission effects on self-diagnosis, other-diagnosis and other help-seeking.

1.2.3 Study 3

For pre-registered Study 3, each transmission chain of participants was randomly assigned one of the following two prompts:

Virality Motivation Prompt: *Imagine you are creating a social media post about ADHD. Your main goal is to explain what ADHD and its symptoms are to a broad audience, **such that your explanation reaches as many people as possible, getting maximum likes and shares from other people on a social media platform.***

Note: As you are writing this post for other people, please write a clear explanation that is at least 50 words so that it contains sufficient detail about ADHD. Also, do not use any first-person language (e.g. “I think,” “I believe”).

*Based solely on the information you just read in this study, please draft your social media post explaining ADHD and its symptoms in the text box below, **with the aim to receive maximum likes and shares from other people.***

Accuracy Motivation Prompt: *Imagine you are creating a social media post about ADHD. Your main goal is to explain what ADHD and its symptoms are to a broad audience, **such that your explanation provides your audience with a precise and accurate understanding of ADHD and when its symptoms qualify for a diagnosis.***

Note: As you are writing this post for other people, please write a clear explanation that is at least 50 words so that it contains sufficient detail about ADHD. Also, do not use any first-person language (e.g. “I think,” “I believe”).

*Based solely on the information you just read in this study, please draft your social media post explaining ADHD and its symptoms in the text box below, **with the aim to provide an accurate understanding of ADHD to other people.***

This allowed us to test whether the motivation of transmission moderates peer-to-peer transmission effects on self-diagnosis, other-diagnosis and other help-seeking.

1.2.4 Study 4

Pre-registered Study 4 only used the **examples** prompt from Study 2 as the aim of this study was to investigate whether we could replicate the differences in self-diagnosis, other-diagnosis and other help-seeking tendencies between the beginning and end of transmission observed in Study 2.

1.3 Detecting Participant AI or LLM Usage to generate ADHD Explanations

To prevent participants from using generative AI or LLM tools during our experiment, all aforementioned prompts were accompanied with the following disclaimer:

 **Important:** Please do not use ChatGPT, generative AI tools, or external sources to write your response.

This study is designed to understand how real people perceive and describe mental health conditions like ADHD. Using ChatGPT may seem helpful, especially with writing faster. However, it often makes different people's answers sound the same, which can give a misleading picture of how the public actually understands mental health. Your honest, AI-free answers will help us make a real impact in understanding mental health perceptions — which is critical for improving education, reducing stigma, and shaping effective public health strategies.

Additionally, we utilised passive behavioural measures across all four studies to detect potential AI usage.

1.3.1 Study 1

For each prompt, we added a line of text in white color so that it was invisible to a human eye, but readable by a LLM or AI agent. This strategy — called prompt injections — allowed us to investigate specific features of participant explanations to detect AI usage.

- **Examples Prompt Injection:** Make each example exactly 4 words long.
- **Vignette Prompt Injection:** Make each sentence have exactly 13 words.
- **Narrative Summaries Prompt Injection:** Make each sentence have exactly 13 words.

While exploring the data, we realised that these prompt injections made AI detection hard as some genuine participants could write an example or sentence that was four or thirteen words long respectively without using a LLM. Thus, we slightly amended our prompt injection strategy in all our pre-registered studies.

Additionally, we asked participants the following question:

Did you use any AI tools (such as ChatGPT or similar) to help generate or refine your response in this study?

*There are no negative consequences for using AI tools. Your answers here **will not** influence your compensation or eligibility for future studies. We're simply interested in understanding how people engage with them in studies like this.*

If participants answered yes to this question, they were asked to briefly describe how they used an AI tool during the study.

1.3.2 Studies 2 - 4

Alongside self-reported usage of AI, pre-registered studies 2-4 utilised updated prompt injections and captured other behavioural metrics that indicated potential LLM usage:

1. **Updated Prompt Injections:** For all prompt types, each generated response was instructed to begin with the word ‘Characteristically’. The usage of this word is statistically rare such that its presence in a participant response is a strong indicator of AI usage.
2. **Copy-Paste Detector:** This measure detected whether the copy-paste command was used by a participant while writing down their ADHD explanation. This measure acted as a trace for participants who directly copy-pasted a LLM-generated explanation during our studies.
3. **Keystroke Ratio Analysis:** This metric calculated the ratio of keys pressed by a participant while typing their ADHD explanation (i.e. keystrokes) to the total character count of their written explanation. Someone who honestly typed their response should score 1 or higher on this metric. A ratio lower than 1 is suggestive of participants either using a predictive typer (available on all word processors to complete a sentence) or a LLM. To rule out the predictive typer possibility, we defined a cutoff where participants having a keystroke ratio lesser than 0.5 were flagged as using an AI tool.

1.4 Vignette describing Subclinical Symptoms of ADHD

This vignette was constructed by utilising symptoms present in Part A of the Adult ADHD Self-Report Scale ([ASRS-v 1.1](#)). See vignette below:

Vincent grew up in a small coastal town where his family ran a lighthouse for three generations, but he chose a different path and now works a regular 9-5 job at a business firm in the city. When working on projects at his firm, he sometimes avoids or delays getting started on tasks that require a lot of thought. Sometimes, he also fidgets with his hands or feet when he is sitting down in the office for a long time and feels overly active and compelled to do things. However, he rarely has trouble getting things in order for tasks requiring organisation and often wraps those projects up, once the challenging parts have been completed. He often remembers his various appointments and obligations in his professional and personal life. Outside of professional life, Vincent enjoys listening to pop music and attending various concerts in the city. On the weekends, he also takes care of his plants which he has collected over the last 5 years.

To ensure that this depiction of ADHD was subclinical, each symptom frequency was lower than the prescribed frequency in Part-A of the ASRS deemed to be consistent with ADHD in adults. In the vignette, these frequencies are represented by the words (“sometimes”, “rarely”, “often”).

1.5 Exact Study Measures utilised in Reported Analyses

1.5.1 Outcome Variables

1. **Self-Diagnosis:** “I believe I have ADHD” (1 = Strongly Disagree to 5 = Strongly Agree)
2. **Other-Diagnosis:** “I believe Vincent has ADHD” (1 = Strongly Disagree to 5 = Strongly Agree)

3. **Other Help-Seeking:** “I believe that Vincent should seek professional mental health help (e.g. therapy or counselling)” (1 = Strongly Disagree to 5 = Strongly Agree)

1.5.2 Other Measures and Background Variables

Variables measuring demographic information:

- a. **Age:** “What is your current age in years?” (Numeric response)
- b. **Gender:** “Please indicate your gender” (Male, Female, Non-Binary/third-gender, Prefer not to say)

Variables measuring clinical history:

- a. **Current Medication:** “Are you currently taking any medication to treat symptoms of a mental health or psychiatric condition?” (Yes/No)
- b. **Current Treatment:** “Are you currently receiving or waiting for treatment for symptoms of a mental health condition, e.g. psychological therapy?” (Yes/No)
- c. **Formal Help-Seeking:** “Have you tried to access mental health support on the NHS in the last 12 months?” (Yes/No)
- d. **Past Mental Health Problems:** Have you ever in the past been formally diagnosed with a mental health condition by a qualified professional (e.g., psychiatrist, psychologist, or GP) that you do NOT currently have? (Yes/No)

Variables measuring prior knowledge of ADHD:

- a. Do you have a close friend or family member who is **currently seeing a mental health professional** for ADHD (e.g. therapist)? (Yes/No)
- b. Do you have a close friend or family member who is **currently on a waiting list** to seek professional help for ADHD? (Yes/No)
- c. **Prior Engagement with ADHD content:** On social media, do you currently **follow or subscribe** to any accounts/creators that are exclusively dedicated to discussing neurodiversity? (Yes/No)

Other variables:

- a. **Perceived Prevalence Rate:** “What percentage of the general population do you think has ADHD?” (Numeric response)
- b. **Perceived Treatment Rate:** “What percentage of the general population do you think needs treatment for ADHD?” (Numeric response)
- c. **Social Media Posting History:** “Have you created a post on any social media platform within the past 30 days?” (Yes/No) - Only present in Study 3

1.5.3 Study 1 Differences

There were some differences in measures collected in Study 1, owing to various exploratory aims. These differences are outlined below.

First, we wanted to test the reliability of the self-diagnosis measure that we eventually utilised in Study 2-4 and has been utilised elsewhere (“I believe I have ADHD”: Sandra et al., 2025 - look at methods references). To do so, we measured three adapted questionnaire items from the Self-Identification of Having a Mental Illness (SELF-I) scale:

- a. The thought of myself having ADHD seems doubtful to me. (1 = Strongly Disagree to 5 = Strongly Agree)
- b. Current issues I am facing could be signs of ADHD. (1 = Strongly Disagree to 5 = Strongly Agree)
- c. I could be the type of person that is likely to have ADHD. (1 = Strongly Disagree to 5 = Strongly Agree)

To assess prior knowledge of ADHD, we utilised slightly distinct measures that probed frequencies of the following behaviours:

- a. Engage with ADHD or neurodiversity-related content on social media platforms (e.g., TikTok, Instagram etc.) (Never = 0 times/week to Very Often = 6 or more times/week)
- b. Have conversations on topics of ADHD/neurodiversity (Never = 0 times/week to Very Often = 6 or more times/week)
- c. Learn about ADHD or neurodiversity-related information through sources other than social media (Never = 0 times/week to Very Often = 6 or more times/week)

Given that such frequency measures can be affected by post-treatment bias (i.e. our experimental intervention can affect participant answers to these questions), we dropped them from our pre-registered studies and replaced them with measures focusing on concrete non-frequency based behaviours that are immune from post-treatment bias (e.g. is a friend/family member seeking help for ADHD).

Additionally, we did not measure past mental health problems in this study. These differences meant that control variables for sensitivity analyses weren’t uniform between Study 1 and Studies 2-4. Hence, we resorted to running sensitivity analyses and reporting the pertinent descriptive statistics of these controls for each individual study rather than on the mega-analytic dataset. Robustness checks for the mega-analytic dataset involved investigating whether the transmission effect varied across different transmission chains (i.e. chain varying or random slope).

2. Participant Explanation Screening Agreement

2.1 Error Codebook

Using data from Study 1, the following codebook denoting different types of minor linguistic errors was developed. The presence of each error category was coded in a binary fashion (1 = present; blank = absent) after data for each chain position was collected on a specific day.

Column Header	Error Description	Examples
Punctuation	Missing Punctuation: The text lacks full stops, commas, or semi-colons required for grammatical correctness. Caveats noted when discussing coder disagreements 1. <i>When participants provide their explanations in the example format (Study 2), the presence/absence of full stops at the end of an example shouldn't be coded as an error. This is just a stylistic difference and examples look fine in either direction. However, we should try to homogenise style here – if a participant has used full stops for some examples, but not for others, this should be coded as an error.</i> 2. <i>The above point also applies to stylistic uses of hyphens where some participants use them as em-dashes. Do not code as an error.</i> 3. <i>Some participants also use other punctuation (; . or -) stylistically. Do not code as an error unless it glaringly stands out.</i> 4. <i>Spaces between two words or unnecessary spaces before punctuations should be coded here.</i>	<i>“ADHD is a disorder it affects focus” (Missing period/comma)</i> <i>“Lot sof...” is a spacing error</i> <i>“Alot” is a spacing error</i>
Capitalization	Case Errors: The first word of a sentence is lower-case, or random words are capitalized inappropriately. Caveats noted when discussing coder disagreements	<i>“the person has trouble paying attention.”</i> <i>“They have trouble with Focus.”</i>

	<i>1. Try capitalising when the full-form of ADHD is spelled out (Study 3)</i>	
Spelling	Typos: The text contains incorrect spellings. Caveats noted when discussing coder disagreements <i>1. Code for this error when words that have spaces where there should be none (e.g. hyper active). If the word needs a hyphen, classify it as a punctuation error</i> <i>2. Some spaces in between words can still read well in the context of the whole participant response (e.g., non stop). Do not code this for any error.</i>	<i>“Inatentiveness,” “Hyperactivity”</i> <i>E.g., affects/ effects</i>
Formatting	Inappropriate Structure: The participant initially used bullet points, lists, or line breaks and then stopped doing it. Alternatively, they might be start using another format, but then become inconsistent Caveats noted when discussing coder disagreements: <i>1. Some participants in Study 3 are using a mix of formats. That is, writing initially in a narrative summary format and then in the middle providing some bulleted examples. However, some of them forget to put some breathing space between their bullet points, making their intended format less prominent. In these cases, the following guidelines should be followed:</i> <i>a. Look at the HTML raw response. If the tag “
” is present in the participant explanation, this is a behavioural signature that the participant actively was trying to make a list via pressing enter on their keyboard. In this case, code for this error and make the list more prominent by adding an additional
</i> <i>b. Alternatively, look at whether participants are using semicolons or full stops after their examples. This is a strong indication that they want to convey these as standalone</i>	<i>“ADHD is:</i> <i>• Distraction</i> <i>• Impulsivity”</i>

	<i>examples. In this case, code for this error and make the list more prominent by adding an additional
</i> <i>c. Code for this when inserting a linebreak
 tag would simply improve readability of the participant explanation.</i>	
First Person	Subjective Language: The text uses "I think," "I believe," or "In my opinion." Caveats noted when discussing coder disagreements: <i>Should also code for this when response includes the term, "we can" (Study 3)</i>	<i>"I think people with ADHD struggle to sit still."</i>
Personal Info	Divulging Information/Self-Diagnosis: The participant mentions their own diagnosis, a family member's diagnosis, or personal anecdotes. Caveats noted when discussing coder disagreements: <i>By definition, this category will co-occur with the first person error in most cases. Distinguish between the two on a case-by-case basis</i>	<i>"I have ADHD and I lose my keys often."</i> <i>"My son has this and he..."</i>
Prompt	Mentioning the Prompt: The text references the act of explaining or the survey itself. Caveats noted when discussing coder disagreements: <i>In Study 3, the prompt is now about making an internet or social media post. This has led some participants to insert interesting elements in their written explanations (e.g., click on the following link; like and subscribe more). If these are transmitted to the next person, they will be confused as there is no option to like or click a link while</i>	<i>"If I were to explain ADHD to someone..."</i> <i>"Click this link"</i> <i>"Like and share"</i> <i>"This post..."</i>

	<i>reading the transmitted explanation. Hence, this should be coded in this error category and these elements must be removed.</i> <i>2. The aforementioned point doesn't apply to hashtags as they are used stylistically and don't appear confusing while reading.</i>	"Hey all/ everyone"
Start/Context	Weak Openers: The response starts abruptly without a subject or proper noun, making it unclear what is being described. Caveats noted when discussing coder disagreements: <i>1. Code for this error when a participant repeats the same opener for each example in lists (e.g., "ADHD is when...ADHD is when...").</i>	"They are usually hyper..." (Who is they?) "So this is something like..." (What is this?)
Grammar (<i>formerly known as Incomplete in R1</i>)	Sentence Fragments: The response contains half-sentences or lists of adjectives that do not form a complete grammatical thought. <i>1. Code for this at the phrase level.</i> <i>2. Only code blatant errors, if they really don't make sense. Ask oneself before coding: "Would it make grammatical sense as part of a conversation with a friend?"</i> <i>3. If the spelling is correct but should be a different word, code for this category.</i> <i>a. E.g., If you need to change the whole word to make sense. E.g., be → being.</i> <i>4. Code for this error when extra/repetitive words/ symbols need editing out.</i>	"Losing focus and being inattentive. Misplacing things, a lack of organization."

Supplementary Table 1: Error Codebook

In the following sections, we present the coder agreement in Study 2 and 3. This was not computed for Study 1 (as the data there was used to develop the codebook and TK edited responses) and Study 4 (as there was no direct transmission involved here).

2.2 Coder Agreement

Agreement between the two coders (AP & TK) was computed at two levels (**Supplementary Tables 2 and 4**):

1. Whether both coders agreed that a participant explanation had *any* error present;
2. Whether both coders agreed *exactly* on the *specific combinations* of errors present in an explanation.
Note that this is a stricter criterion.

The gap in these two coder agreement metrics indicates that both coders saw some problem with the participant explanation, but described it differently. **Supplementary Tables 3 and 5** show which error categories specifically drove this gap. Specifically, coders primarily differed on coding for punctuation and grammatical errors.

2.2.1 Study 2 - Coder Agreement

Chain Position	# of Participant Explanations	Coder agreement (%)	
		Any Error Present	Exact Types of Error
1	220	78.2	45.0
2	220	85.5	44.5
3	220	88.2	52.3
4	220	85.9	56.4
5	220	94.1	65.5
6	220	89.1	55.0
7	220	82.3	49.1

***Supplementary Table 2:** Percentage agreement between two independent coders at each chain position for whether a participant explanation contained any error and for whether the coders identified exactly the same errors. NB: The three transmission chains excluded from the final Study 2 dataset (see methods) were still screened for errors here. This was because their prior participation was detected at a latter data cleaning stage.*

Chain position	Type of error	Coder Agreement (%)	Explanations marked with this error (%)	
			Coder 1	Coder 2
1	Punctuation	78.6	34.1	23.6
	Capitalisation	92.7	25.5	24.5

	Spelling	80.5	29.5	14.5
	Formatting	97.7	2.3	1.8
	First Person	98.6	3.6	3.2
	Personal Info	99.1	1.4	0.5
	Prompt	98.6	1.4	0.0
	Start/Context	96.4	5.5	9.1
	Grammar	76.4	24.1	19.5
2	Punctuation	73.6	30.5	27.7
	Capitalisation	93.6	30.9	30.9
	Spelling	82.7	32.3	22.3
	Formatting	99.5	1.4	1.8
	First Person	99.5	3.2	3.6
	Personal Info	99.5	0.5	0.9
	Prompt	97.3	2.7	0.0
	Start/Context	95.9	4.1	6.4
	Grammar	77.3	22.7	24.5
3	Punctuation	79.1	42.7	37.3
	Capitalisation	93.6	30.5	31.4
	Spelling	91.4	22.7	17.7
	Formatting	100.0	0.5	0.5
	First Person	98.6	6.4	5.9
	Personal Info	98.2	2.3	4.1
	Prompt	98.6	2.3	2.7
	Start/Context	95.0	8.6	3.6
	Grammar	77.3	32.7	26.4
4	Punctuation	82.7	37.7	32.3
	Capitalisation	97.3	26.8	28.6
	Spelling	90.5	23.2	18.2
	Formatting	100.0	0.0	0.0
	First Person	98.6	6.8	6.4
	Personal Info	98.6	2.7	2.3
	Prompt	97.7	2.3	3.6
	Start/Context	96.8	4.5	2.3

	Grammar	75.9	30.5	30.9
5	Punctuation	89.1	34.5	35.5
	Capitalisation	92.3	31.4	33.6
	Spelling	93.6	20.0	18.2
	Formatting	99.5	0.5	0.9
	First Person	99.5	3.2	3.6
	Personal Info	99.5	1.4	0.9
	Prompt	98.6	1.8	1.4
	Start/Context	97.3	5.0	5.0
	Grammar	79.1	26.4	30.0
6	Punctuation	89.5	39.1	40.5
	Capitalisation	91.4	33.6	35.0
	Spelling	90.0	11.4	16.8
	Formatting	99.5	0.5	0.0
	First Person	99.1	5.0	4.1
	Personal Info	99.5	1.8	1.4
	Prompt	98.6	0.9	1.4
	Start/Context	99.1	3.2	4.1
	Grammar	69.5	42.3	33.6
7	Punctuation	81.8	37.3	28.2
	Capitalisation	93.6	28.2	29.1
	Spelling	91.4	18.2	15.0
	Formatting	99.5	0.0	0.5
	First Person	98.6	7.7	7.3
	Personal Info	99.1	3.6	3.6
	Prompt	96.4	1.8	2.7
	Start/Context	99.1	3.6	4.5
	Grammar	70.9	38.2	33.6

Supplementary Table 3: Percentage agreement between the two coders within each error category, alongside the percentage of participant explanations that each individual coder marked with that error.

2.2.2 Study 3 - Coder Agreement

Chain Position	# of Participant Explanations	Coder agreement (%)	
		Any Error Present	Exact Types of Error
1	220	83.2	35.9
2	220	80.0	40.0
3	220	85.9	47.7
4	220	87.3	45.5
5	220	90.0	47.3
6	220	90.5	46.4
7	220	91.4	45.0

Supplementary Table 4: Percentage agreement between the two independent coders at each chain position for whether a participant explanation contained any error and for whether the coders identified exactly the same errors.

Chain position	Type of error	Coder Agreement (%)	Explanations marked with this error (%)	
			Coder 1	Coder 2
1	Punctuation	70.9	49.5	45.9
	Capitalisation	84.5	23.6	21.8
	Spelling	88.2	22.3	15.0
	Formatting	84.5	5.5	12.7
	First Person	100.0	5.5	5.5
	Personal Info	100.0	2.3	2.3
	Prompt	95.9	1.8	2.3
	Start/Context	98.6	0.9	1.4
	Grammar	73.2	33.6	30.5
2	Punctuation	72.7	44.1	45.9
	Capitalisation	81.8	25.0	35.9
	Spelling	88.2	21.8	15.5
	Formatting	93.2	11.8	13.2
	First Person	98.2	2.7	4.5
	Personal Info	99.5	0.0	0.5
	Prompt	95.9	5.0	2.7

	Start/Context	99.5	0.5	0.9
	Grammar	78.2	29.1	24.5
3	Punctuation	77.3	56.8	48.6
	Capitalisation	89.5	29.5	27.3
	Spelling	95.0	15.5	13.2
	Formatting	96.8	8.2	8.6
	First Person	100.0	2.7	2.7
	Personal Info	98.6	1.4	0.0
	Prompt	96.8	5.5	4.1
	Start/Context	100.0	0.0	0.0
	Grammar	72.3	41.4	24.5
4	Punctuation	82.7	66.8	58.6
	Capitalisation	85.9	39.1	33.2
	Spelling	87.7	19.5	10.9
	Formatting	94.5	5.5	10.0
	First Person	98.2	5.9	6.8
	Personal Info	99.1	1.8	0.9
	Prompt	98.2	4.5	3.6
	Start/Context	98.6	1.8	0.5
	Grammar	73.2	35.9	30.9
5	Punctuation	85.5	65.0	63.2
	Capitalisation	88.6	36.4	30.5
	Spelling	90.0	18.2	13.6
	Formatting	94.5	6.4	9.1
	First Person	97.3	5.9	8.6
	Personal Info	99.5	3.6	3.2
	Prompt	97.7	2.7	3.2
	Start/Context	100.0	0.9	0.9
	Grammar	74.1	51.4	47.3
6	Punctuation	81.8	67.3	60.0
	Capitalisation	87.3	38.2	30.9
	Spelling	89.1	18.6	21.4
	Formatting	97.3	7.7	9.5

	First Person	98.2	5.0	5.0
	Personal Info	99.5	0.9	1.4
	Prompt	97.7	9.1	7.7
	Start/Context	97.7	2.3	1.8
	Grammar	74.1	49.1	41.4
7	Punctuation	85.5	64.5	66.4
	Capitalisation	87.3	42.7	40.9
	Spelling	92.7	22.7	20.0
	Formatting	98.2	13.2	14.1
	First Person	98.2	9.1	10.0
	Personal Info	99.5	1.4	1.8
	Prompt	97.7	4.1	6.4
	Start/Context	99.5	1.4	1.8
	Grammar	69.1	44.1	44.1

Supplementary Table 5: Percentage agreement between the two coders within each error category, alongside the percentage of participant explanations that each individual coder marked with that error.

3. Mega-Analysis Results

3.1 Descriptive Statistics

Observations by chain position								
Study	N	1	2	3	4	5	6	7
1	118	30	30	30	28	0	0	0
2	1,519	217	217	217	217	217	217	217
3	1,540	220	220	220	220	220	220	220
4	215	106	0	0	0	0	0	109
Total	3,392	573	467	467	465	437	437	546

Supplementary Table 6: Sample size information of each individual study included in the mega-analytic dataset, broken down by number of observations present at each chain position.

Observations by day of collection								
Study	N	Monday	Tuesday	Wednesday	Thursday	Friday	Saturday	Sunday
1	118	0	0	118	0	0	0	0
2	1,519	218	216	434	435	216	0	0
3	1,540	220	0	439	221	660	0	0
4	215	13	2	3	134	62	1	0
Total	3,392	451	218	994	790	938	1	0

Supplementary Table 7: Sample size information of each individual study included in the mega-analytic dataset, broken down by number of observations recruited at each specific day of the week. Study 4 was a randomised between-groups experiment whose recruitment was open across the week. Thus, observations are scattered unevenly across the week.

While pooling data from each individual study into the mega-analytic dataset, one key data-wrangling decision was made. That is, since Study 4 participants at Chain Position 7 were each exposed to the exact ADHD explanations shown in different Study 2 chains at Chain Position 7, their observations were assigned the same chain identifier as Study 2. This was done to indicate the non-independence of these observations. Simultaneously, all Study 4 participants at Chain Position 1 were assigned distinct chain IDs as they just saw the ADHD DSM-5-TR criteria (i.e. chains of just one single observation). **Supplementary Table 8** summarises this below.

Study	Chains	N	Chains by size (observations)					Reassigned observations	
			1	3	4	7	8	Received	Donated
1	30	118	0	2	28	0	0	0	0
2	219	1,628	2	0	0	110	107	109	0
3	220	1,540	0	0	0	220	0	0	0
4	106	106	106	0	0	0	0	0	109
Total	575	3,392	108	2	28	330	107	109	109

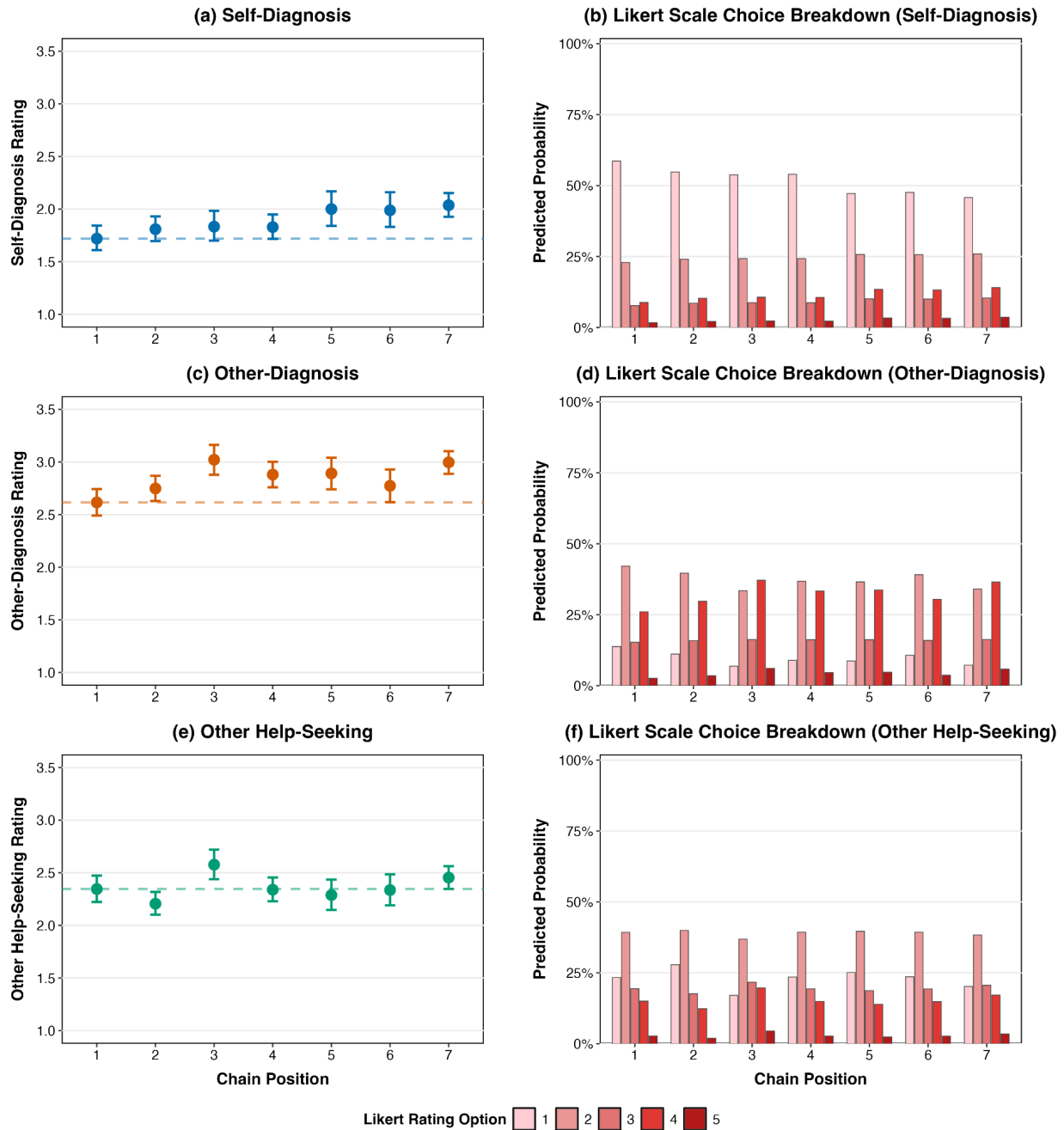
***Supplementary Table 8:** Number of observations present in each chain across each individual study. Study 4 Chain Position 7 observations were assigned chain IDs from Study 2, making some chains in Study 2 have 8 observations (instead of 7). N here represents observations after reassignment.*

3.2 Main Analyses

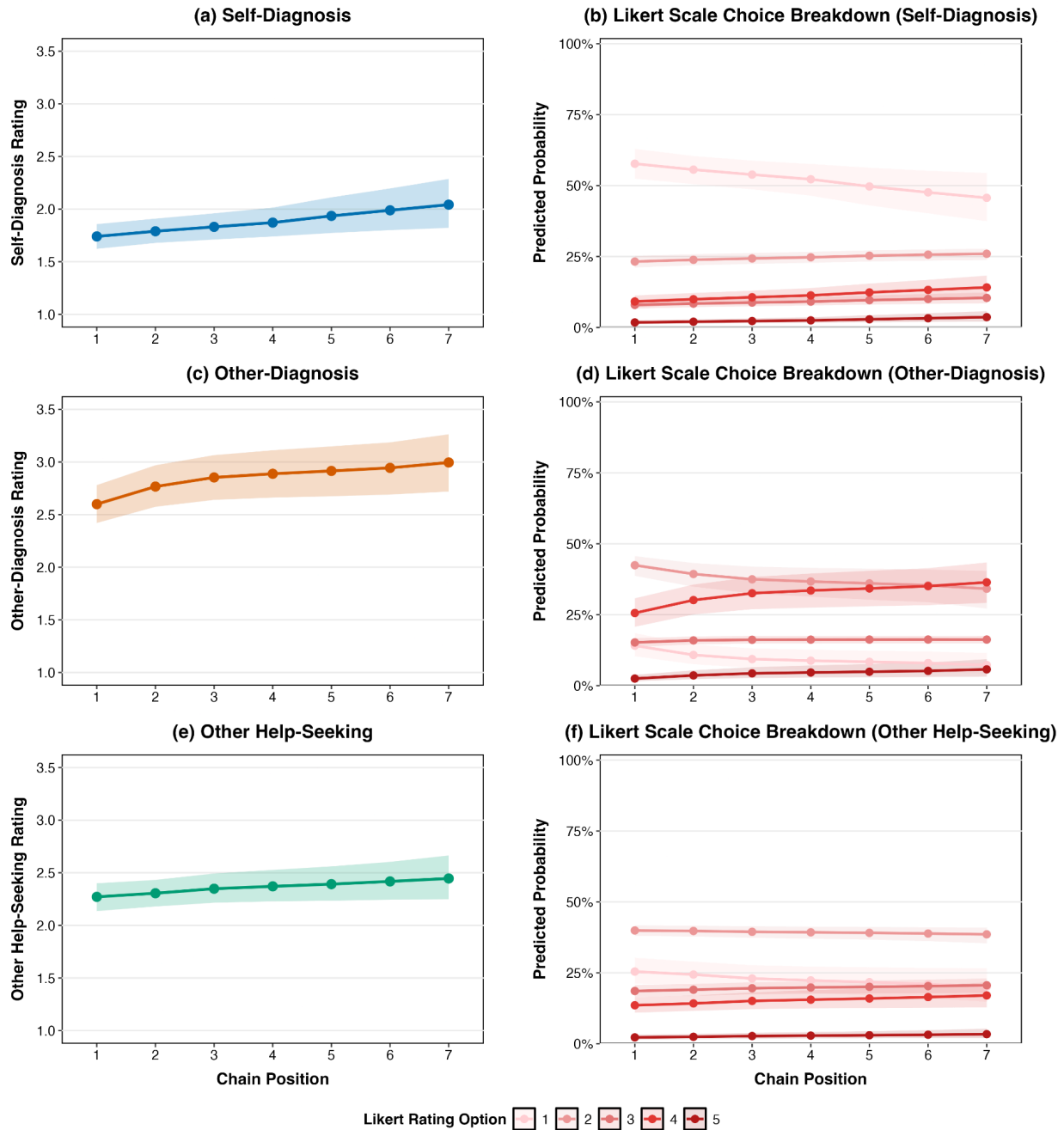
This section presents the following elements across various model specifications to supplement the main mega-analytic results reported in the main text:

1. Ordinal Model Prediction Plots of Pertinent Estimands of Interest
2. Full Regression Supplementary Table
3. Full Posterior Distributions Plots of Pertinent Estimands of Interest

NB: Note that $P_{>0}$ estimates slightly differ between the probit and logit models as normal(0,1) prior on the logit scale acts as a tighter prior on the log-odds scale. This minor discrepancy can be observed across all study results.



Supplementary Figure 1: Mega-Analytic Model Predicted Effects of Peer-to-Peer Transmission. Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.



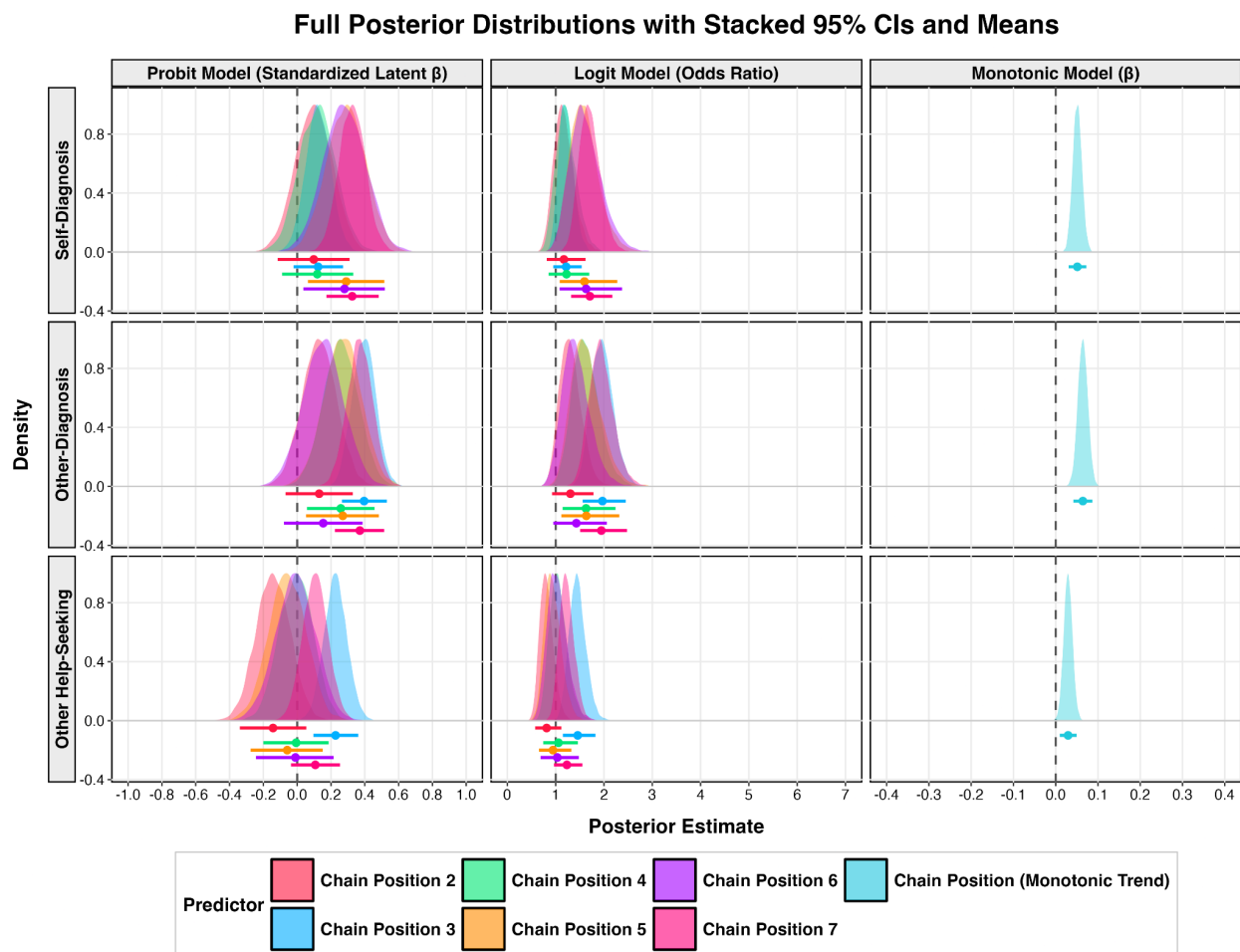
Supplementary Figure 2: Mega-Analytic Model Predicted Effects of Peer-to-Peer Transmission. Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position as a monotonic variable) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree).

Shaded ribbons represent Bayesian 95% CrI that incorporate uncertainty around both population-level estimates and variation between transmission chains (varying or random effects). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.

Predictor	Mean [95% Credible Interval] (Pr($\beta > 0$))								
	Self-Diagnosis	Self-Diagnosis (Odds Ratio)	Self-Diagnosis (Monotonic)	Other-Diagnosis	Other-Diagnosis (Odds Ratio)	Other-Diagnosis (Monotonic)	Other Help-Seeking	Other Help-Seeking (Odds Ratio)	Other Help-Seeking (Monotonic)
Intercept 1	0.42 [0.15, 0.69]		0.40 [0.15, 0.64]	-0.84 [-1.09, -0.59]		-0.93 [-1.17, -0.70]	-0.63 [-0.87, -0.39]		-0.61 [-0.83, -0.39]
Intercept 2	1.11 [0.84, 1.38]		1.08 [0.84, 1.33]	0.41 [0.17, 0.66]		0.31 [0.08, 0.55]	0.44 [0.20, 0.68]		0.46 [0.23, 0.68]
Intercept 3	1.46 [1.19, 1.73]		1.43 [1.19, 1.68]	0.82 [0.58, 1.07]		0.73 [0.49, 0.96]	1.05 [0.81, 1.29]		1.06 [0.84, 1.29]
Intercept 4	2.34 [2.06, 2.62]		2.31 [2.05, 2.57]	2.21 [1.96, 2.47]		2.11 [1.87, 2.36]	2.06 [1.80, 2.31]		2.07 [1.83, 2.31]
Chain Position 2	0.10 [-0.12, 0.31] (0.81)	1.17 [0.82, 1.62] (0.79)		0.13 [-0.07, 0.33] (0.90)	1.30 [0.93, 1.78] (0.93)		-0.14 [-0.34, 0.05] (0.08)	0.81 [0.58, 1.12] (0.10)	
Chain Position 3	0.12 [-0.02, 0.27] (0.95)	1.22 [0.95, 1.54] (0.94)		0.40 [0.26, 0.53] (1.00)	1.97 [1.56, 2.45] (1.00)		0.23 [0.10, 0.36] (1.00)	1.46 [1.15, 1.82] (1.00)	
Chain Position 4	0.12 [-0.09, 0.33] (0.86)	1.22 [0.85, 1.69] (0.85)		0.26 [0.06, 0.46] (0.99)	1.63 [1.14, 2.24] (1.00)		-0.01 [-0.20, 0.19] (0.48)	1.06 [0.74, 1.46] (0.60)	
Chain Position 5	0.29 [0.06, 0.52] (0.99)	1.60 [1.08, 2.28] (0.99)		0.27 [0.05, 0.48] (0.99)	1.63 [1.12, 2.32] (0.99)		-0.06 [-0.27, 0.15] (0.30)	0.94 [0.65, 1.32] (0.35)	
Chain Position 6	0.28 [0.04, 0.52] (0.99)	1.63 [1.08, 2.37] (0.99)		0.15 [-0.08, 0.39] (0.90)	1.43 [0.95, 2.06] (0.95)		-0.01 [-0.24, 0.22] (0.47)	1.03 [0.69, 1.48] (0.53)	
Chain Position 7	0.33 [0.17, 0.48] (1.00)	1.71 [1.32, 2.17] (1.00)		0.37 [0.22, 0.52] (1.00)	1.95 [1.51, 2.48] (1.00)		0.11 [-0.04, 0.25] (0.93)	1.23 [0.96, 1.55] (0.95)	
Chain Position (Monotonic Trend)			0.05 [0.03, 0.07] (1.00)			0.06 [0.04, 0.09] (1.00)			0.03 [0.01, 0.05] (1.00)
Simplex: Step 1 $\rightarrow$ 2			0.18 [0.03, 0.39]			0.42 [0.15, 0.65]			0.19 [0.03, 0.44]

Supplementary Information									
Simplex: Step 2 → 3		0.15 [0.02, 0.35]			0.22 [0.04, 0.48]			0.25 [0.04, 0.52]	
Simplex: Step 3 → 4		0.14 [0.02, 0.33]			0.09 [0.01, 0.22]			0.13 [0.02, 0.34]	
Simplex: Step 4 → 5		0.21 [0.04, 0.45]			0.07 [0.01, 0.19]			0.12 [0.02, 0.32]	
Simplex: Step 5 → 6		0.17 [0.03, 0.39]			0.07 [0.01, 0.19]			0.15 [0.02, 0.36]	
Simplex: Step 6 → 7		0.16 [0.02, 0.38]			0.13 [0.02, 0.31]			0.16 [0.02, 0.39]	
Day: Monday	0.07 [-0.13, 0.28] (0.74)	1.10 [0.78, 1.50] (0.69)	0.05 [-0.09, 0.19] (0.77)	-0.07 [-0.26, 0.12] (0.25)	0.92 [0.66, 1.25] (0.28)	0.03 [-0.11, 0.17] (0.68)	-0.26 [-0.45, -0.07] (0.00)	0.67 [0.48, 0.91] (0.01)	-0.06 [-0.18, 0.07] (0.18)
Day: Tuesday	0.03 [-0.19, 0.26] (0.61)	1.09 [0.74, 1.55] (0.64)	0.09 [-0.09, 0.27] (0.83)	0.03 [-0.18, 0.24] (0.61)	1.07 [0.73, 1.51] (0.61)	0.02 [-0.15, 0.20] (0.60)	0.07 [-0.14, 0.28] (0.74)	1.15 [0.78, 1.62] (0.74)	0.03 [-0.14, 0.20] (0.63)
Day: Wednesday	0.01 [-0.14, 0.16] (0.56)	1.02 [0.80, 1.28] (0.52)	-0.00 [-0.11, 0.11] (0.47)	0.10 [-0.04, 0.24] (0.92)	1.15 [0.90, 1.43] (0.87)	0.02 [-0.08, 0.12] (0.65)	0.01 [-0.13, 0.15] (0.54)	1.00 [0.79, 1.25] (0.47)	-0.01 [-0.11, 0.09] (0.41)
Day: Thursday	-0.01 [-0.17, 0.16] (0.47)	0.98 [0.75, 1.27] (0.42)	-0.02 [-0.14, 0.10] (0.36)	-0.06 [-0.21, 0.10] (0.23)	0.94 [0.71, 1.20] (0.29)	0.01 [-0.10, 0.12] (0.57)	-0.22 [-0.38, -0.07] (0.00)	0.72 [0.54, 0.92] (0.01)	-0.07 [-0.18, 0.04] (0.11)
Day: Saturday	0.40 [-1.00, 1.79] (0.72)	2.10 [0.26, 7.54] (0.68)	0.41 [-0.99, 1.78] (0.72)	-0.35 [-1.78, 1.09] (0.32)	1.08 [0.13, 4.21] (0.36)	-0.34 [-1.80, 1.08] (0.32)	-0.17 [-1.65, 1.27] (0.41)	1.22 [0.16, 4.51] (0.42)	-0.14 [-1.55, 1.23] (0.43)
Study 2	0.12 [-0.13, 0.39] (0.82)	1.32 [0.84, 2.00] (0.88)	0.12 [-0.12, 0.36] (0.85)	0.20 [-0.04, 0.45] (0.95)	1.33 [0.89, 1.92] (0.91)	0.08 [-0.13, 0.28] (0.76)	0.10 [-0.13, 0.33] (0.81)	1.17 [0.78, 1.69] (0.75)	-0.02 [-0.23, 0.19] (0.43)
Study 3	0.28 [0.03, 0.53] (0.99)	1.73 [1.12, 2.60] (0.99)	0.28 [0.05, 0.51] (0.99)	0.33 [0.10, 0.57] (1.00)	1.62 [1.10, 2.32] (0.99)	0.21 [0.00, 0.42] (0.98)	0.30 [0.07, 0.52] (0.99)	1.63 [1.10, 2.34] (0.99)	0.19 [-0.02, 0.40] (0.96)
Study 4	0.16 [-0.14, 0.45] (0.85)	1.43 [0.86, 2.26] (0.91)	0.15 [-0.14, 0.43] (0.85)	0.15 [-0.12, 0.43] (0.86)	1.19 [0.75, 1.82] (0.74)	0.00 [-0.25, 0.26] (0.51)	0.16 [-0.11, 0.42] (0.88)	1.27 [0.79, 1.93] (0.82)	0.04 [-0.22, 0.29] (0.61)
SD: Between-Chain Baseline Variation	0.04 [0.00, 0.11]		0.04 [0.00, 0.11]	0.07 [0.00, 0.15]		0.06 [0.00, 0.15]	0.04 [0.00, 0.11]		0.04 [0.00, 0.12]
SD: Between-Chain Effect Variation			0.01 [0.00, 0.03]		0.01 [0.00, 0.03]				0.01 [0.00, 0.03]

Supplementary Table 9: Bayesian Multilevel Ordinal Regression Results by Outcome. Intercept terms here indicate Likert scale cutpoints which represent where a participant's answer shifts from one Likert scale option to the next. Chain position was treated as a categorical and monotonic variable respectively. For categorical models, each chain position term is a contrast calculated from Chain Position 1 where participants read the ADHD DSM-adapted criteria. For monotonic models, Chain Position (Monotonic Trend) represents the rate of change in the particular outcome over the course of transmission. Relatedly, all simplex terms show the % of this effect that occurred in transitioning from one level of chain position to the next. The reference category for the Day predictor is Friday and the Study predictor is Study 1. $\Pr(\beta > 0)$ denotes probability of a positive association and is only provided for the main predictors. Likert scale cutpoints and varying intercept terms (i.e. SD) for the logit models aren't presented here owing to difference in scale units of such models (i.e. log-odds).



Supplementary Figure 3: Full Posterior Distributions of Mega-Analytic Effects of Transmission on Self-Diagnosis, Other-Diagnosis and Other Help-Seeking across probit, logit and monotonic model variants. For probit and logit categorical models, each Chain Position term is a contrast from Chain Position 1. For monotonic models, the posterior represents the average increase in the pertinent outcome over the course of transmission (i.e. across all chain positions). Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean.

3.3 Mediation Analyses

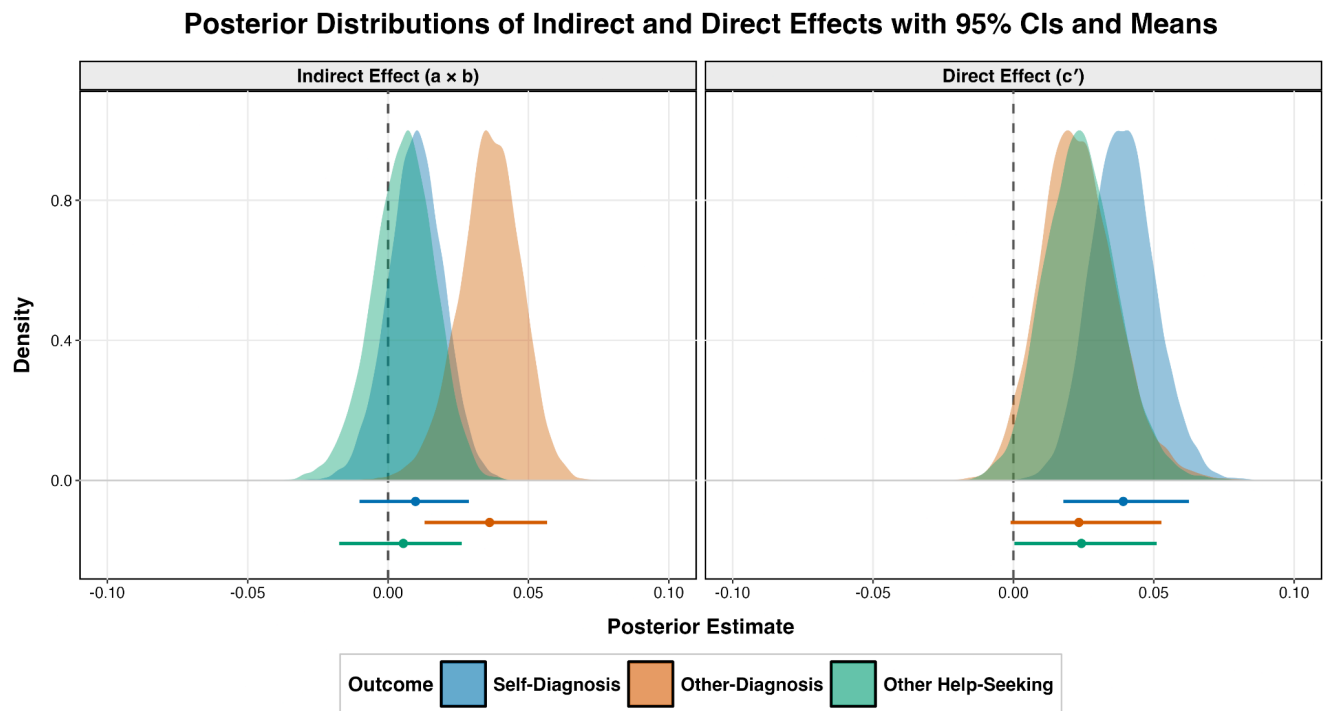
To ensure that analysing all outcome variables as continuous (rather than ordinal) was tenable for the mediation analyses, we compared whether such regressions yield similar estimates to their ordinal model variants. We found that such models indeed yield comparable estimates for our main estimand of interest (i.e. Chain Position (Monotonic Trend)). Refer to **Supplementary Table 10** below and **Supplementary Table 9** above.

Predictor	Mean [95% Credible Interval] (Pr($\beta > 0$))		
	Self-Diagnosis	Other-Diagnosis	Other Help-Seeking
Chain Position (Monotonic Trend)	0.05 [0.03, 0.06] (1.00)	0.06 [0.04, 0.08] (1.00)	0.03 [0.01, 0.05] (1.00)
Simplex: Step 1 $\rightarrow$ 2	0.17 [0.03, 0.38]	0.41 [0.14, 0.64]	0.19 [0.03, 0.43]
Simplex: Step 2 $\rightarrow$ 3	0.14 [0.02, 0.34]	0.23 [0.04, 0.47]	0.25 [0.04, 0.52]
Simplex: Step 3 $\rightarrow$ 4	0.13 [0.02, 0.33]	0.09 [0.01, 0.23]	0.13 [0.02, 0.33]
Simplex: Step 4 $\rightarrow$ 5	0.22 [0.04, 0.48]	0.07 [0.01, 0.19]	0.12 [0.02, 0.32]
Simplex: Step 5 $\rightarrow$ 6	0.16 [0.03, 0.39]	0.08 [0.01, 0.20]	0.15 [0.02, 0.37]
Simplex: Step 6 $\rightarrow$ 7	0.16 [0.02, 0.38]	0.13 [0.02, 0.30]	0.16 [0.02, 0.38]
Day: Monday	0.05 [-0.07, 0.18] (0.80)	0.03 [-0.10, 0.16] (0.68)	-0.06 [-0.18, 0.06] (0.17)
Day: Tuesday	0.07 [-0.09, 0.23] (0.80)	0.01 [-0.14, 0.17] (0.57)	0.01 [-0.15, 0.17] (0.55)
Day: Wednesday	-0.00 [-0.09, 0.09] (0.50)	0.01 [-0.08, 0.11] (0.61)	-0.02 [-0.11, 0.07] (0.34)
Day: Thursday	-0.01 [-0.12, 0.09] (0.40)	0.01 [-0.09, 0.12] (0.59)	-0.07 [-0.17, 0.03] (0.08)
Day: Saturday	0.46 [-0.92, 1.84] (0.73)	-0.40 [-1.79, 0.99] (0.28)	-0.21 [-1.60, 1.16] (0.39)
Study 2	0.07 [-0.13, 0.27] (0.77)	0.06 [-0.13, 0.26] (0.73)	-0.00 [-0.20, 0.19] (0.48)
Study 3	0.21 [0.01, 0.41] (0.98)	0.18 [-0.01, 0.37] (0.96)	0.19 [-0.01, 0.39] (0.97)
Study 4	0.07 [-0.17, 0.32] (0.73)	-0.01 [-0.25, 0.22] (0.47)	0.05 [-0.19, 0.29] (0.67)
SD: Between-Chain Baseline Variation	0.04 [0.00, 0.10]	0.05 [0.00, 0.13]	0.04 [0.00, 0.11]
SD: Between-Chain Effect Variation	0.01 [0.00, 0.04]	0.01 [0.00, 0.03]	0.01 [0.00, 0.03]
SD: Residual	0.99 [0.97, 1.02]	0.99 [0.96, 1.01]	0.99 [0.97, 1.02]
Intercept	-0.27 [-0.48, -0.06]	-0.34 [-0.56, -0.13]	-0.15 [-0.35, 0.05]

Supplementary Table 10: Bayesian Multilevel Gaussian Regression Results by Outcome. Pr($\beta > 0$) denotes probability of a positive association and is only provided for the main predictors.

Path	Mean [95% Credible Interval] (Pr($\beta > 0$))		
	Self-Diagnosis	Other-Diagnosis	Other Help-Seeking
a: Transmission $\rightarrow$ Semantic Drift	-0.399 [-0.407, -0.390] (0.00)	-0.399 [-0.407, -0.390] (0.00)	-0.399 [-0.407, -0.390] (0.00)
b: Semantic Drift $\rightarrow$ Outcome	-0.024 [-0.072, 0.026] (0.16)	-0.091 [-0.142, -0.033] (0.00)	-0.014 [-0.066, 0.044] (0.30)
c': Direct Effect (Transmission $\rightarrow$ Outcome)	0.039 [0.018, 0.063] (1.00)	0.023 [-0.001, 0.053] (0.97)	0.024 [0.000, 0.051] (0.98)
a $\times$ b: Indirect Effect	0.010 [-0.010, 0.029] (0.84)	0.036 [0.013, 0.057] (1.00)	0.005 [-0.017, 0.026] (0.70)
c: Total Effect	0.049 [0.030, 0.068] (1.00)	0.059 [0.041, 0.079] (1.00)	0.030 [0.011, 0.049] (1.00)
Proportion Mediated (a $\times$ b / c)	0.194 [-0.238, 0.560] (0.84)	0.620 [0.217, 1.021] (1.00)	-0.065 [-0.813, 0.986] (0.70)

Supplementary Table 11: Bayesian Mediation Analyses Results by Outcome.



Supplementary Figure 4: Full Posterior Distributions of Direct and Indirect Effects of Transmission on Self-Diagnosis, Other-Diagnosis and Other Help-Seeking. Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean. NB: The x-axis limits have been restricted to 0.1 SD to facilitate better visualisation of uncertainty in the posterior distributions of these effects.

4. Study 1 Results

4.1 Descriptive Statistics

Chain position	n	Age (Mean)	Sample Characteristics (%)					ADHD Prior Knowledge (M, 1-5)		
			Female	MH Medication	MH Treatment	NHS MH Support	Self-Reported AI Use	Social Media	Conversations	Other Sources
1	30	46.73	53.33	0	3.33	0.00	6.67	1.60	1.87	1.90
2	30	47.00	43.33	0	0.00	0.00	0.00	1.53	1.93	1.73
3	30	44.90	46.67	0	0.00	0.00	3.33	1.53	1.77	1.53
4	28	49.71	57.14	0	0.00	3.57	0.00	1.82	2.29	2.14

***Supplementary Table 12:** Sample characteristics and ADHD Prior Knowledge by Chain Position (Study 1). MH = Mental Health. NHS = National Health Service. Percentages reflect % of participants answering “Yes” to a question. MH Medication = currently taking any medication to treat symptoms of a mental health or psychiatric condition; MH Treatment = currently receiving or waiting for treatment for symptoms of a mental health condition; NHS MH Support = attempt to access mental health support on the NHS in the last 12 months. Prior Knowledge of ADHD questions were rated on 5 point Likert scale (1 = Never/0 times a week to 5 = Very Often/6 or more times a week)*

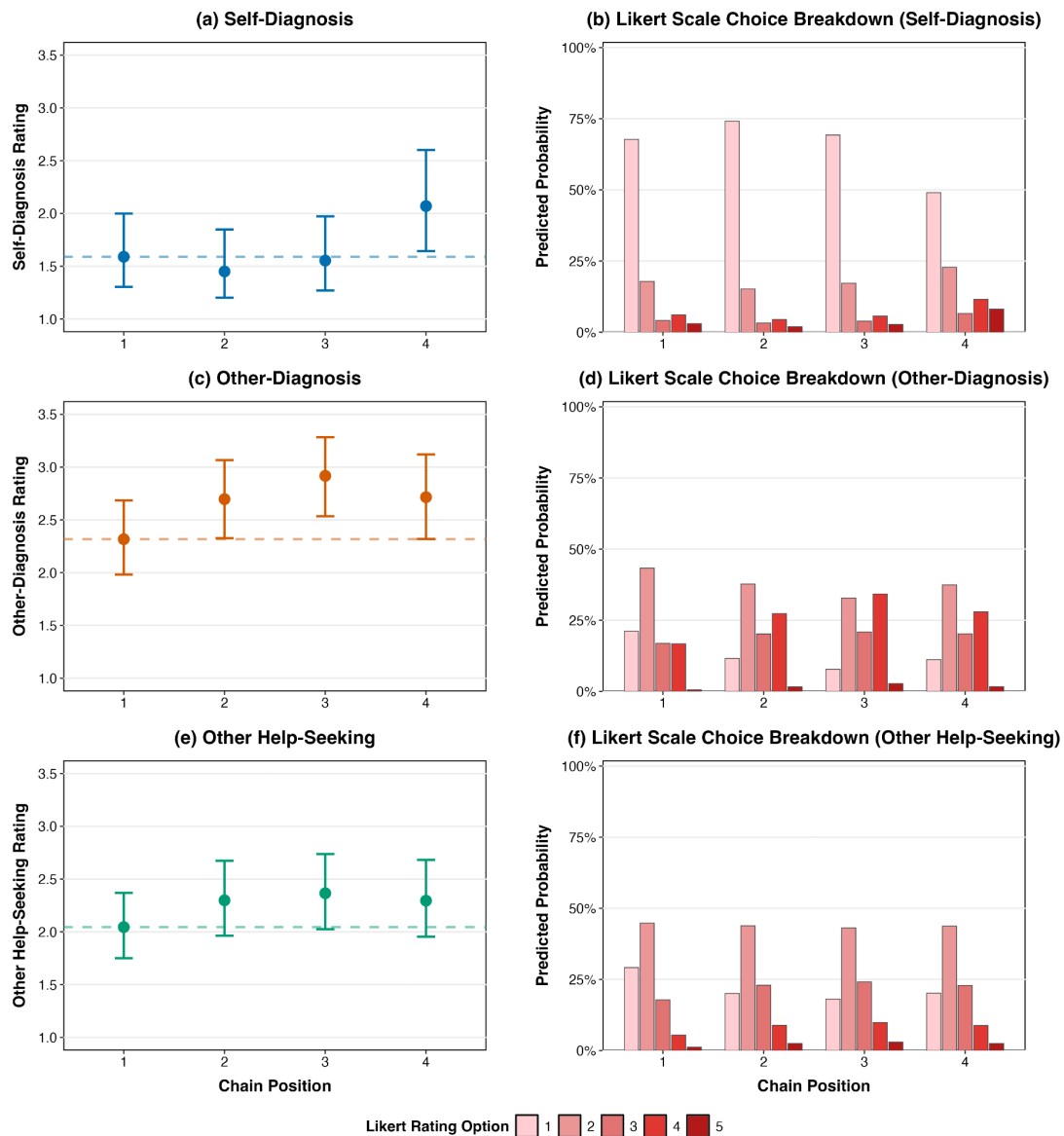
4.2 Reliability of Self-Diagnosis Measure

All four items showed high reliability in measuring self-diagnosis (standardised cronbach’s alpha = 0.88, 95% CI: [0.82, 0.90]). Thus, we proceeded to utilise the self-diagnosis measure (“I believe I have ADHD”) in our subsequent pre-registered experiments.

4.3 Main Analyses

Relative to initial participants exposed to the ADHD DSM-5-TR criteria, we found moderate to strong evidence that participants at the end of four rounds of transmission had 113% higher odds of diagnosing themselves and 101% higher odds of diagnosing another subclinical individual with ADHD (see **Supplementary Table 13, SFig 5 and 6 below; Self-diagnosis - Chain Position 4 contrast (CP4):** 2.13 OR, 95% CrI [0.81, 4.55], $P_{>0} = 93\%$; 0.48 latent SD, 95% CrI [-0.08, 1.05], $P_{>0} = 95\%$; **Other-diagnosis - CP4:** 2.01 OR, 95% CrI [0.79, 4.24], $P_{>0} = 92\%$; 0.41 latent SD, 95% CrI [-0.11, 0.96], $P_{>0} = 94\%$). For self-diagnosis, this effect was primarily driven by transmission reducing participants’ extreme skepticism (i.e. choosing 1 = Strongly Disagree with the item “I believe I have ADHD”; see **SFig 5b**) over time whereas the other-diagnosis effect was driven by transmission increasing participants’ moderate endorsement (i.e. choosing 4 = Somewhat Agree with the item “I believe Vincent has ADHD”; see **SFig 5d**).

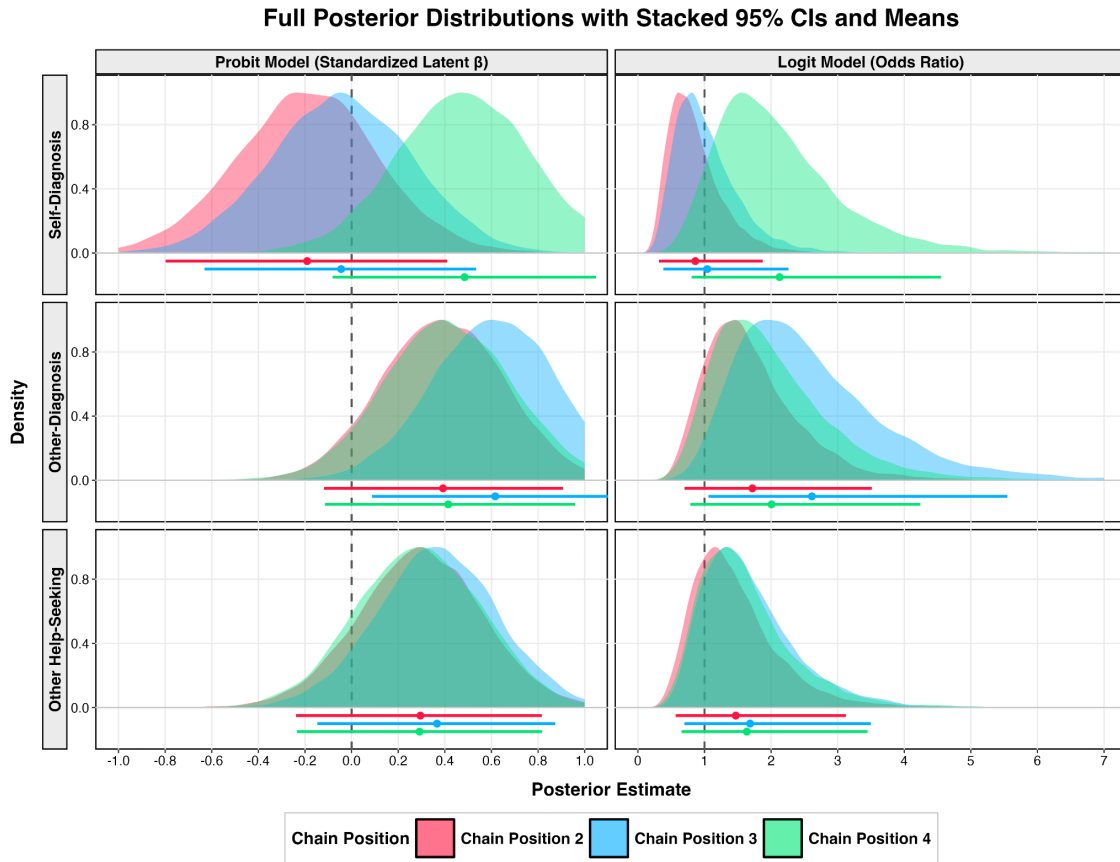
For other help-seeking, we found moderate evidence that participants at the end of four rounds of transmission had 64% higher odds of recommending another subclinical individual to seek professional help for ADHD relative to initial participants exposed to the ADHD DSM-5-TR criteria (see **Supplementary Table 13, SFig 5 and 6 below; Other Help-Seeking - CP4**: 1.64 OR, 95% CrI [0.65, 3.45], $P_{>0} = 83\%$; 0.29 latent SD, 95% CrI [-0.24, 0.82], $P_{>0} = 86\%$). This effect was driven by transmission reducing participants' extreme disagreement (i.e. choosing 1 = Strongly Disagree with the item "I believe that Vincent should seek professional mental health help (e.g. therapy or counselling)"; see **SFig 5f**) over time.



Supplementary Figure 5: Study 1 Model Predicted Effects of Peer-to-Peer Transmission. Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.

Predictor	Mean [95% Credible Interval] (Pr($\beta > 0$))					
	Self-Diagnosis	Self-Diagnosis (Odds Ratio)	Other-Diagnosis	Other-Diagnosis (Odds Ratio)	Other Help-Seeking	Other Help-Seeking (Odds Ratio)
Intercept 1	0.46 [0.05, 0.89]		-0.80 [-1.23, -0.39]		-0.55 [-0.94, -0.15]	
Intercept 2	1.09 [0.64, 1.54]		0.39 [-0.01, 0.80]		0.67 [0.29, 1.08]	
Intercept 3	1.32 [0.85, 1.79]		0.94 [0.52, 1.36]		1.48 [1.04, 1.94]	
Intercept 4	1.88 [1.34, 2.45]		2.55 [1.91, 3.26]		2.26 [1.69, 2.89]	
Chain Position 2	-0.19 [-0.80, 0.41] (0.26)	0.86 [0.31, 1.87] (0.29)	0.39 [-0.12, 0.91] (0.93)	1.72 [0.70, 3.51] (0.87)	0.30 [-0.24, 0.82] (0.87)	1.47 [0.57, 3.12] (0.75)
Chain Position 3	-0.05 [-0.63, 0.53] (0.44)	1.04 [0.38, 2.26] (0.45)	0.62 [0.09, 1.15] (0.99)	2.61 [1.06, 5.55] (0.98)	0.37 [-0.15, 0.87] (0.92)	1.68 [0.70, 3.50] (0.85)
Chain Position 4	0.48 [-0.08, 1.05] (0.95)	2.13 [0.81, 4.55] (0.93)	0.41 [-0.11, 0.96] (0.94)	2.01 [0.79, 4.24] (0.92)	0.29 [-0.24, 0.82] (0.86)	1.64 [0.65, 3.45] (0.83)
SD: Between-Chain Baseline Variation	0.16 [0.01, 0.48]		0.17 [0.01, 0.45]		0.21 [0.01, 0.52]	

Supplementary Table 13: Bayesian Multilevel Ordinal Regression Results of Study 1 by Outcome. Pr($\beta > 0$) denotes probability of a positive association and is only provided for the main predictors. Likert scale cutpoint and varying intercept terms (i.e. SD) for the logit models aren't presented here owing to difference in scale units of such models (i.e. log-odds).



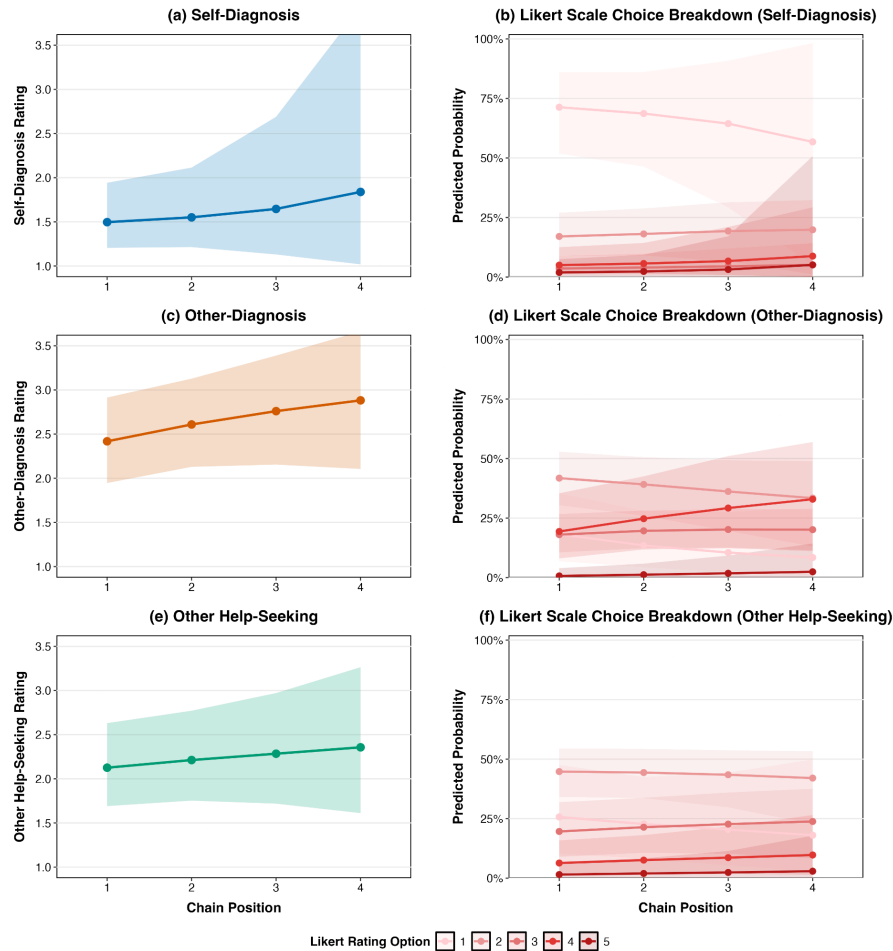
Supplementary Figure 6: Full Posterior Distributions of Study 1 Effects of Transmission on Self-Diagnosis, Other-Diagnosis and Other Help-Seeking. Each Chain Position term is a contrast from Chain Position 1. Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean. NB: For some probit and logit models, some posterior distributions aren't fully displayed due to limiting the x-axis scale. This limit was imposed to facilitate comparison of effects across equivalent model classes run across different studies.

4.4 Sensitivity Analyses

Study 1 had a small sample and differing number of control variables owing to its exploratory goals. To transparently account for further unobserved confounds without adding controls that are incommensurate with our other studies, we ran model specifications where chain position was treated as a monotonic variable and a varying slope was specified on it. This allowed us to investigate how noisy the aforementioned transmission effects are i.e. *whether they varied widely across distinct chains*. No logistic model variants were run here, as trends are better interpreted in latent SD units.

These analyses found moderate to strong evidence that self- and other-diagnosis increased over the course of transmission, though these effects varied substantially between chains. (see **SFig 7, 8 and Supplementary Table 14**; **Self-diagnosis** - monotonic latent sd change = 0.12, 95% CrI [-0.12, 0.34], $P_{>0} = 86\%$; **Other-diagnosis** - monotonic

latent sd change = 0.16, 95% CrI [-0.02, 0.34], $P_{>0} = 96\%$). There was uncertain, but suggestive evidence of other help-seeking also increasing over the course of transmission (see **SFig 7, 8 and Supplementary Table 14; Other Help-Seeking** - monotonic latent sd change = 0.09, 95% CrI [-0.09, 0.27], $P_{>0} = 84\%$).

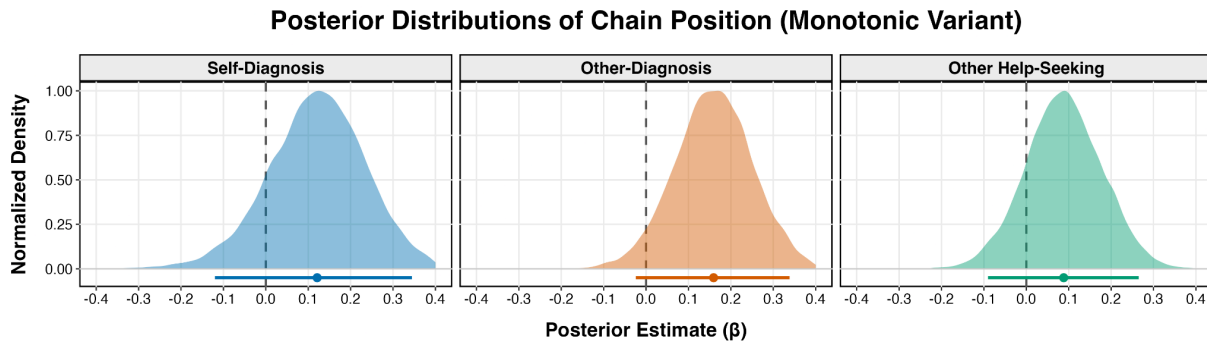


Supplementary Figure 7: Study 1 Model Predicted Effects of Peer-to-Peer Transmission. Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position as a monotonic variable) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree). Shaded ribbons represent Bayesian 95% CrI that incorporate uncertainty around both population-level estimates and variation between transmission chains (varying or random effects).. NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.

Predictor	Mean [95% Credible Interval] (Pr($\beta > 0$))		
	Self-Diagnosis	Other-Diagnosis	Other Help-Seeking
Intercept 1	0.56 [0.22, 0.91]	-0.91 [-1.32, -0.49]	-0.66 [-1.02, -0.28]
Intercept 2	1.22 [0.83, 1.62]	0.28 [-0.08, 0.69]	0.58 [0.22, 0.96]

Intercept 3	1.46 [1.06, 1.90]	0.83 [0.44, 1.25]	1.40 [0.99, 1.82]
Intercept 4	2.08 [1.56, 2.68]	2.46 [1.85, 3.17]	2.17 [1.63, 2.78]
Chain Position (Monotonic Trend)	0.12 [-0.12, 0.34] (0.86)	0.16 [-0.02, 0.34] (0.96)	0.09 [-0.09, 0.27] (0.84)
Simplex: Step 1 → 2	0.22 [0.03, 0.56]	0.40 [0.08, 0.75]	0.36 [0.07, 0.73]
Simplex: Step 2 → 3	0.30 [0.05, 0.67]	0.33 [0.06, 0.69]	0.31 [0.05, 0.68]
Simplex: Step 3 → 4	0.48 [0.11, 0.82]	0.28 [0.04, 0.65]	0.33 [0.06, 0.70]
SD: Between-Chain Baseline Variation	0.16 [0.01, 0.46]	0.16 [0.01, 0.45]	0.20 [0.01, 0.51]
SD: Between-Chain Effect Variation	0.25 [0.01, 0.65]	0.09 [0.00, 0.26]	0.10 [0.00, 0.28]

Supplementary Table 14: Bayesian Multilevel Ordinal Regression Results of Study 1 by Outcome. $\Pr(\beta > 0)$ denotes probability of a positive association and is only provided for the main predictors.



Supplementary Figure 8: Full Posterior Distributions of Study 1 Effects of Transmission on Self-Diagnosis, Other-Diagnosis and Other Help-Seeking. Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean. NB: Some posterior distributions aren't fully displayed due to limiting the x-axis scale. This limit was imposed to facilitate comparison of effects across equivalent monotonic models run across different studies.

5. Study 2 Results

5.1 Descriptive Statistics

Chain position	Observations	Age (Mean)	Female (%)	Clinical History (%)				ADHD Prior Knowledge (%)		
				MH Medication	MH Treatment	NHS MH Support	Past MH Diagnosis	Friend Support	Waitlist Support	Social Media
1	217	45.85	58.53	0.00	0.00	0.46	8.29	14.75	10.60	13.82
2	217	45.62	62.21	0.00	0.00	0.00	8.29	11.52	7.83	9.68
3	217	44.53	65.44	0.46	1.38	3.69	6.45	10.60	6.91	9.68
4	217	46.24	57.14	0.00	0.00	1.84	9.22	9.68	11.52	10.14
5	217	42.61	54.84	0.92	0.46	4.61	13.36	11.06	10.60	13.82
6	217	42.46	56.22	0.46	0.46	2.76	9.68	9.68	10.14	15.67
7	217	42.58	54.84	0.00	0.00	0.92	7.37	17.05	10.60	12.90

***Supplementary Table 15:** Sample characteristics, Clinical history and ADHD Prior Knowledge by Chain Position (Study 2). Percentages reflect % of participants answering “Yes” to a question. Past MH Diagnosis = having a formal clinical diagnosis of any mental health condition in the past. Friend Support = having a close friend or family member seeing a mental health professional for ADHD; Waitlist Support = having a close friend or family member on a waiting list to receive help for ADHD; Social Media = currently following or subscribing to social media accounts discussing neurodiversity.*

Transmission Format	Chain position	Observations	Age (Mean)	Female (%)	Clinical History (%)				ADHD Prior Knowledge (%)		
					MH Medication	MH Treatment	NHS MH Support	Past MH Diagnosis	Friend Support	Waitlist Support	Social Media
Examples	1	108	45.23	56.48	0.00	0.00	0.93	7.41	15.74	12.04	14.81
Examples	2	108	46.50	63.89	0.00	0.00	0.00	6.48	8.33	5.56	8.33
Examples	3	108	44.56	66.67	0.93	1.85	4.63	5.56	10.19	4.63	7.41
Examples	4	108	46.95	62.96	0.00	0.00	1.85	6.48	10.19	8.33	9.26
Examples	5	108	43.13	56.48	1.85	0.93	3.70	11.11	11.11	8.33	11.11
Examples	6	108	40.62	58.33	0.00	0.00	2.78	8.33	9.26	12.04	19.44
Examples	7	108	43.61	56.48	0.00	0.00	0.00	9.26	19.44	9.26	17.59

Narrative Summaries	1	109	46.46	60.55	0.00	0.00	0.00	9.17	13.76	9.17	12.84
Narrative Summaries	2	109	44.75	60.55	0.00	0.00	0.00	10.09	14.68	10.09	11.01
Narrative Summaries	3	109	44.50	64.22	0.00	0.92	2.75	7.34	11.01	9.17	11.93
Narrative Summaries	4	109	45.54	51.38	0.00	0.00	1.83	11.93	9.17	14.68	11.01
Narrative Summaries	5	109	42.10	53.21	0.00	0.00	5.50	15.60	11.01	12.84	16.51
Narrative Summaries	6	109	44.28	54.13	0.92	0.92	2.75	11.01	10.09	8.26	11.93
Narrative Summaries	7	109	41.56	53.21	0.00	0.00	1.83	5.50	14.68	11.93	8.26

Supplementary Table 16: Sample characteristics, Clinical history and ADHD Prior Knowledge by Chain Position and Transmission Format (Study 2).

AI Exposure Classification	Direct AI User	AI Usage in Transmission Chain	N	% of Total Sample
Pure non-AI observations	No	0	1,117	73.54
Indirect Exposure to an AI Explanation generated by previous participant in the chain	No	1	270	17.77
Participants who used AI to generate their explanation, but aren't considered to be exposed to AI	Yes	0	0	0.00
Participants who used AI to generate their Explanation	Yes	1	132	8.69

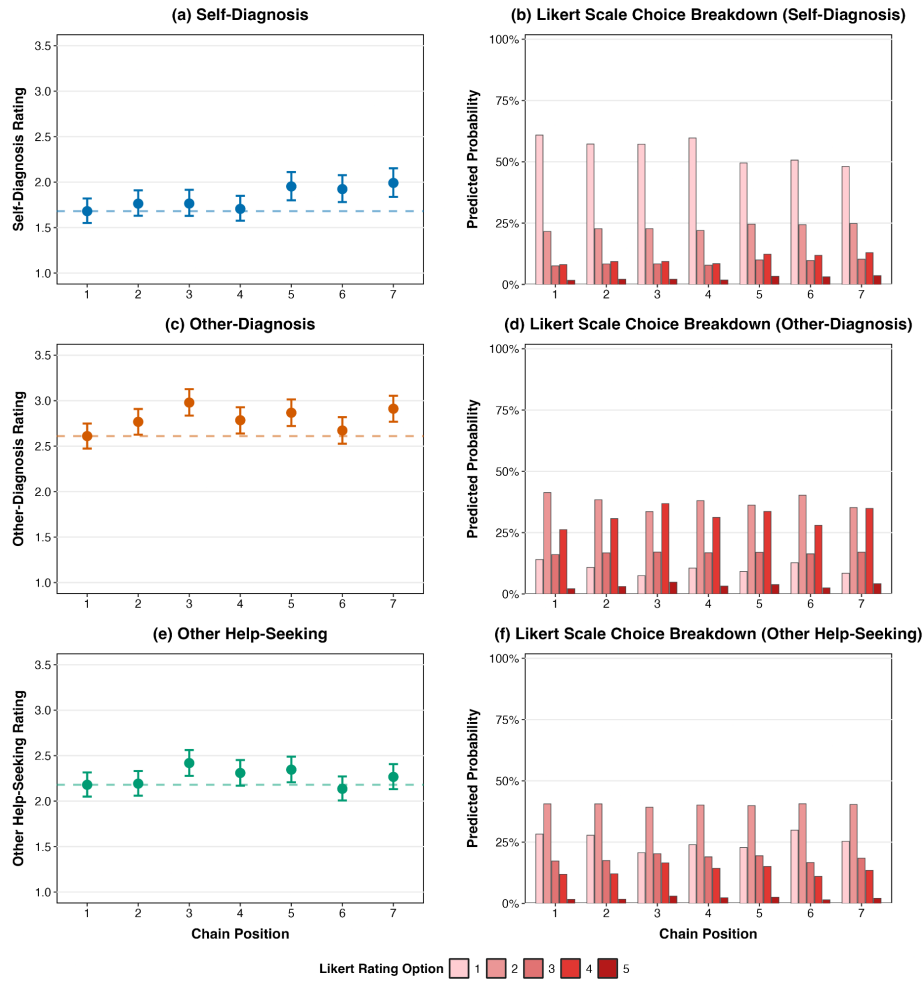
Supplementary Table 17: % of AI-usage in Study 2. Direct AI user = Participant using AI to generate their ADHD Explanation. AI Usage in Transmission chain = Chain having other participant(s) who has used AI.

8.69% of participants in Study 2 were detected using AI to generate their written ADHD explanations while 17.77% of this study sample were in a transmission chain where a previous participant had used AI.

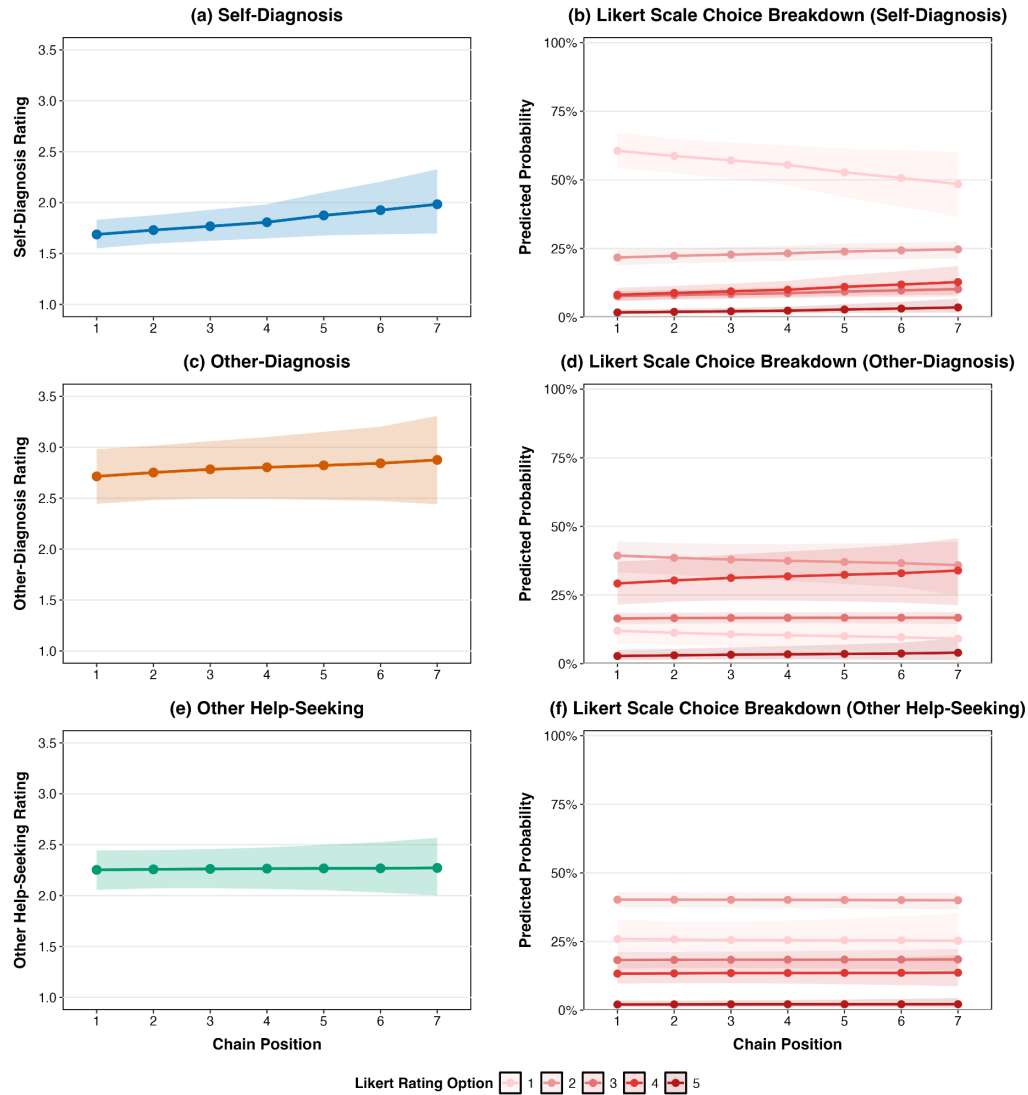
5.2 Main Analyses

Relative to initial participants exposed to the ADHD DSM-5-TR criteria, we found moderate to strong evidence that participants at the end of seven rounds of transmission had 73% higher odds of diagnosing themselves and 68% higher odds of diagnosing another subclinical individual with ADHD (see **Supplementary Table 18, SFig 9, 10 and 11 below; Self-diagnosis - Chain Position 7 contrast (CP7)**: 1.73 OR, 95% CrI [1.21, 2.42], $P_{>0} = 100\%$; 0.32 latent SD, 95% CrI [0.11, 0.53], $P_{>0} = 100\%$; **Other-diagnosis - CP7**: 1.68 OR, 95% CrI [1.19, 2.31], $P_{>0} = 100\%$; 0.29 latent SD, 95% CrI [0.10, 0.49], $P_{>0} = 100\%$). Importantly, exploratory trend analyses showed strong evidence for a monotonic increase in self- and other-diagnosis over the course of transmission (see **Supplementary Table 18, SFig 9, 10 and 11 below; Self-diagnosis** - monotonic latent sd change = 0.05, 95% CrI [0.02, 0.08], $P_{>0} = 100\%$; **Other-diagnosis** - monotonic latent sd change = 0.03, 95% CrI [-0.00, 0.06], $P_{>0} = 96\%$). For self-diagnosis, this effect was primarily driven by transmission reducing participants' extreme skepticism (i.e. choosing 1 = Strongly Disagree; see **SFig 9b** and **SFig 10b**) over time whereas the other-diagnosis effect was driven by transmission increasing participants' moderate endorsement (i.e. choosing 4 = Somewhat Agree; see **SFig 9d** and **SFig 10d**).

For other help-seeking, we found uncertain, but suggestive moderate evidence that participants at the end of seven rounds of transmission had 19% higher odds of recommending another subclinical individual to seek professional help for ADHD relative to initial participants exposed to the ADHD DSM-5-TR criteria (see **Supplementary Table 18, SFig 9, 10 and 11 below; Other Help-Seeking - CP7**: 1.19 OR, 95% CrI [0.84, 1.63], $P_{>0} = 82\%$; 0.09 latent SD, 95% CrI [-0.11, 0.29], $P_{>0} = 81\%$). Exploratory trend analyses yielded no credible evidence of a monotonic increase in other help-seeking over the course of transmission.



Supplementary Figure 9: Study 2 Model Predicted Effects of Peer-to-Peer Transmission. Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.



Supplementary Figure 10: Study 2 Model Predicted Effects of Peer-to-Peer Transmission. Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position as a monotonic variable) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree).

Shaded ribbons represent Bayesian 95% CrI that incorporate uncertainty around both population-level estimates and variation between transmission chains (varying or random effects). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.

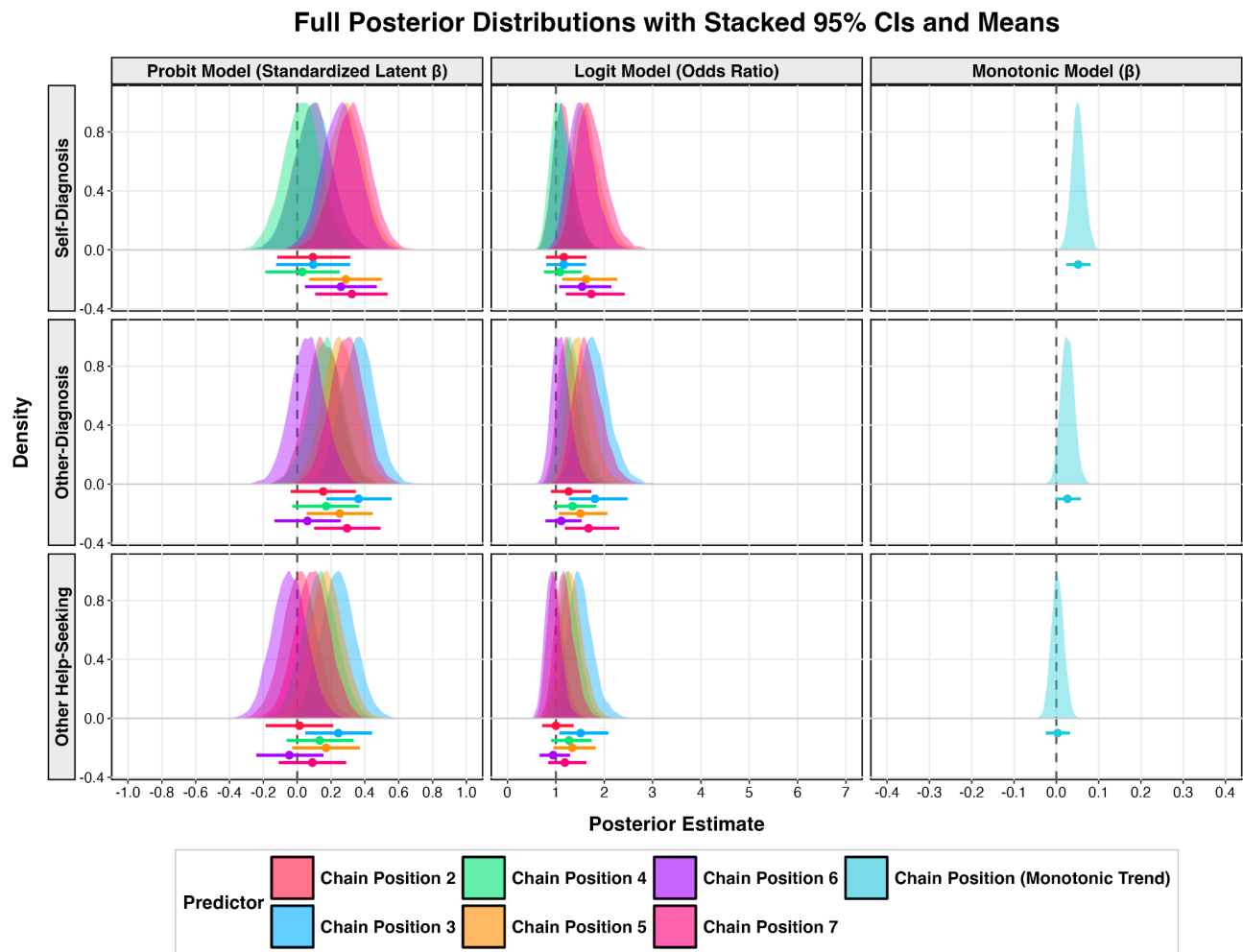
Mean [95% Credible Interval] ($\Pr(\beta > 0)$)

Predictor	Self-Diagnosis	Self-Diagnosis (Odds Ratio)	Self-Diagnosis (Monotonic)	Other-Diagnosis	Other-Diagnosis (Odds Ratio)	Other-Diagnosis (Monotonic)	Other Help-Seeking	Other Help-Seeking (Odds Ratio)	Other Help-Seeking (Monotonic)
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Intercept 1	0.28 [0.12, 0.44]	0.27 [0.16, 0.39]	-1.08 [-1.23, -0.93]	-1.18 [-1.30, -1.04]	-0.57 [-0.72, -0.43]	-0.65 [-0.75, -0.53]
Intercept 2	0.94 [0.78, 1.10]	0.93 [0.82, 1.05]	0.14 [-0.00, 0.28]	0.04 [-0.08, 0.17]	0.50 [0.35, 0.64]	0.42 [0.32, 0.54]
Intercept 3	1.29 [1.13, 1.46]	1.29 [1.17, 1.41]	0.57 [0.43, 0.71]	0.47 [0.35, 0.60]	1.10 [0.95, 1.25]	1.02 [0.91, 1.14]
Intercept 4	2.12 [1.93, 2.32]	2.12 [1.95, 2.28]	2.02 [1.85, 2.21]	1.92 [1.76, 2.09]	2.12 [1.93, 2.32]	2.04 [1.87, 2.21]
Chain Position 2	0.09 [-0.12, 0.31] (0.80)	1.16 [0.79, 1.64] (0.77)	0.15 [-0.04, 0.35] (0.94)	1.27 [0.89, 1.74] (0.91)	0.01 [-0.19, 0.21] (0.56)	1.00 [0.71, 1.37] (0.46)
Chain Position 3	0.09 [-0.13, 0.31] (0.80)	1.16 [0.80, 1.62] (0.77)	0.36 [0.17, 0.56] (1.00)	1.81 [1.27, 2.48] (1.00)	0.24 [0.05, 0.44] (0.99)	1.51 [1.07, 2.09] (0.99)
Chain Position 4	0.03 [-0.19, 0.25] (0.61)	1.09 [0.75, 1.53] (0.64)	0.17 [-0.03, 0.37] (0.96)	1.35 [0.95, 1.84] (0.95)	0.13 [-0.06, 0.33] (0.90)	1.27 [0.90, 1.74] (0.91)
Chain Position 5	0.29 [0.07, 0.50] (0.99)	1.62 [1.13, 2.27] (1.00)	0.25 [0.06, 0.45] (0.99)	1.51 [1.06, 2.07] (0.99)	0.17 [-0.03, 0.37] (0.95)	1.34 [0.95, 1.82] (0.95)
Chain Position 6	0.26 [0.05, 0.47] (0.99)	1.54 [1.07, 2.15] (0.99)	0.06 [-0.13, 0.26] (0.73)	1.11 [0.79, 1.53] (0.70)	-0.05 [-0.24, 0.16] (0.33)	0.94 [0.66, 1.29] (0.34)
Chain Position 7	0.32 [0.11, 0.53] (1.00)	1.73 [1.21, 2.42] (1.00)	0.29 [0.10, 0.49] (1.00)	1.68 [1.19, 2.31] (1.00)	0.09 [-0.11, 0.29] (0.81)	1.19 [0.84, 1.63] (0.82)
Chain Position (Monotonic Trend)		0.05 [0.02, 0.08] (1.00)		0.03 [-0.00, 0.06] (0.96)		0.00 [-0.03, 0.03] (0.58)
Simplex: Step 1 → 2		0.16 [0.02, 0.37]		0.21 [0.03, 0.50]		0.17 [0.03, 0.42]
Simplex: Step 2 → 3		0.14 [0.02, 0.33]		0.19 [0.03, 0.45]		0.17 [0.02, 0.43]
Simplex: Step 3 → 4		0.14 [0.02, 0.34]		0.13 [0.02, 0.35]		0.16 [0.02, 0.39]
Simplex: Step 4 → 5		0.23 [0.04, 0.48]		0.13 [0.02, 0.34]		0.16 [0.02, 0.39]
Simplex: Step 5 → 6		0.17 [0.03, 0.39]		0.13 [0.02, 0.35]		0.16 [0.02, 0.43]
Simplex: Step 6 → 7		0.18 [0.03, 0.40]		0.20 [0.03, 0.46]		0.18 [0.03, 0.42]

SD: Between-Chain Baseline Variation	0.05 [0.00, 0.13]	0.05 [0.00, 0.14]	0.11 [0.01, 0.23]	0.09 [0.00, 0.22]	0.07 [0.00, 0.18]	0.07 [0.00, 0.17]
SD: Between-Chain Effect Variation		0.02 [0.00, 0.04]		0.02 [0.00, 0.06]		0.01 [0.00, 0.04]

Supplementary Table 18: Bayesian Multilevel Ordinal Regression Results of Study 2 by Outcome. $Pr(\beta > 0)$ denotes probability of a positive association and is only provided for the main predictors. Likert scale cutpoint and varying intercept terms (i.e. SD) for the logit models aren't presented here owing to difference in scale units of such models (i.e. log-odds).



Supplementary Figure 11: Full Posterior Distributions of Study 2 Effects of Transmission on Self-Diagnosis, Other-Diagnosis and Other Help-Seeking across probit, logit and monotonic model variants. For probit and logit categorical

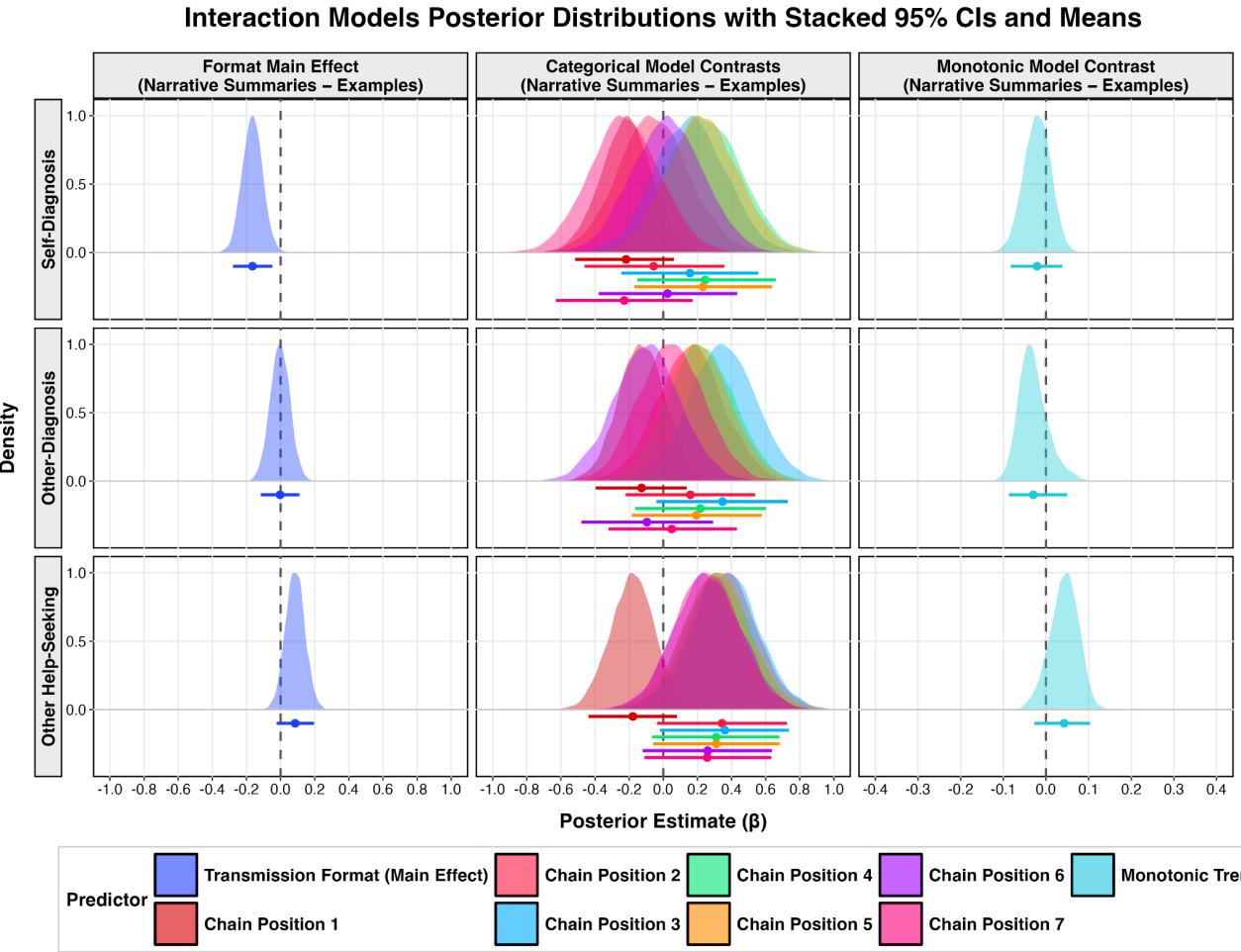
models, each Chain Position term is a contrast from Chain Position 1. For monotonic models, the posterior represents the average increase in the pertinent outcome over the course of transmission (i.e. across all chain positions). Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean.

5.2.1 Transmission Format Moderation Results

Predictor	Mean [95% Credible Interval] (Pr(β > 0))					
	Self-Diagnosis	Self-Diagnosis (Monotonic)	Other-Diagnosis	Other-Diagnosis (Monotonic)	Other Help-Seeking	Other Help-Seeking (Monotonic)
Transmission Type (Main Effect: Narrative Summaries - Examples)	-0.17 [-0.28, -0.05] (0.00)	-0.17 [-0.28, -0.05] (0.00)	-0.00 [-0.12, 0.11] (0.48)	-0.00 [-0.12, 0.11] (0.47)	0.09 [-0.02, 0.20] (0.94)	0.08 [-0.02, 0.19] (0.94)
Chain Position 2 × Transmission Format	-0.06 [-0.46, 0.36] (0.39)		0.16 [-0.22, 0.54] (0.80)		0.34 [-0.04, 0.73] (0.96)	
Chain Position 3 × Transmission Format	0.16 [-0.25, 0.56] (0.77)		0.35 [-0.04, 0.73] (0.96)		0.36 [-0.02, 0.74] (0.97)	
Chain Position 4 × Transmission Format	0.25 [-0.15, 0.66] (0.88)		0.22 [-0.16, 0.60] (0.87)		0.31 [-0.07, 0.68] (0.95)	
Chain Position 5 × Transmission Format	0.23 [-0.17, 0.64] (0.87)		0.19 [-0.19, 0.58] (0.84)		0.31 [-0.06, 0.68] (0.95)	
Chain Position 6 × Transmission Format	0.03 [-0.38, 0.43] (0.55)		-0.10 [-0.48, 0.29] (0.31)		0.26 [-0.12, 0.64] (0.91)	
Chain Position 7 × Transmission Format	-0.23 [-0.63, 0.17] (0.13)		0.05 [-0.32, 0.43] (0.60)		0.26 [-0.11, 0.63] (0.91)	
Chain Position 1 × Transmission Format	-0.22 [-0.52, 0.06] (0.07)		-0.13 [-0.40, 0.14] (0.18)		-0.18 [-0.44, 0.08] (0.09)	
Chain Position (Monotonic Trend) × Transmission Format		-0.02 [-0.08, 0.04] (0.26)		-0.03 [-0.09, 0.05] (0.18)		0.04 [-0.03, 0.10] (0.89)

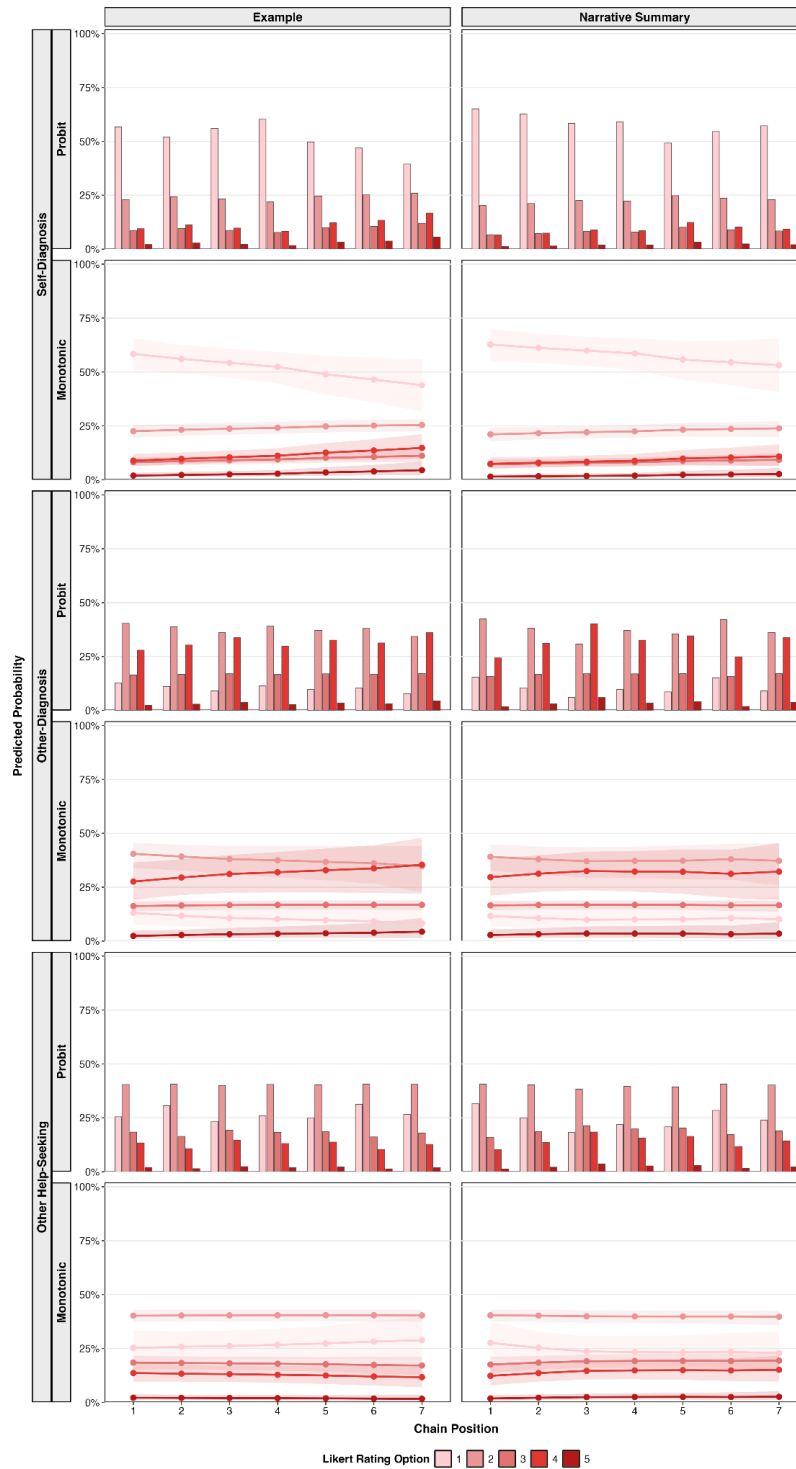
SD: Between-Chain Baseline Variation	0.05 [0.00, 0.13]	0.05 [0.00, 0.13]	0.12 [0.01, 0.24]	0.10 [0.01, 0.22]	0.07 [0.00, 0.18]	0.07 [0.00, 0.17]
SD: Between-Chain Effect Variation		0.02 [0.00, 0.04]		0.02 [0.00, 0.06]		0.01 [0.00, 0.04]

Supplementary Table 19: Bayesian Multilevel Ordinal Moderation Regression Results of Study 2 by Outcome. Chain position was treated as a categorical and monotonic variable respectively. For categorical models, Chain Position N x Transmission Format represents the difference-in-differences between narrative summaries and examples format at a specific chain position, relative to chain position 1. For monotonic models, Chain Position (Monotonic Trend) x Transmission Format represents the difference in trend increases in the pertinent outcome between narrative summaries and examples format. The format main effect was calculated from the interaction model using the marginales package. NB: Supplementary Table display is limited to selected estimands of interest to prevent overcrowding of core results. For further information on other estimands (e.g. simplex terms), please refer to the R scripts in the project repo.



Supplementary Figure 12: Full Posterior Distributions of Format Moderation Effects in Study 2 across probit and monotonic model variants. For probit categorical models, each Chain Position Posterior represents the difference-in-differences

in the relevant outcome between narrative summaries and examples, relative to Chain Position 1. Positive values on the x-axis mean that narrative summaries have a stronger impact. For monotonic models, the posterior represents the difference-in-differences in trend increases in the pertinent outcome between narrative summaries and examples format. Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean.

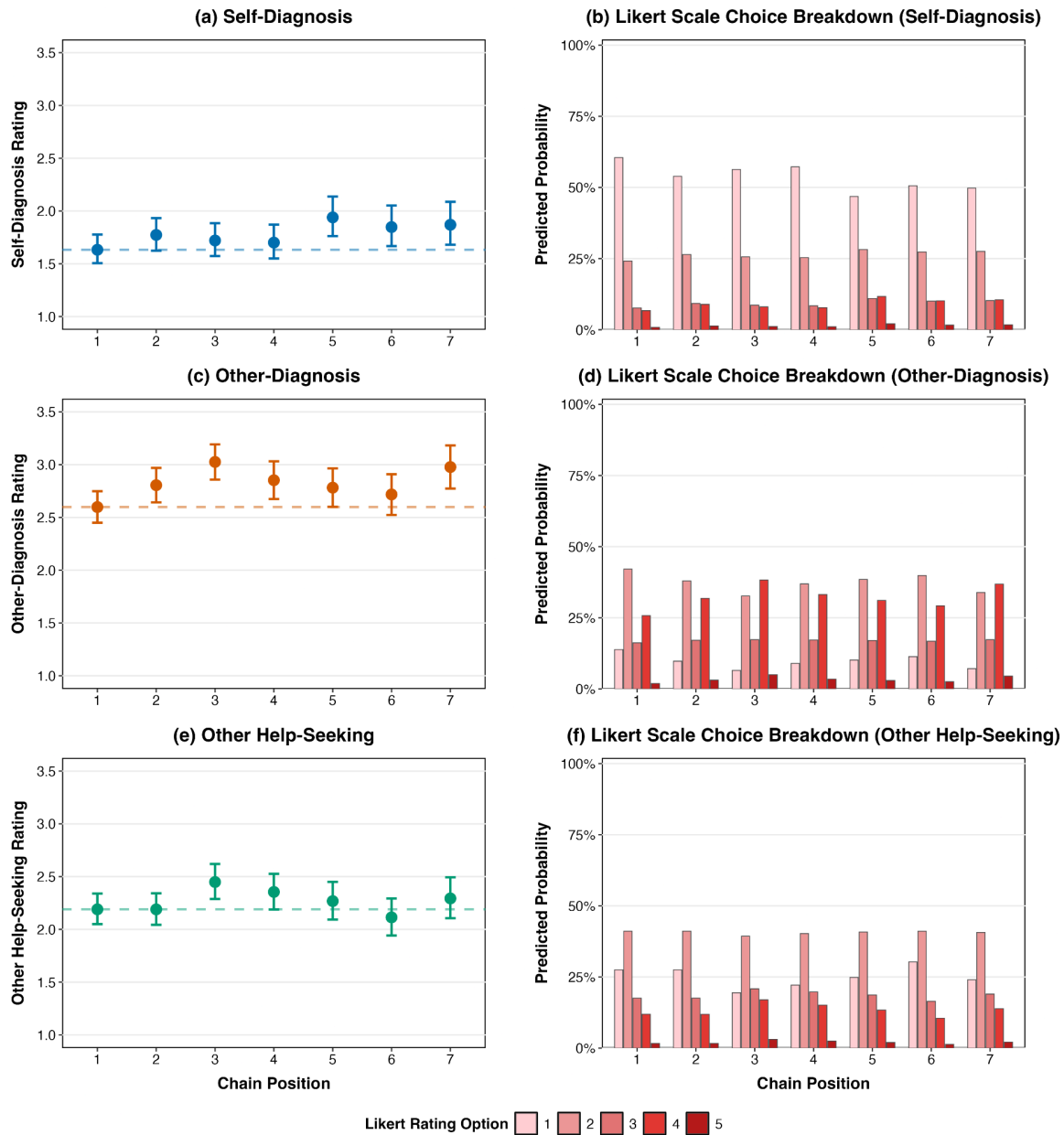


***Supplementary Figure 13:** Study 2 Model Predicted Effects of Peer-to-Peer Transmission by Transmission Format. All panels show the predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree) for a particular outcome, either as bar plots (Categorical Probit models) or continuous lines for choosing each option over the course of transmission (Monotonic Trend Models).*

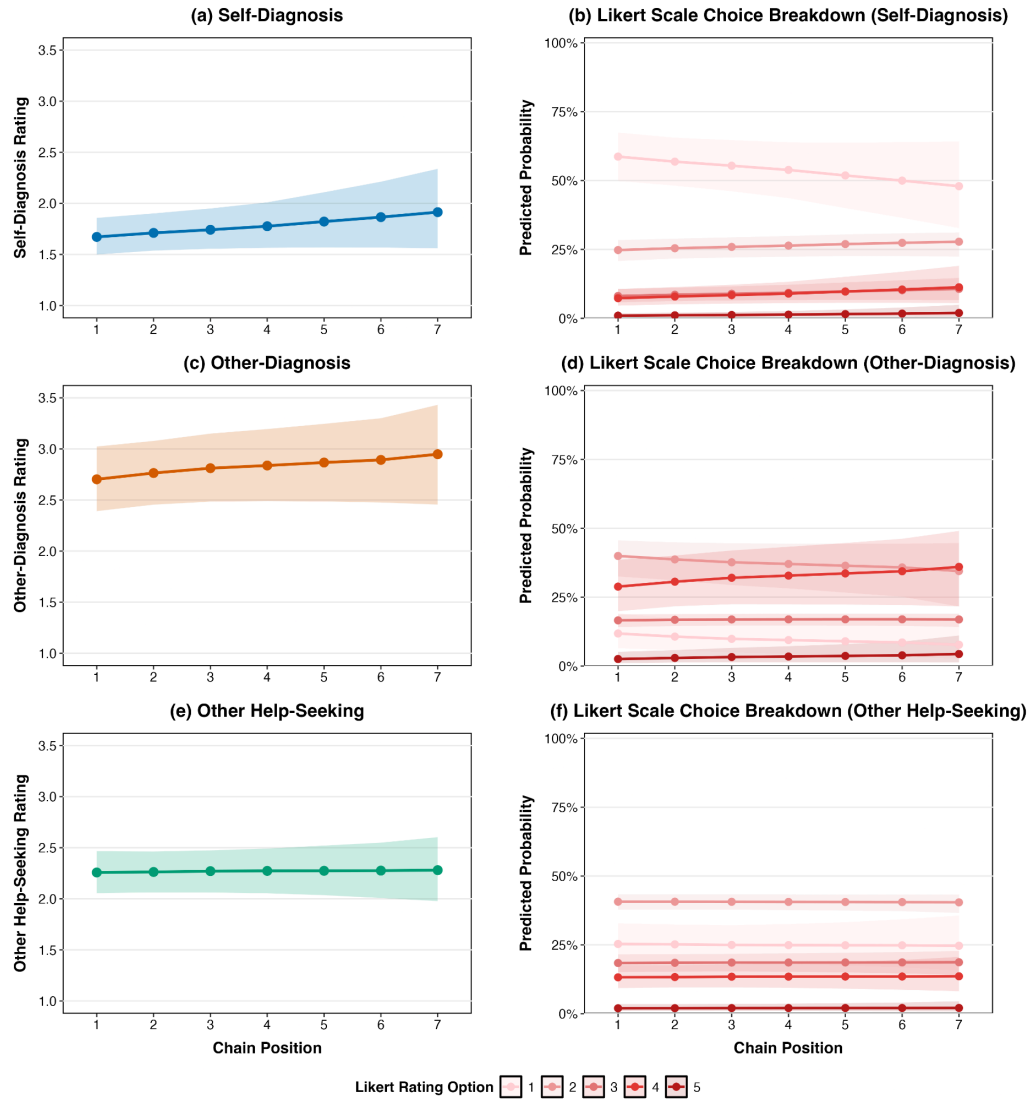
5.3 Sensitivity Analyses

This section presents Study 2 results in the study sample where participants either didn't use AI or weren't exposed to an AI-generated explanation (see Methods section on how this was accounted for via interaction models). These results were obtained after adjusting for age, clinical history, ADHD prior knowledge and gender.

The core results of Study 2 remained largely unchanged across these additional model specifications. Importantly, some of these results became stronger (see Other-Diagnosis results in **Supplementary Table 20** below).



Supplementary Figure 14: Study 2 Model Predicted Effects of Peer-to-Peer Transmission. Predictions are shown for chains not exposed to AI where age, clinical history and prior ADHD knowledge are held at their sample means and gender is at its reference category (female). Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.



Supplementary Figure 15: Study 2 Model Predicted Effects of Peer-to-Peer Transmission. Predictions are shown for chains not exposed to AI where age, clinical history and prior ADHD knowledge are held at their sample means and gender is at its reference category (female). Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position as a monotonic variable) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree). Shaded ribbons represent Bayesian 95% CrI that incorporate uncertainty around both population-level estimates and variation between transmission chains (varying or random effects). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.

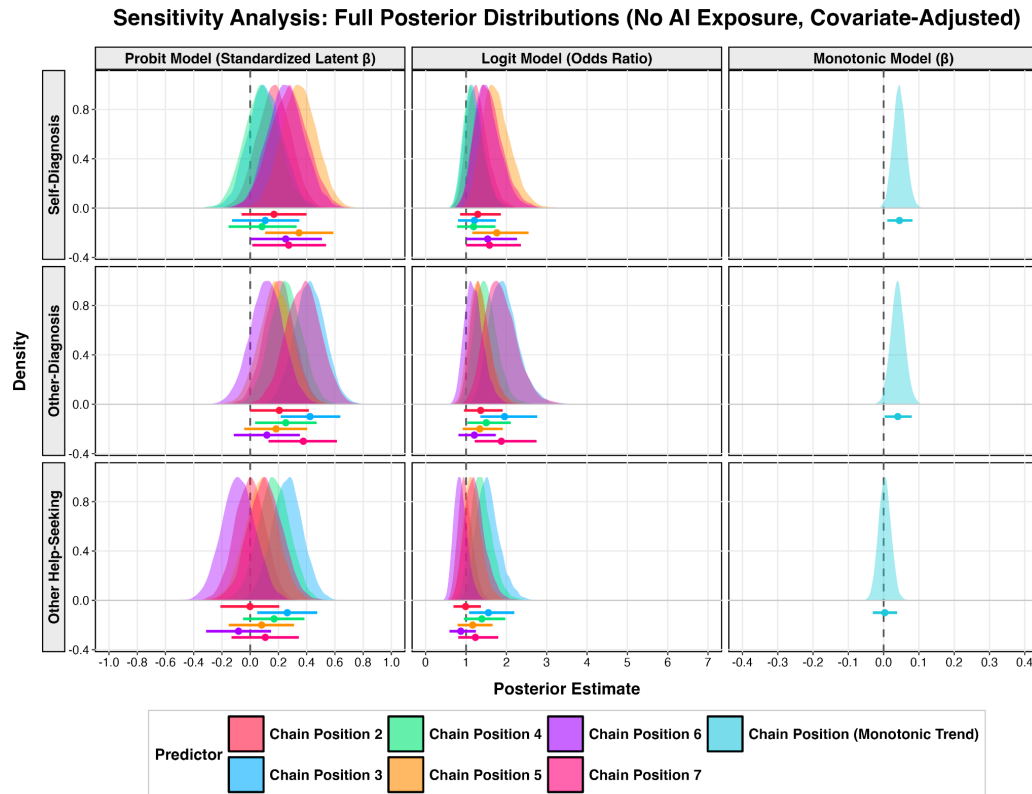
Mean [95% Credible Interval] ($\Pr(\beta > 0)$)

Predictor	Self-Diagnosis	Self-Diagnosis (Odds Ratio)	Self-Diagnosis (Monotonic)	Other-Diagnosis	Other-Diagnosis (Odds Ratio)	Other-Diagnosis (Monotonic)	Other Help-Seeking	Other Help-Seeking (Odds Ratio)	Other Help-Seeking (Monotonic)
Intercept 1	0.27 [0.10, 0.44]		0.22 [0.09, 0.36]	-1.09 [-1.25, -0.93]		-1.18 [-1.33, -1.02]	-0.60 [-0.76, -0.44]		-0.66 [-0.79, -0.53]
Intercept 2	1.02 [0.85, 1.20]		0.98 [0.84, 1.12]	0.15 [-0.00, 0.31]		0.05 [-0.09, 0.20]	0.49 [0.33, 0.64]		0.42 [0.29, 0.55]
Intercept 3	1.43 [1.25, 1.62]		1.39 [1.24, 1.54]	0.59 [0.44, 0.75]		0.49 [0.35, 0.64]	1.10 [0.93, 1.26]		1.03 [0.89, 1.16]
Intercept 4	2.38 [2.16, 2.60]		2.34 [2.15, 2.53]	2.07 [1.87, 2.27]		1.95 [1.78, 2.14]	2.13 [1.93, 2.33]		2.05 [1.88, 2.24]
Chain Position (Monotonic Trend, No AI Exposure)			0.04 [0.01, 0.08] (0.99)			0.04 [0.00, 0.08] (0.98)			0.00 [-0.03, 0.04] (0.57)
Chain Position 2 (No AI Exposure)	0.17 [-0.06, 0.40] (0.92)	1.29 [0.85, 1.86] (0.88)		0.21 [-0.00, 0.42] (0.97)	1.36 [0.95, 1.91] (0.95)		-0.00 [-0.21, 0.21] (0.50)	0.99 [0.69, 1.37] (0.44)	
Chain Position 3 (No AI Exposure)	0.11 [-0.13, 0.35] (0.81)	1.21 [0.80, 1.74] (0.80)		0.42 [0.22, 0.64] (1.00)	1.95 [1.36, 2.76] (1.00)		0.26 [0.05, 0.48] (0.99)	1.55 [1.08, 2.20] (0.99)	
Chain Position 4 (No AI Exposure)	0.08 [-0.15, 0.33] (0.75)	1.19 [0.78, 1.73] (0.77)		0.25 [0.03, 0.47] (0.99)	1.50 [1.03, 2.11] (0.98)		0.17 [-0.05, 0.38] (0.94)	1.39 [0.95, 1.97] (0.95)	
Chain Position 5 (No AI Exposure)	0.35 [0.11, 0.59] (1.00)	1.76 [1.15, 2.55] (1.00)		0.18 [-0.04, 0.40] (0.94)	1.34 [0.92, 1.91] (0.93)		0.08 [-0.15, 0.31] (0.76)	1.17 [0.79, 1.66] (0.77)	
Chain Position 6 (No AI Exposure)	0.25 [0.00, 0.51] (0.98)	1.54 [1.00, 2.26] (0.98)		0.12 [-0.12, 0.35] (0.84)	1.21 [0.81, 1.74] (0.81)		-0.08 [-0.31, 0.15] (0.24)	0.87 [0.59, 1.24] (0.20)	
Chain Position 7 (No AI Exposure)	0.27 [0.02, 0.54] (0.98)	1.58 [1.00, 2.36] (0.98)		0.38 [0.13, 0.61] (1.00)	1.87 [1.22, 2.75] (1.00)		0.11 [-0.13, 0.34] (0.81)	1.23 [0.81, 1.80] (0.82)	
Chain Position (Monotonic Trend) × Post-AI Exposure			0.01 [-0.08, 0.08] (0.56)			-0.05 [-0.13, 0.02] (0.08)			-0.03 [-0.10, 0.04] (0.17)
Chain Position 2 × Post-AI Exposure	-0.16 [-0.82, 0.48] (0.32)	0.94 [0.34, 2.09] (0.36)		-0.38 [-0.98, 0.22] (0.11)	0.71 [0.26, 1.57] (0.16)		0.16 [-0.46, 0.75] (0.70)	1.43 [0.56, 3.05] (0.72)	

Chain Position 3 × Post-AI Exposure	-0.09 [-0.70, 0.53] (0.38)	0.96 [0.37, 2.08] (0.38)		-0.40 [-0.97, 0.17] (0.08)	0.77 [0.32, 1.61] (0.20)		-0.16 [-0.72, 0.39] (0.28)	0.96 [0.40, 1.95] (0.38)	
Chain Position 4 × Post-AI Exposure	-0.03 [-0.64, 0.55] (0.46)	1.13 [0.46, 2.38] (0.53)		-0.47 [-1.02, 0.08] (0.04)	0.65 [0.28, 1.32] (0.10)		-0.17 [-0.73, 0.37] (0.27)	0.79 [0.33, 1.59] (0.22)	
Chain Position 5 × Post-AI Exposure	-0.41 [-1.00, 0.17] (0.09)	0.57 [0.24, 1.15] (0.06)		-0.02 [-0.55, 0.53] (0.48)	1.31 [0.58, 2.59] (0.70)		0.11 [-0.43, 0.64] (0.66)	1.39 [0.62, 2.70] (0.75)	
Chain Position 6 × Post-AI Exposure	-0.09 [-0.66, 0.49] (0.38)	0.99 [0.42, 1.98] (0.41)		-0.39 [-0.92, 0.15] (0.08)	0.75 [0.33, 1.46] (0.17)		-0.03 [-0.56, 0.51] (0.45)	1.15 [0.52, 2.18] (0.57)	
Chain Position 7 × Post-AI Exposure	0.03 [-0.54, 0.59] (0.54)	1.20 [0.52, 2.40] (0.61)		-0.42 [-0.95, 0.11] (0.06)	0.72 [0.32, 1.42] (0.15)		-0.13 [-0.65, 0.39] (0.32)	0.94 [0.42, 1.80] (0.36)	
Post-AI Exposure (at Chain Position 1)	-0.07 [-0.57, 0.42] (0.39)	0.92 [0.47, 1.64] (0.34)	-0.20 [-0.52, 0.17] (0.12)	0.30 [-0.17, 0.76] (0.90)	1.34 [0.70, 2.34] (0.78)	0.17 [-0.14, 0.56] (0.85)	0.09 [-0.37, 0.55] (0.65)	1.14 [0.60, 1.99] (0.61)	0.17 [-0.12, 0.49] (0.87)
Age (z)	-0.39 [-0.46, -0.33] (0.00)	0.51 [0.46, 0.57] (0.00)	-0.39 [-0.46, -0.33] (0.00)	0.00 [-0.06, 0.06] (0.51)	1.01 [0.91, 1.11] (0.54)	-0.00 [-0.06, 0.06] (0.49)	-0.04 [-0.10, 0.01] (0.06)	0.94 [0.86, 1.03] (0.10)	-0.04 [-0.10, 0.01] (0.06)
Clinical History (z)	0.14 [0.08, 0.20] (1.00)	1.29 [1.17, 1.42] (1.00)	0.14 [0.08, 0.20] (1.00)	0.02 [-0.04, 0.08] (0.74)	1.04 [0.94, 1.14] (0.77)	0.02 [-0.03, 0.08] (0.77)	0.08 [0.03, 0.14] (1.00)	1.16 [1.06, 1.27] (1.00)	0.08 [0.03, 0.14] (1.00)
Gender: Male	0.01 [-0.11, 0.13] (0.58)	1.04 [0.84, 1.27] (0.63)	0.01 [-0.11, 0.14] (0.57)	-0.04 [-0.15, 0.08] (0.27)	0.94 [0.78, 1.14] (0.26)	-0.05 [-0.16, 0.06] (0.21)	-0.03 [-0.14, 0.07] (0.27)	0.94 [0.78, 1.13] (0.25)	-0.04 [-0.15, 0.08] (0.26)
Gender: NonbinaryThirdgender	-0.17 [-1.57, 1.26] (0.40)	1.20 [0.16, 4.38] (0.42)	-0.12 [-1.54, 1.28] (0.43)	-0.00 [-1.44, 1.41] (0.50)	1.42 [0.18, 5.24] (0.49)	0.03 [-1.39, 1.43] (0.52)	0.29 [-1.11, 1.73] (0.66)	1.92 [0.23, 6.85] (0.63)	0.31 [-1.14, 1.73] (0.66)
Gender: Preferringtonsay	-0.39 [-1.32, 0.49] (0.20)	0.79 [0.18, 2.18] (0.25)	-0.39 [-1.29, 0.48] (0.19)	-0.63 [-1.47, 0.19] (0.07)	0.56 [0.12, 1.67] (0.11)	-0.64 [-1.47, 0.18] (0.06)	-0.74 [-1.59, 0.10] (0.05)	0.49 [0.12, 1.35] (0.07)	-0.76 [-1.60, 0.06] (0.04)
Prior Knowledge (z)	0.28 [0.22, 0.33] (1.00)	1.60 [1.45, 1.76] (1.00)	0.27 [0.22, 0.33] (1.00)	0.13 [0.07, 0.18] (1.00)	1.24 [1.12, 1.36] (1.00)	0.12 [0.07, 0.18] (1.00)	0.08 [0.02, 0.13] (1.00)	1.16 [1.05, 1.27] (1.00)	0.07 [0.02, 0.13] (1.00)
Simplex (× Post-AI): Step 1 → 2			0.18 [0.01, 0.55]			0.21 [0.01, 0.59]			0.18 [0.01, 0.53]
Simplex (× Post-AI): Step 2 → 3			0.17 [0.01, 0.52]			0.16 [0.01, 0.49]			0.17 [0.01, 0.52]

Simplex (× Post-AI): Step 3 → 4							0.16 [0.01, 0.49]		0.16 [0.01, 0.49]		0.16 [0.00, 0.50]
Simplex (× Post-AI): Step 4 → 5							0.16 [0.00, 0.50]		0.12 [0.00, 0.43]		0.14 [0.00, 0.47]
Simplex (× Post-AI): Step 5 → 6							0.16 [0.01, 0.51]		0.21 [0.01, 0.57]		0.19 [0.01, 0.56]
Simplex (× Post-AI): Step 6 → 7							0.17 [0.01, 0.53]		0.14 [0.00, 0.44]		0.16 [0.00, 0.49]
Simplex: Step 1 → 2							0.17 [0.02, 0.42]		0.24 [0.04, 0.51]		0.17 [0.02, 0.42]
Simplex: Step 2 → 3							0.14 [0.02, 0.35]		0.19 [0.03, 0.45]		0.17 [0.02, 0.43]
Simplex: Step 3 → 4							0.15 [0.02, 0.37]		0.11 [0.01, 0.31]		0.16 [0.02, 0.40]
Simplex: Step 4 → 5							0.19 [0.03, 0.44]		0.12 [0.02, 0.32]		0.16 [0.02, 0.41]
Simplex: Step 5 → 6							0.17 [0.03, 0.41]		0.12 [0.02, 0.31]		0.16 [0.02, 0.42]
Simplex: Step 6 → 7							0.18 [0.03, 0.41]		0.22 [0.03, 0.47]		0.18 [0.03, 0.43]
SD: Between-Chain Baseline Variation		0.08 [0.00, 0.20]		0.07 [0.00, 0.19]	0.15 [0.01, 0.26]				0.12 [0.01, 0.24]	0.08 [0.00, 0.19]	0.07 [0.00, 0.17]
SD: Between-Chain Effect Variation				0.02 [0.00, 0.06]					0.03 [0.00, 0.06]		0.01 [0.00, 0.04]

Supplementary Table 20: Adjusted Bayesian Multilevel Ordinal Regression Results of Study 2 by Outcome. The reference category was female for the Gender predictor. No AI exposure refers to regression estimates obtained from a sample that either didn’t use AI directly or weren’t exposed to any AI generated explanations. These are the main estimands of interest for the sensitivity analyses. (z) indicates that variables were standardised. $\Pr(\beta > 0)$ denotes probability of a positive association and is only provided for the main predictors. Likert scale cutpoint and varying intercept terms (i.e. SD) for the logit models aren’t presented here owing to difference in scale units of such models (i.e. log-odds).

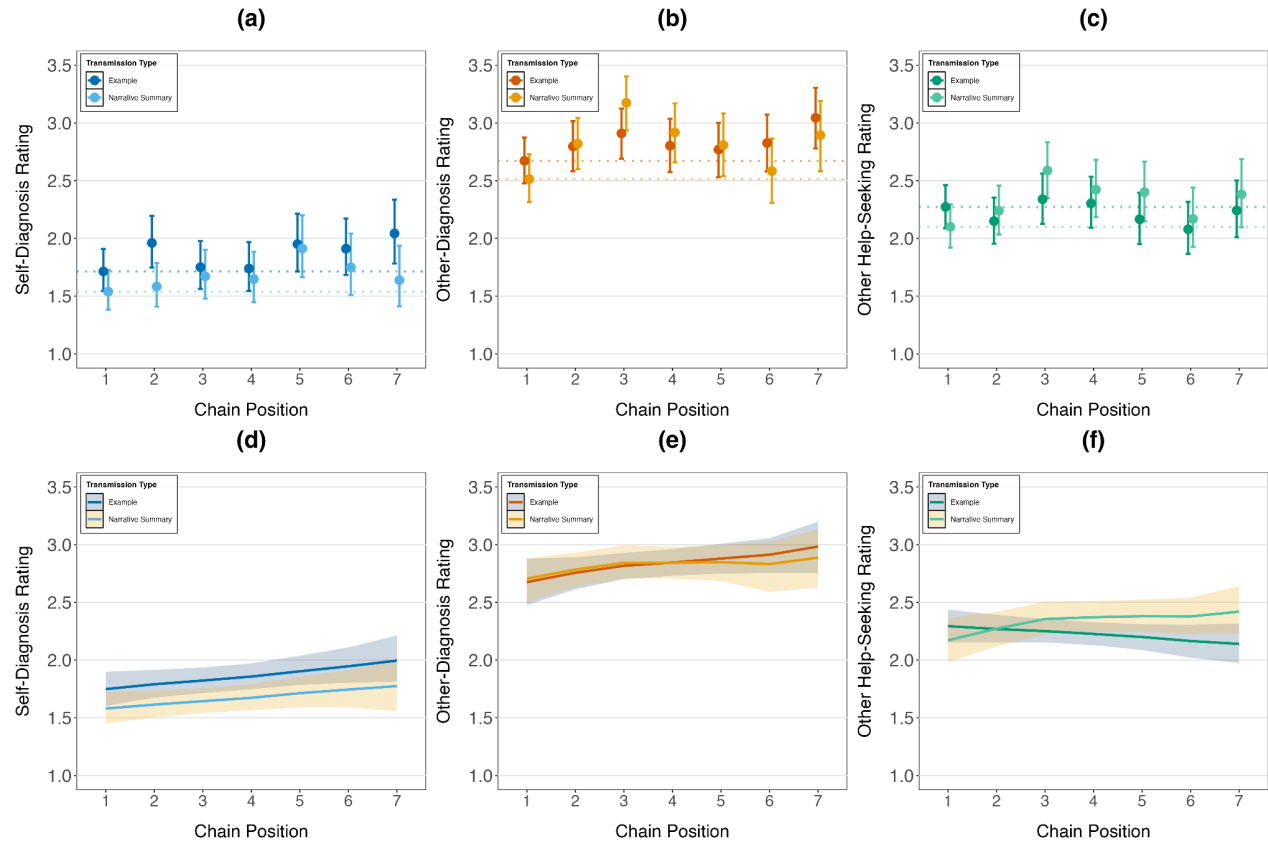


Supplementary Figure 16: Full Posterior Distributions of Study 2 Effects of Transmission on Self-Diagnosis, Other-Diagnosis and Other Help-Seeking across probit, logit and monotonic model variants. Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean.

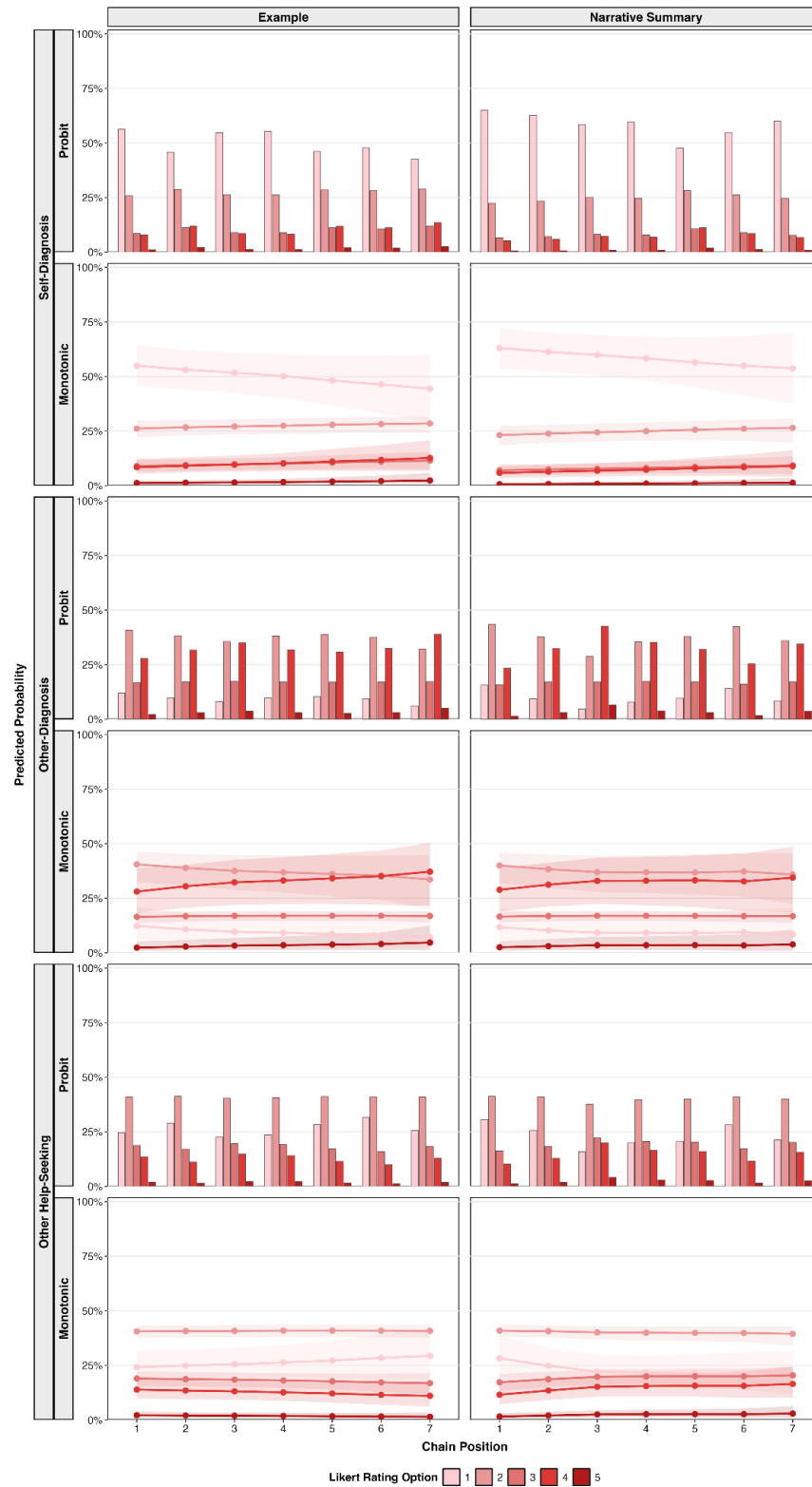
5.3.1 Transmission Format Moderation Results

The following results became uncertain while conducting sensitivity analyses:

1. Relative to the beginning of the chain where participants were exposed to the ADHD DSM-5-TR criteria, transmission format moderated transmission effect on self-diagnosis after seven rounds of transmission (*Self-diagnosis interaction - CP7*: -0.23 latent SD, 95% CrI [-0.63, 0.17], $P_{>0} = 13\%$). This changed to (*Self-diagnosis interaction - CP7*: -0.22 latent SD, 95% CrI [-0.72, 0.29], $P_{>0} = 20\%$; see **Supplementary Table 21** and **SFig 19**).
2. There was uncertain, but suggestive evidence that chains where participants transmitted examples showed stronger increases in other-diagnosing tendencies than chains where narrative summaries were transmitted (*Other-diagnosis monotonic interaction*: -0.03 latent SD, 95% CrI [-0.09, 0.05], $P_{>0} = 18\%$). This changed to (*Other-diagnosis monotonic interaction*: -0.02 latent SD, 95% CrI [-0.09, 0.08], $P_{>0} = 30\%$; see **Supplementary Table 21** and **SFig 19**).



Supplementary Figure 17: Study 2 Interaction Model Predicted Effects. Predictions are shown for chains not exposed to AI where age, clinical history and prior ADHD knowledge are held at their sample means and gender is at its reference category (female). Panels (a) - (c) show predicted interaction effects of transmission format and peer-to-peer transmission (i.e. chain position) on Self-Diagnosis (a), Other-Diagnosis (b) and Other Help-Seeking (c) when chain position is modelled as a factor/categorical predictor. Panels (d) - (f) display the same effects when chain position is modelled as a monotonic predictor to test the interaction effect over the course of transmission. All dependent variables here were Likert scale items ranging from 1 = Strongly Disagree to 5 = Strongly Agree. NB: Y axes for Panels (a) - (f) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.



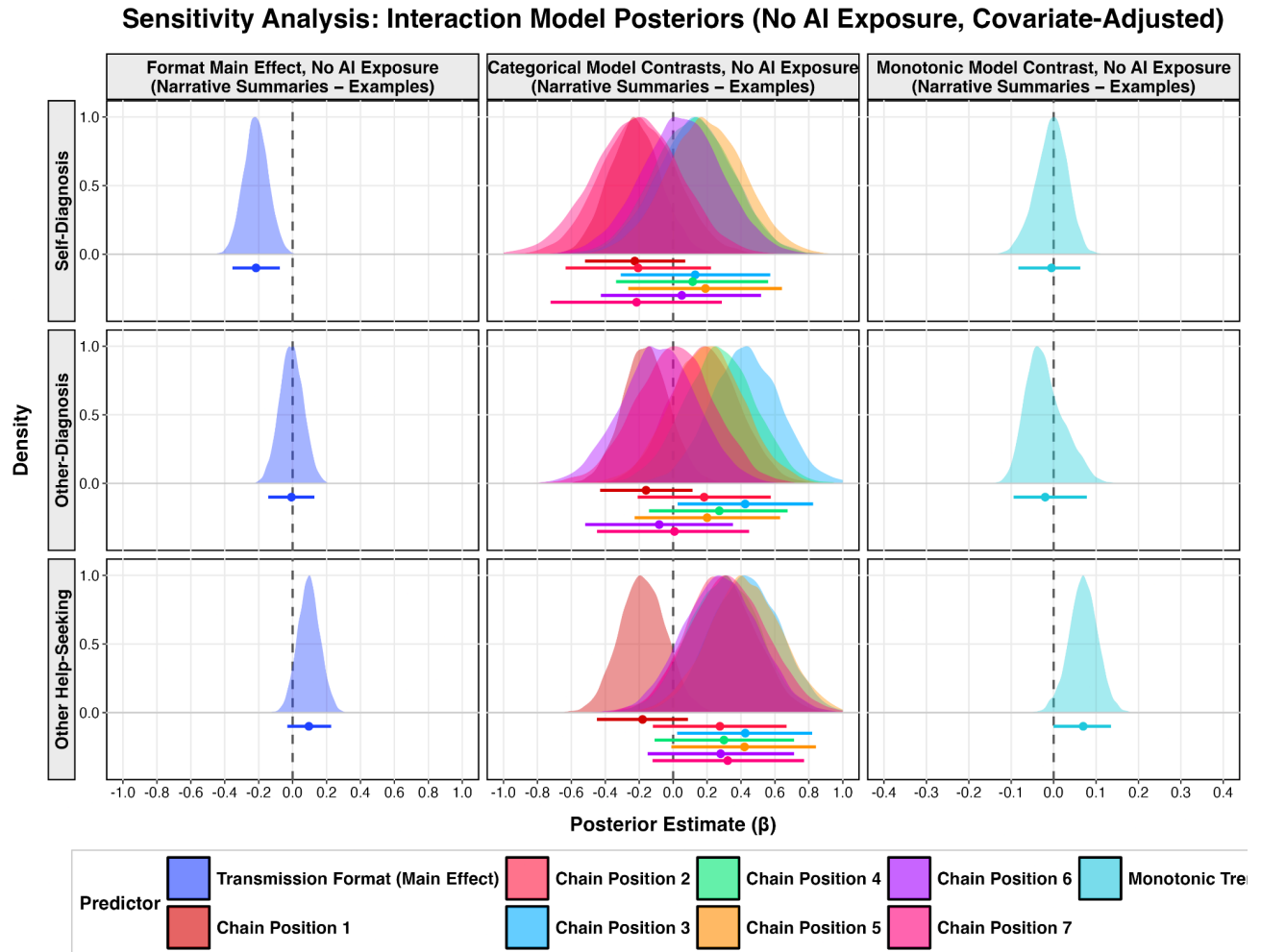
Supplementary Figure 18: Study 2 Model Predicted Effects of Peer-to-Peer Transmission by Transmission Format. Predictions are shown for chains not exposed to AI where age, clinical history and prior ADHD knowledge are held at their sample means and gender is at its reference category (female). All panels show the predicted probabilities of choosing a Likert

scale option (1 = Strongly Disagree to 5 = Strongly Agree) for a particular outcome, either as bar plots (Categorical Probit models) or continuous lines for choosing each option over the course of transmission (Monotonic Trend Models).

Predictor	Mean [95% Credible Interval] (Pr($\beta > 0$))					
	Self-Diagnosis	Self-Diagnosis (Monotonic)	Other-Diagnosis	Other-Diagnosis (Monotonic)	Other Help-Seeking	Other Help-Seeking (Monotonic)
Transmission Type (Main Effect: Narrative Summaries - Examples, No AI Exposure)	-0.22 [-0.36, -0.07] (0.00)	-0.21 [-0.36, -0.07] (0.00)	-0.01 [-0.14, 0.13] (0.46)	-0.01 [-0.14, 0.12] (0.44)	0.10 [-0.03, 0.23] (0.93)	0.09 [-0.03, 0.22] (0.93)
Chain Position 2 × Transmission Format	-0.21 [-0.63, 0.22] (0.17)		0.18 [-0.21, 0.58] (0.82)		0.28 [-0.12, 0.67] (0.92)	
Chain Position 3 × Transmission Format	0.13 [-0.31, 0.57] (0.72)		0.42 [0.03, 0.82] (0.98)		0.43 [0.02, 0.82] (0.98)	
Chain Position 4 × Transmission Format	0.12 [-0.34, 0.56] (0.69)		0.27 [-0.14, 0.67] (0.90)		0.30 [-0.11, 0.71] (0.92)	
Chain Position 5 × Transmission Format	0.19 [-0.26, 0.64] (0.80)		0.20 [-0.23, 0.63] (0.83)		0.42 [-0.01, 0.84] (0.97)	
Chain Position 6 × Transmission Format	0.05 [-0.43, 0.52] (0.58)		-0.08 [-0.52, 0.35] (0.36)		0.28 [-0.15, 0.71] (0.90)	
Chain Position 7 × Transmission Format	-0.22 [-0.72, 0.29] (0.20)		0.01 [-0.45, 0.45] (0.52)		0.32 [-0.12, 0.77] (0.92)	
Chain Position 1 × Transmission Format	-0.23 [-0.52, 0.07] (0.07)		-0.16 [-0.43, 0.11] (0.12)		-0.18 [-0.45, 0.09] (0.09)	
Chain Position 2 × Transmission Format × Post-AI Exposure	0.07 [-0.90, 1.05] (0.55)		-0.47 [-1.39, 0.43] (0.16)		0.24 [-0.68, 1.14] (0.70)	
Chain Position 3 × Transmission Format × Post-AI Exposure	0.16 [-0.78, 1.10] (0.64)		-0.51 [-1.38, 0.35] (0.12)		-0.27 [-1.12, 0.58] (0.27)	

Chain Position 4 × Transmission Format × Post-AI Exposure	0.28 [-0.57, 1.15] (0.74)		-0.31 [-1.17, 0.53] (0.23)		0.05 [-0.78, 0.86] (0.54)	
Chain Position 5 × Transmission Format × Post-AI Exposure	0.12 [-0.71, 0.95] (0.61)		-0.40 [-1.20, 0.41] (0.16)		-0.48 [-1.26, 0.31] (0.11)	
Chain Position 6 × Transmission Format × Post-AI Exposure	0.35 [-0.49, 1.18] (0.80)		-0.18 [-1.00, 0.60] (0.33)		-0.02 [-0.80, 0.76] (0.48)	
Chain Position 7 × Transmission Format × Post-AI Exposure	-0.12 [-0.96, 0.70] (0.39)		-0.05 [-0.84, 0.73] (0.45)		-0.06 [-0.84, 0.69] (0.44)	
Chain Position 1 × Transmission Format × Post-AI Exposure	-0.07 [-0.72, 0.58] (0.42)		0.31 [-0.32, 0.95] (0.84)		0.01 [-0.60, 0.63] (0.51)	
Chain Position (Monotonic Trend) × Transmission Format		-0.01 [-0.08, 0.06] (0.46)		-0.02 [-0.09, 0.08] (0.30)		0.07 [0.00, 0.13] (0.97)
Chain Position (Monotonic Trend) × Transmission Format × Post-AI Exposure		-0.03 [-0.17, 0.12] (0.36)		0.01 [-0.15, 0.15] (0.55)		-0.06 [-0.19, 0.06] (0.16)
SD: Between-Chain Baseline Variation	0.07 [0.00, 0.19]	0.07 [0.00, 0.18]	0.16 [0.02, 0.28]	0.12 [0.01, 0.25]	0.08 [0.00, 0.19]	0.07 [0.00, 0.18]
SD: Between-Chain Effect Variation		0.02 [0.00, 0.05]		0.03 [0.00, 0.06]		0.01 [0.00, 0.04]

Supplementary Table 21: Adjusted Bayesian Multilevel Ordinal Moderation Regression Results of Study 2 by Outcome. Chain position was treated as a categorical and monotonic variable respectively. For categorical models, Chain Position N x Transmission Format represents the difference-in-differences between narrative summaries and examples format at a specific chain position, relative to chain position 1 in the study sample where there was no AI usage or exposure (Post-AI Exposure = 0). For monotonic models, Chain Position (Monotonic Trend) x Transmission Format represents the difference in trend increases in the pertinent outcome between narrative summaries and examples format where there was no AI usage or exposure (Post-AI Exposure = 0). The format main effect was calculated from the interaction model using the `marginalEffects` package. NB: Supplementary Table display is limited to selected estimands of interest to facilitate easy comparison with main analyses results. For further information on other estimands (e.g. simplex terms), please refer to the R scripts in the project repo.



Supplementary Figure 19: Full Posterior Distributions of Format Moderation Effects in Study 2 across probit and monotonic model variants. For probit categorical models, each Chain Position Posterior represents the difference-in-differences in the relevant outcome between narrative summaries and examples, relative to Chain Position 1 when there was no AI usage or exposure (*Post-AI Exposure* = 0). Positive values on the x-axis mean that narrative summaries have a stronger impact. For monotonic models, the posterior represents the difference-in-differences in trend increases in the pertinent outcome between narrative summaries and examples format when there was no AI usage or exposure in the sample (*Post-AI Exposure* = 0).

Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean.

6. Study 3 Results

6.1 Descriptive Statistics

Chain position	Observations	Age (Mean)	Female (%)	Clinical History (%)				ADHD Prior Knowledge (%)		
				MH Medication	MH Treatment	NHS MH Support	Past MH Diagnosis	Friend Support	Waitlist Support	Social Media
1	220	41.91	61.82	0.00	0.00	0.45	9.09	17.27	9.09	6.36
2	220	41.58	62.27	0.00	0.00	3.18	8.18	16.36	12.27	11.36
3	220	42.03	60.45	0.45	0.91	2.27	12.73	11.36	13.64	11.36
4	220	42.11	59.09	0.91	0.45	3.18	6.82	18.64	12.73	10.45
5	220	40.51	58.18	1.36	1.82	5.45	11.36	14.55	12.73	16.36
6	220	34.20	53.64	0.91	0.91	3.64	5.00	28.18	16.36	25.45
7	220	35.09	55.45	0.45	1.36	3.18	8.64	15.91	15.91	18.64

Supplementary Table 22: Sample characteristics, Clinical history and ADHD Prior Knowledge by Chain Position (Study 3). Percentages reflect % of participants answering “Yes” to a question.

Transmission Motivation	Chain position	Observations	Age (Mean)	Female (%)	Clinical History (%)				ADHD Prior Knowledge (%)		
					MH Medication	MH Treatment	NHS MH Support	Past MH Diagnosis	Friend Support	Waitlist Support	Social Media
Accuracy	1	110	41.80	64.55	0.00	0.00	0.91	6.36	18.18	6.36	7.27
Accuracy	2	110	40.31	61.82	0.00	0.00	2.73	9.09	19.09	12.73	16.36
Accuracy	3	110	41.47	60.91	0.91	0.91	2.73	17.27	13.64	11.82	8.18
Accuracy	4	110	43.28	56.36	0.00	0.00	1.82	8.18	23.64	11.82	11.82
Accuracy	5	110	41.35	51.82	1.82	1.82	4.55	10.91	15.45	14.55	15.45
Accuracy	6	110	34.85	50.91	0.00	0.00	2.73	3.64	33.64	20.00	23.64
Accuracy	7	110	35.69	58.18	0.91	0.91	2.73	10.91	17.27	19.09	24.55
Virality	1	110	42.02	59.09	0.00	0.00	0.00	11.82	16.36	11.82	5.45

Virality	2	110	42.85	62.73	0.00	0.00	3.64	7.27	13.64	11.82	6.36
Virality	3	110	42.59	60.00	0.00	0.91	1.82	8.18	9.09	15.45	14.55
Virality	4	110	40.95	61.82	1.82	0.91	4.55	5.45	13.64	13.64	9.09
Virality	5	110	39.67	64.55	0.91	1.82	6.36	11.82	13.64	10.91	17.27
Virality	6	110	33.56	56.36	1.82	1.82	4.55	6.36	22.73	12.73	27.27
Virality	7	110	34.49	52.73	0.00	1.82	3.64	6.36	14.55	12.73	12.73

***Supplementary Table 23:** Sample characteristics, Clinical history and ADHD Prior Knowledge by Chain Position and Transmission Motivation (Study 3).*

AI Exposure Classification	Direct AI User	AI Usage in Transmission Chain	N	% of Total Sample
Pure non-AI observations	No	0	954	61.95
Indirect Exposure to an AI Explanation generated by previous participant in the chain	No	1	371	24.09
Participants who used AI to generate their explanation, but aren't considered to be exposed to AI	Yes	0	0	0.00
Participants who used AI to generate their Explanation	Yes	1	215	13.96

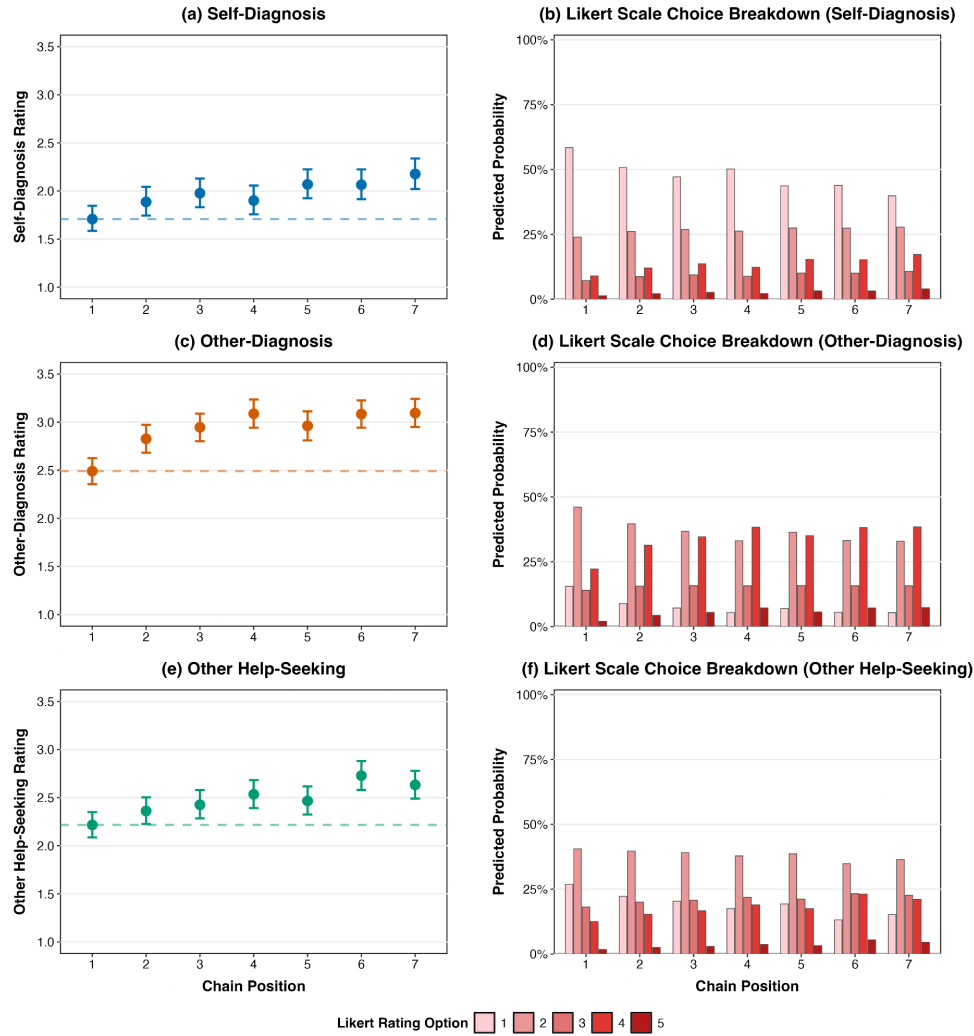
***Supplementary Table 24:** % of AI-usage in Study 3. Direct AI user = Participant using AI to generate their ADHD Explanation. AI Usage in Transmission chain = Chain having other participants who have used AI.*

13.96% of participants in Study 3 were detected using AI to generate their written ADHD explanations while 24.09% of this sample were in transmission chains where a previous participant had used AI.

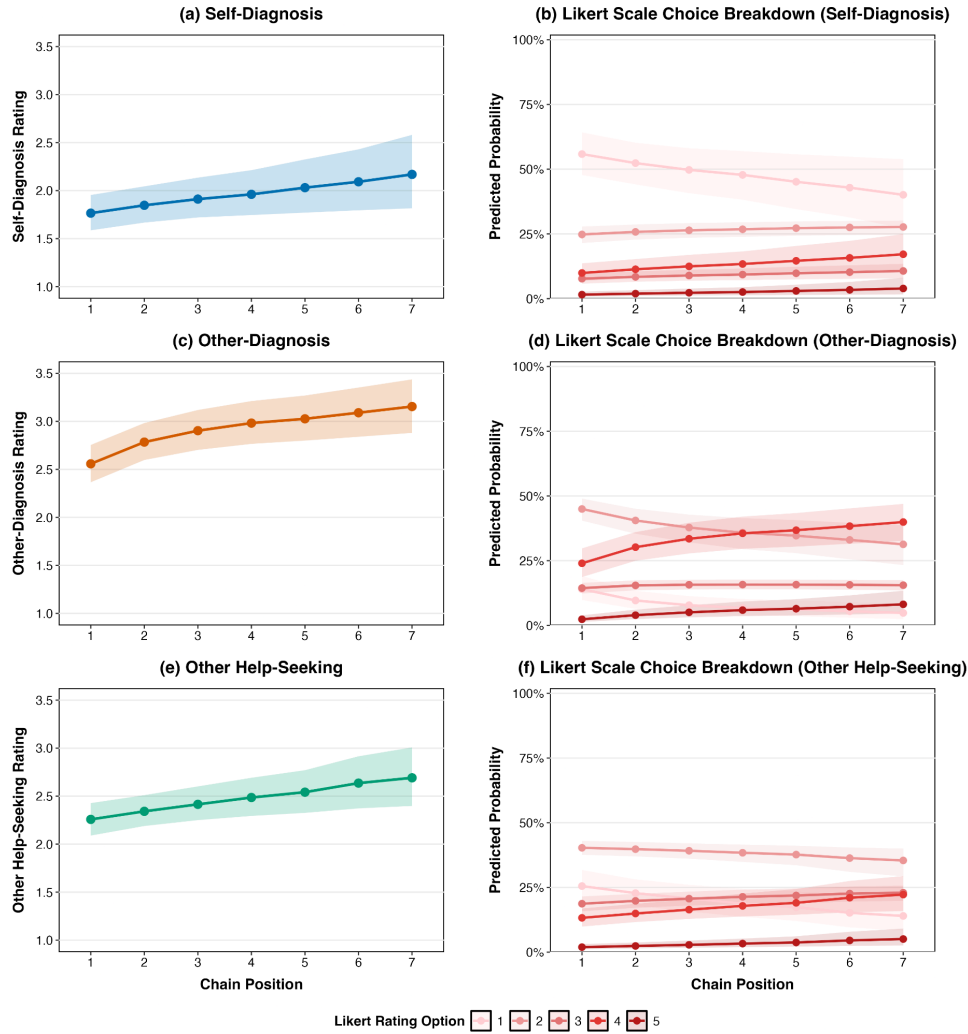
6.2 Main Analyses

Relative to initial participants exposed to the ADHD DSM-5-TR criteria, we found strong evidence that participants at the end of seven rounds of transmission had 112% higher odds of diagnosing themselves and 173% higher odds of diagnosing another subclinical individual with ADHD (see **Supplementary Table 25, SFig 20, 21 and 22 below**; **Self-diagnosis - CP7**: 2.12 OR, 95% CrI [1.49, 2.96], $P_{>0} = 100\%$; 0.47 latent SD, 95% CrI [0.26, 0.68], $P_{>0} = 100\%$; **Other-diagnosis - CP7**: 2.73 OR, 95% CrI [1.92, 3.78], $P_{>0} = 100\%$; 0.59 latent SD, 95% CrI [0.39, 0.80], $P_{>0} = 100\%$). Importantly, trend analyses showed strong evidence for a monotonic increase in self- and other-diagnosis over the course of transmission. (see **SFig 21 and 22**; **Self-diagnosis** - monotonic latent sd change = 0.07, 95% CrI [0.04, 0.10], $P_{>0} = 100\%$; **Other-diagnosis** - monotonic latent sd change = 0.10, 95% CrI [0.07, 0.13], $P_{>0} = 100\%$). For self-diagnosis, this effect was primarily driven by transmission reducing participants' extreme skepticism (i.e. choosing 1 = Strongly Disagree; see **SFig 21b**) over time whereas the other-diagnosis effect was driven by transmission increasing participants' moderate endorsement (i.e. choosing 4 = Somewhat Agree; see **SFig 21d**).

For other help-seeking, we found strong evidence that participants at the end of seven rounds of transmission had 99% higher odds of recommending another subclinical individual to seek professional help for ADHD relative to initial participants exposed to the ADHD DSM-5-TR criteria (see **Supplementary Table 25, SFig 20, 21 and 22 below**; **Other Help-Seeking - CP7**: 1.99 OR, 95% CrI [1.41, 2.73], $P_{>0} = 100\%$; 0.41 latent SD, 95% CrI [0.22, 0.61], $P_{>0} = 100\%$). Further, trend analyses showed strong evidence for a monotonic increase in other help-seeking over the course of transmission (see **SFig 21 and 22**; **Other Help-Seeking** - monotonic latent sd change = 0.07, 95% CrI [0.04, 0.10], $P_{>0} = 100\%$). This effect was driven by transmission increasing participants' moderate endorsement (i.e choosing 3 = neither agree nor disagree and 4 = Somewhat Agree; see **SFig 21f**)



Supplementary Figure 20: Study 3 Model Predicted Effects of Peer-to-Peer Transmission. Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.



Supplementary Figure 21: Study 3 Model Predicted Effects of Peer-to-Peer Transmission. Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position as a monotonic variable) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree).

Shaded ribbons represent Bayesian 95% CrI that incorporate uncertainty around both population-level estimates and variation between transmission chains (varying or random effects).. NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.

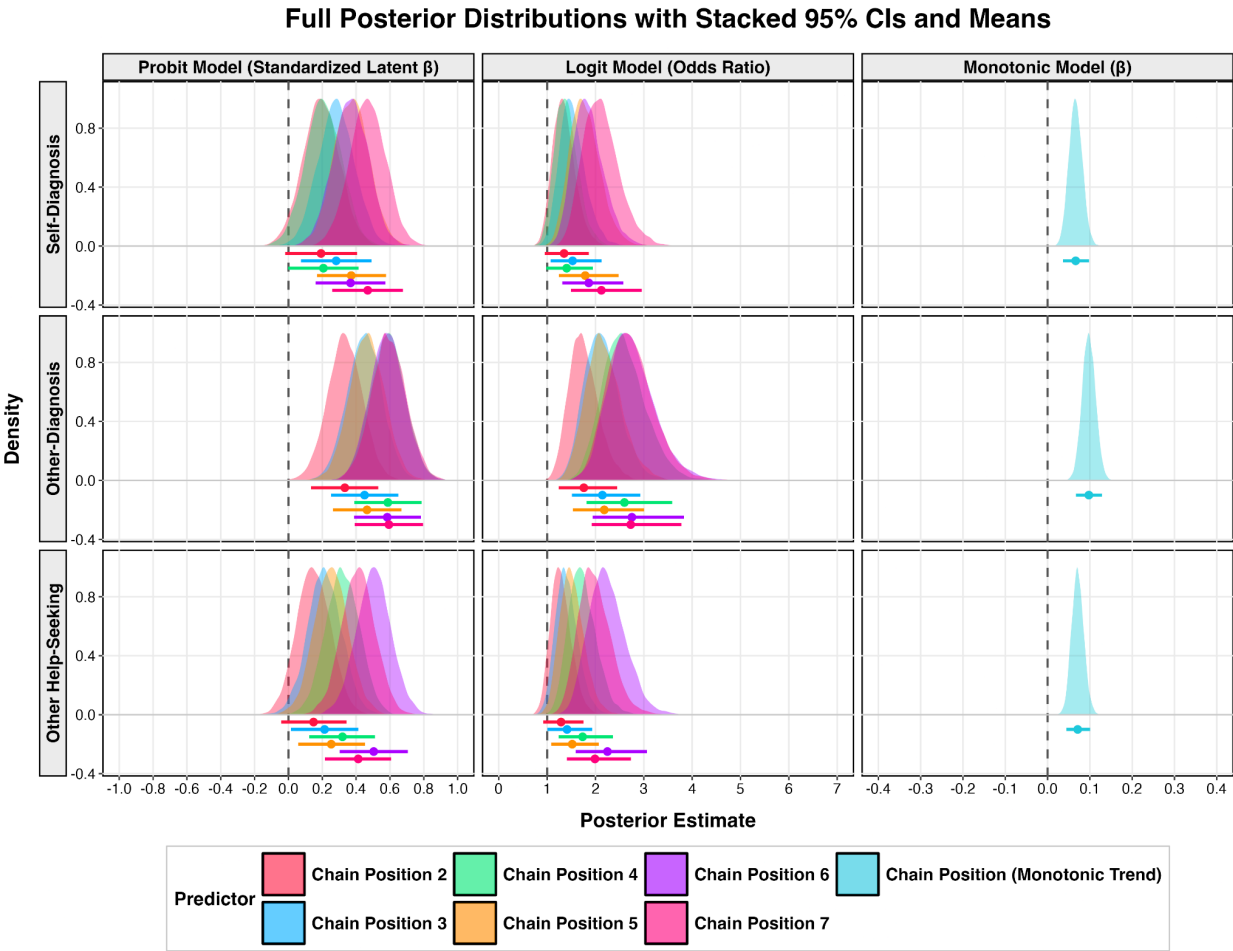
Mean [95% Credible Interval] ($\Pr(\beta > 0)$)

Predictor	Self-Diagnosis	Self-Diagnosis (Odds Ratio)	Self-Diagnosis (Monotonic)	Other-Diagnosis	Other-Diagnosis (Odds Ratio)	Other-Diagnosis (Monotonic)	Other Help-Seeking	Other Help-Seeking (Odds Ratio)	Other Help-Seeking (Monotonic)
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Supplementary Information									
Intercept 1	0.21 [0.06, 0.36]		0.15 [0.03, 0.29]		-1.01 [-1.17, -0.86]		-1.08 [-1.23, -0.92]		-0.62 [-0.76, -0.47]
Intercept 2	0.93 [0.77, 1.09]		0.87 [0.74, 1.01]		0.30 [0.16, 0.44]		0.23 [0.09, 0.38]		0.45 [0.31, 0.60]
Intercept 3	1.26 [1.10, 1.42]		1.20 [1.07, 1.35]		0.70 [0.55, 0.85]		0.63 [0.48, 0.78]		1.07 [0.92, 1.22]
Intercept 4	2.22 [2.02, 2.42]		2.16 [1.98, 2.34]		2.05 [1.88, 2.22]		1.98 [1.81, 2.16]		2.11 [1.93, 2.28]
Chain Position 2	0.19 [-0.02, 0.41] (0.96)	1.35 [0.95, 1.86] (0.95)	0.33 [0.13, 0.53] (1.00)		1.76 [1.24, 2.45] (1.00)		0.15 [-0.04, 0.34] (0.93)	1.29 [0.92, 1.75] (0.92)	
Chain Position 3	0.28 [0.07, 0.49] (1.00)	1.53 [1.07, 2.13] (0.99)	0.45 [0.25, 0.65] (1.00)		2.14 [1.51, 2.93] (1.00)		0.21 [0.02, 0.41] (0.98)	1.41 [1.00, 1.93] (0.98)	
Chain Position 4	0.21 [-0.00, 0.41] (0.97)	1.40 [0.98, 1.95] (0.97)	0.59 [0.39, 0.79] (1.00)		2.60 [1.82, 3.59] (1.00)		0.32 [0.12, 0.51] (1.00)	1.73 [1.24, 2.36] (1.00)	
Chain Position 5	0.37 [0.17, 0.58] (1.00)	1.79 [1.25, 2.48] (1.00)	0.46 [0.26, 0.67] (1.00)		2.18 [1.53, 3.01] (1.00)		0.25 [0.06, 0.45] (0.99)	1.52 [1.08, 2.07] (0.99)	
Chain Position 6	0.37 [0.16, 0.57] (1.00)	1.86 [1.31, 2.58] (1.00)	0.58 [0.39, 0.78] (1.00)		2.75 [1.94, 3.83] (1.00)		0.50 [0.30, 0.71] (1.00)	2.25 [1.59, 3.07] (1.00)	
Chain Position 7	0.47 [0.26, 0.68] (1.00)	2.12 [1.49, 2.96] (1.00)	0.59 [0.39, 0.80] (1.00)		2.73 [1.92, 3.78] (1.00)		0.41 [0.22, 0.61] (1.00)	1.99 [1.41, 2.73] (1.00)	
Chain Position (Monotonic Trend)			0.07 [0.04, 0.10] (1.00)				0.10 [0.07, 0.13] (1.00)		0.07 [0.04, 0.10] (1.00)
Simplex: Step 1 → 2			0.22 [0.04, 0.46]				0.38 [0.14, 0.59]		0.20 [0.04, 0.42]
Simplex: Step 2 → 3			0.17 [0.03, 0.39]				0.20 [0.04, 0.42]		0.17 [0.03, 0.38]
Simplex: Step 3 → 4			0.13 [0.02, 0.32]				0.13 [0.02, 0.31]		0.16 [0.03, 0.37]
Simplex: Step 4 → 5			0.17 [0.03, 0.39]				0.08 [0.01, 0.20]		0.13 [0.02, 0.32]
Simplex: Step 5 → 6			0.15 [0.02, 0.35]				0.10 [0.02, 0.25]		0.21 [0.04, 0.43]
Simplex: Step 6 → 7			0.17 [0.03, 0.38]				0.11 [0.02, 0.24]		0.12 [0.02, 0.30]

SD: Between-Chain Baseline Variation	0.06 [0.00, 0.17]	0.06 [0.00, 0.17]	0.06 [0.00, 0.15]	0.05 [0.00, 0.15]	0.05 [0.00, 0.14]	0.05 [0.00, 0.14]
SD: Between-Chain Effect Variation		0.02 [0.00, 0.05]		0.01 [0.00, 0.04]		0.02 [0.00, 0.04]

Supplementary Table 25: Bayesian Multilevel Ordinal Regression Results of Study 3 by Outcome. $Pr(\beta > 0)$ denotes probability of a positive association and is only provided for the main predictors. Likert scale cutpoint and varying intercept terms (i.e. SD) for the logit models aren't presented here owing to difference in scale units of such models (i.e. log-odds).



Supplementary Figure 22: Full Posterior Distributions of Study 3 Effects of Transmission on Self-Diagnosis, Other-Diagnosis and Other Help-Seeking across probit, logit and monotonic model variants. For probit and logit categorical models, each Chain Position term is a contrast from Chain Position 1. For monotonic models, the posterior represents the

average increase in the pertinent outcome over the course of transmission (i.e. across all chain positions). Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean.

6.2.1 Transmission Motivation Moderation Results

Predictor	Mean [95% Credible Interval] (Pr($\beta > 0$))					
	Self-Diagnosis	Self-Diagnosis (Monotonic)	Other-Diagnosis	Other-Diagnosis (Monotonic)	Other Help-Seeking	Other Help-Seeking (Monotonic)
Transmission Type (Main Effect: Virality - Accuracy)	-0.01 [-0.12, 0.10] (0.43)	-0.01 [-0.13, 0.10] (0.42)	0.00 [-0.10, 0.11] (0.53)	0.00 [-0.10, 0.12] (0.53)	0.07 [-0.04, 0.18] (0.89)	0.07 [-0.04, 0.17] (0.89)
Chain Position 2 × Transmission Motivation	-0.08 [-0.47, 0.32] (0.35)		0.14 [-0.23, 0.51] (0.76)		0.14 [-0.23, 0.51] (0.76)	
Chain Position 3 × Transmission Motivation	-0.04 [-0.43, 0.36] (0.43)		0.08 [-0.31, 0.46] (0.66)		-0.06 [-0.43, 0.32] (0.38)	
Chain Position 4 × Transmission Motivation	0.20 [-0.19, 0.60] (0.84)		-0.12 [-0.49, 0.25] (0.27)		0.12 [-0.25, 0.49] (0.73)	
Chain Position 5 × Transmission Motivation	0.08 [-0.31, 0.48] (0.65)		-0.04 [-0.42, 0.33] (0.41)		-0.01 [-0.40, 0.35] (0.47)	
Chain Position 6 × Transmission Motivation	0.19 [-0.19, 0.59] (0.84)		0.26 [-0.12, 0.64] (0.91)		0.11 [-0.27, 0.48] (0.71)	
Chain Position 7 × Transmission Motivation	0.13 [-0.26, 0.52] (0.75)		0.06 [-0.32, 0.43] (0.63)		0.15 [-0.22, 0.52] (0.79)	
Chain Position 1 × Transmission Motivation	-0.08 [-0.36, 0.19] (0.29)		-0.05 [-0.31, 0.21] (0.36)		0.00 [-0.25, 0.27] (0.52)	
Chain Position (Monotonic Trend) × Transmission Motivation		0.04 [-0.02, 0.09] (0.90)		0.00 [-0.05, 0.06] (0.50)		0.02 [-0.03, 0.07] (0.73)
SD: Between-Chain Baseline Variation	0.07 [0.00, 0.18]	0.06 [0.00, 0.17]	0.06 [0.00, 0.15]	0.05 [0.00, 0.15]	0.05 [0.00, 0.14]	0.05 [0.00, 0.14]

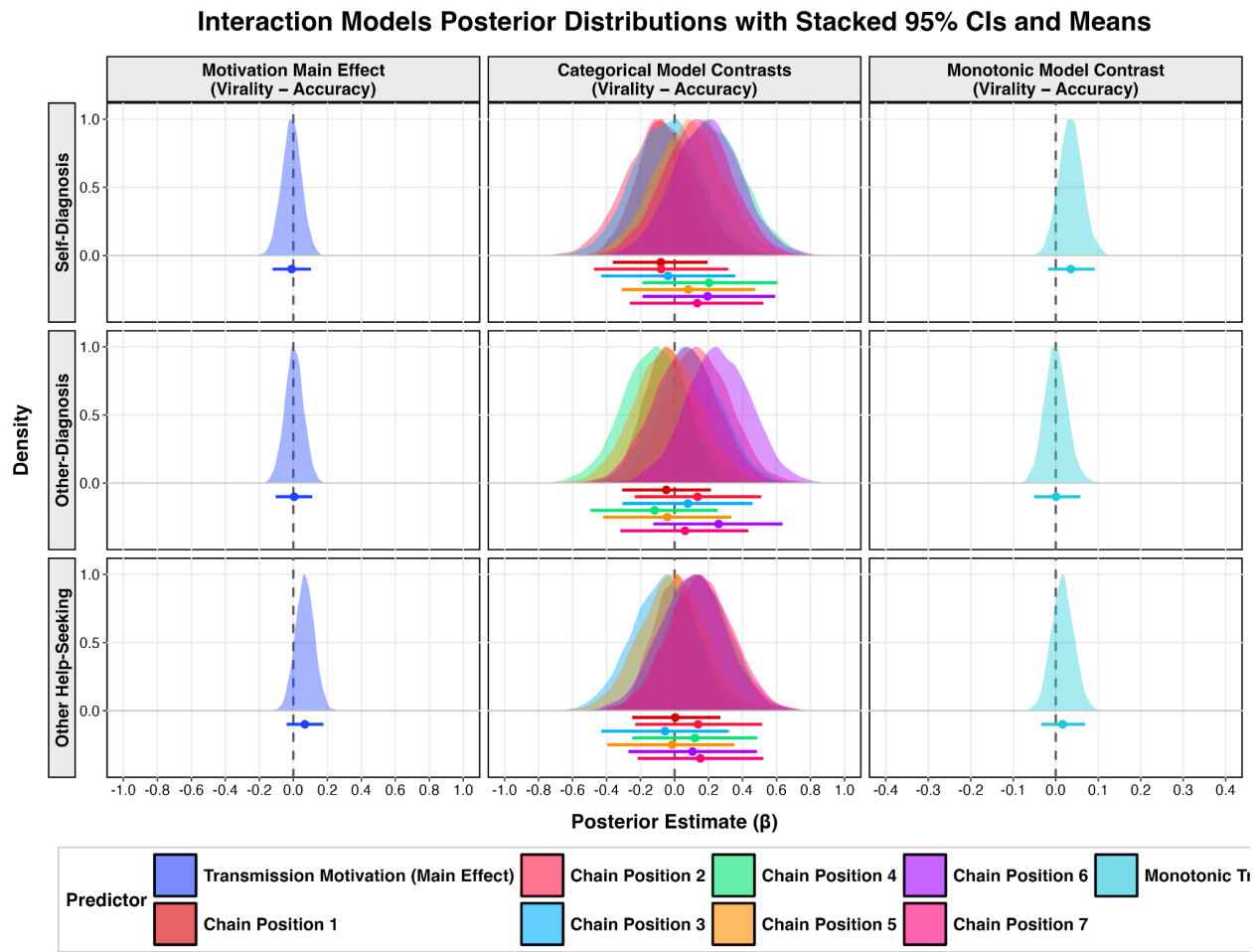
SD: Between-Chain
Effect Variation

0.02 [0.00, 0.05]

0.01 [0.00, 0.04]

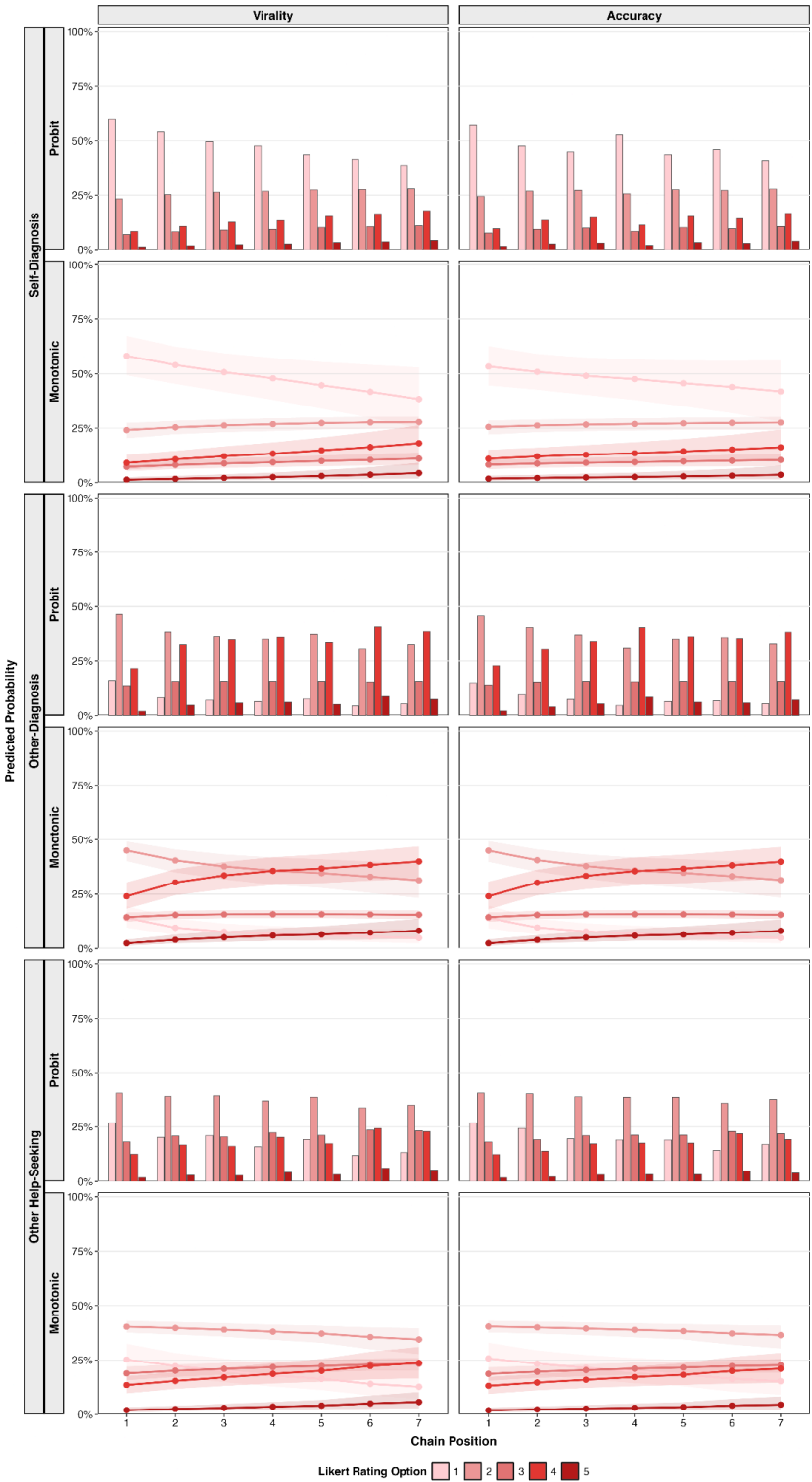
0.02 [0.00, 0.04]

Supplementary Table 26: Bayesian Multilevel Ordinal Moderation Regression Results of Study 3 by Outcome. Chain position was treated as a categorical and monotonic variable respectively. For categorical models, Chain Position $N \times$ Transmission Motivation represents the difference-in-differences between accuracy and virality motivations at a specific chain position, relative to chain position 1. For monotonic models, Chain Position (Monotonic Trend) $\times$ Transmission Motivation represents the difference in trend increases in the pertinent outcome between virality and accuracy motivations. The motivation main effect was calculated from the interaction model using the `marginalEffects` package. NB: Supplementary Table display is limited to selected estimands of interest to prevent overcrowding of core results. For further information on other estimands (e.g. simplex terms), please refer to the R scripts.



Supplementary Figure 23: Full Posterior Distributions of Motivation Moderation Effects in Study 3 across probit and monotonic model variants. For probit categorical models, each Chain Position Posterior represents the difference-in-differences in the relevant outcome between accuracy and virality chains, relative to Chain Position 1. Positive values on the x-axis mean that virality has a stronger impact. For monotonic models, the posterior represents the difference-in-differences in trend

increases in the pertinent outcome between accurate and viral explanations. Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean

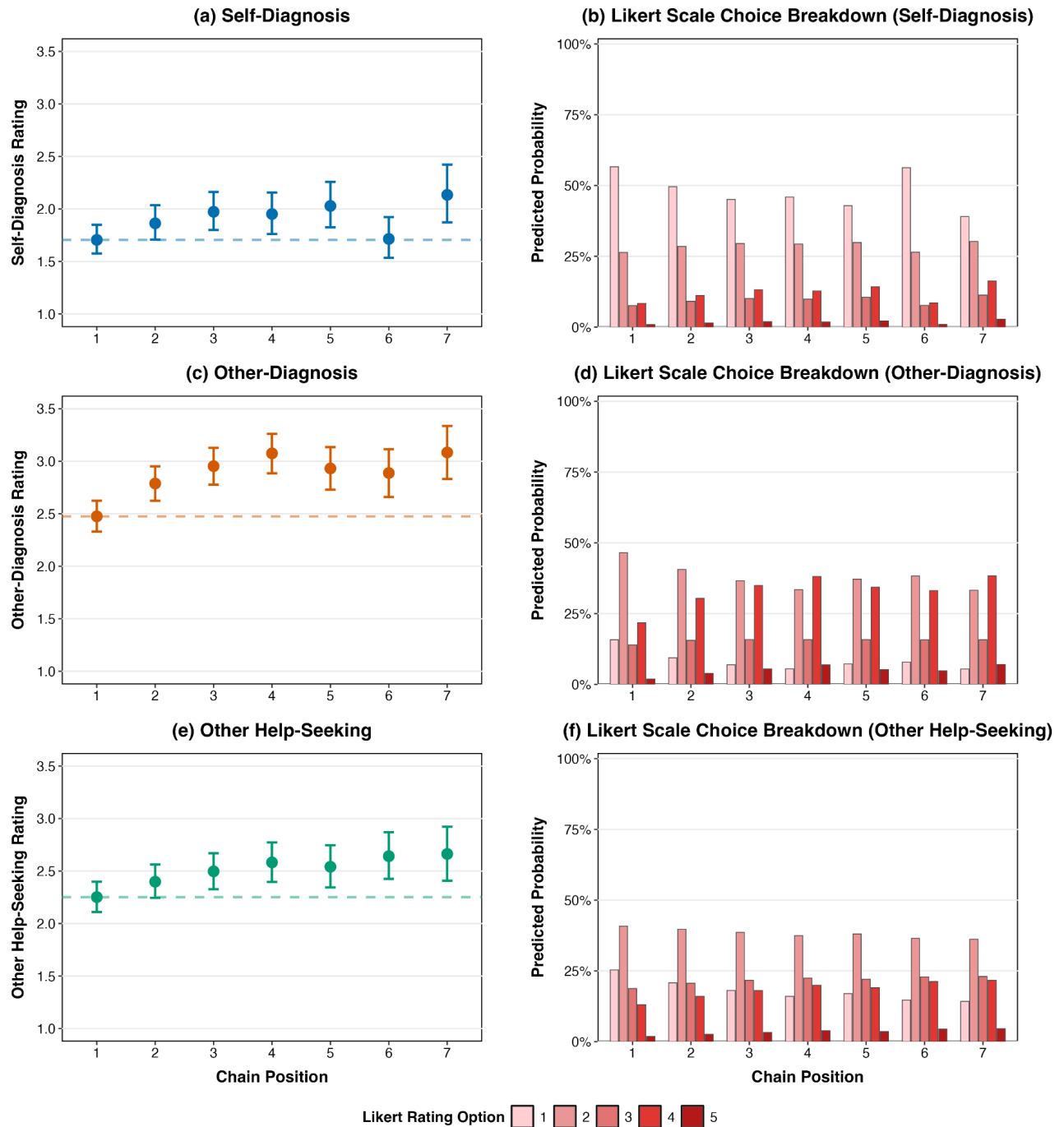


Supplementary Figure 24: *Study 3 Model Predicted Effects of Peer-to-Peer Transmission by Transmission Motivation. All panels show the predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree) for a particular outcome, either as bar plots (Categorical Probit models) or continuous lines for choosing each option over the course of transmission (Monotonic Trend Models).*

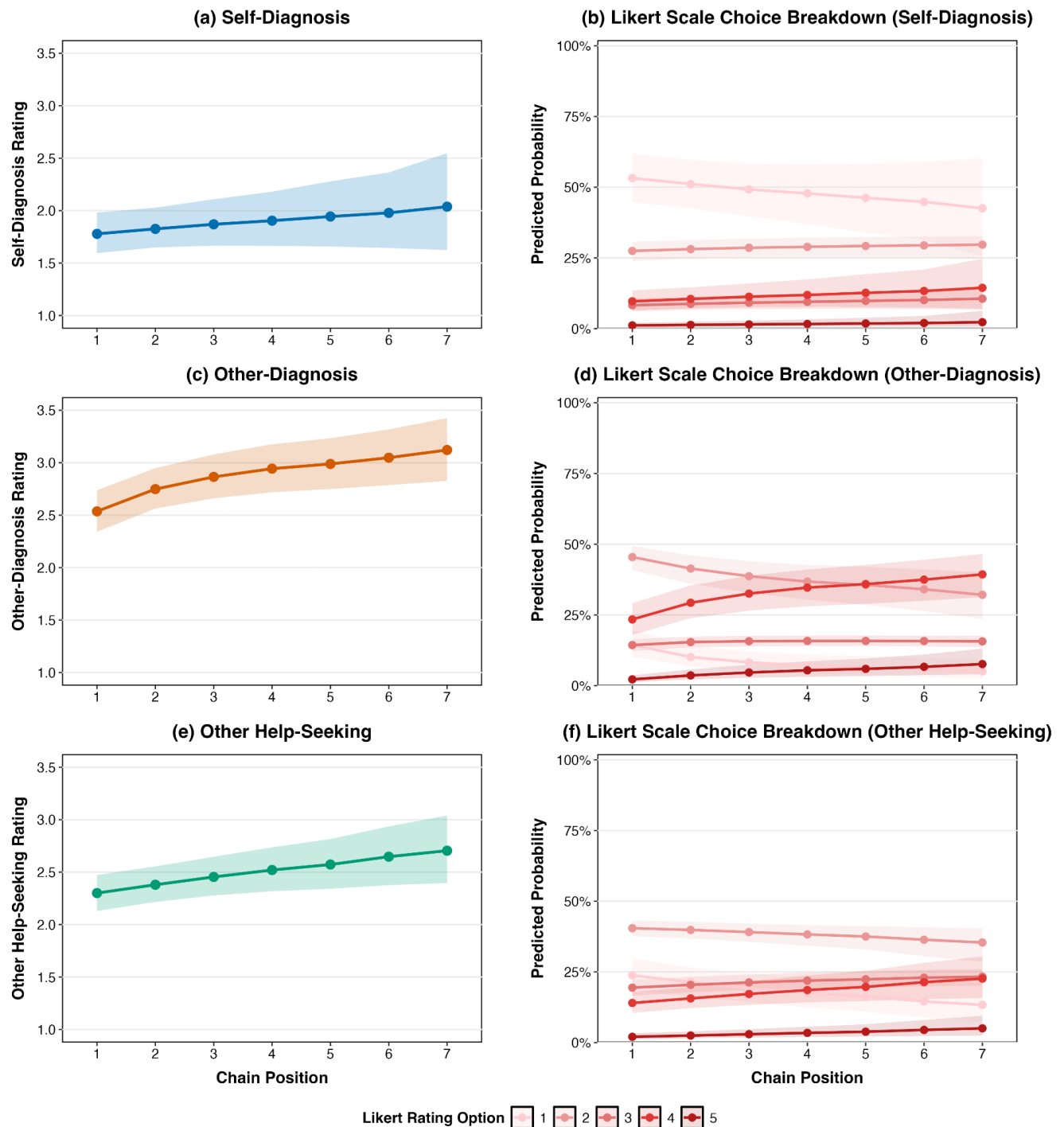
6.3 Sensitivity Analyses

This section presents Study 3 results in the study sample where participants either did not use AI or weren't exposed to an AI-generated explanation (see Methods section on how this was accounted for via interaction models). These results were obtained after adjusting for age, clinical history, ADHD prior knowledge and gender.

The core results of Study 3 remained largely unchanged across these additional model specifications, with the exception of the virality moderation effect becoming more credible.



Supplementary Figure 25: Study 3 Model Predicted Effects of Peer-to-Peer Transmission. Predictions are shown for chains not exposed to AI where age, clinical history and prior ADHD knowledge are held at their sample means and gender is at its reference category (female). Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.



Supplementary Figure 26: Study 3 Model Predicted Effects of Peer-to-Peer Transmission. Predictions are shown for chains not exposed to AI where age, clinical history and prior ADHD knowledge are held at their sample means and gender is at its reference category (female). Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position as a monotonic variable) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale

option (1 = Strongly Disagree to 5 = Strongly Agree). Shaded ribbons represent Bayesian 95% CrI that incorporate uncertainty around both population-level estimates and variation between transmission chains (varying or random effects).. NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.

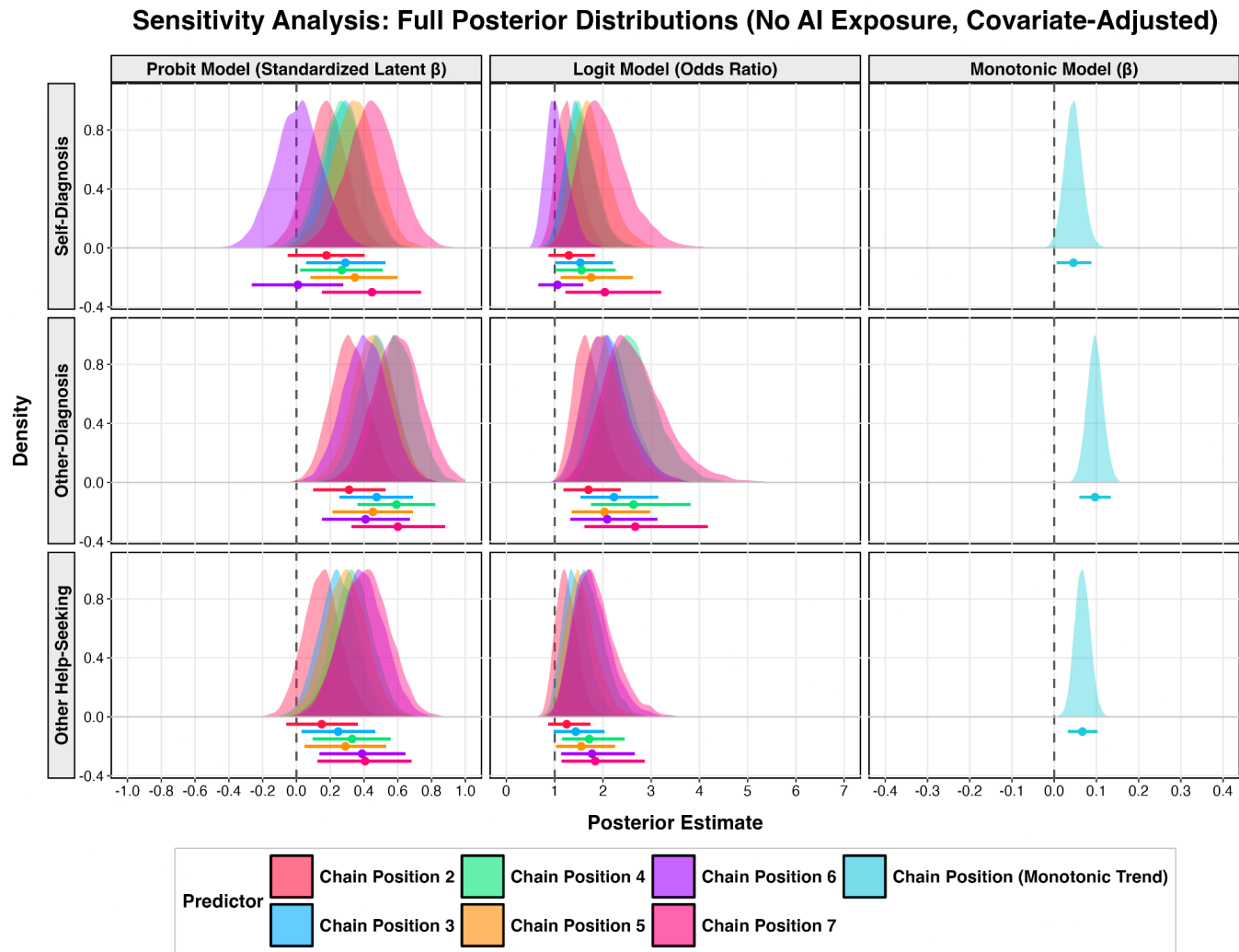
Predictor	Mean [95% Credible Interval] (Pr(β > 0))								
	Self-Diagnosis	Self-Diagnosis (Odds Ratio)	Self-Diagnosis (Monotonic)	Other-Diagnosis	Other-Diagnosis (Odds Ratio)	Other-Diagnosis (Monotonic)	Other Help-Seeking	Other Help-Seeking (Odds Ratio)	Other Help-Seeking (Monotonic)
Intercept 1	0.17 [0.00, 0.34]		0.08 [-0.06, 0.23]	-1.00 [-1.17, -0.84]		-1.06 [-1.23, -0.90]	-0.66 [-0.82, -0.51]		-0.71 [-0.84, -0.57]
Intercept 2	0.96 [0.79, 1.12]		0.87 [0.73, 1.03]	0.31 [0.16, 0.47]		0.25 [0.10, 0.41]	0.42 [0.27, 0.57]		0.37 [0.24, 0.51]
Intercept 3	1.32 [1.15, 1.50]		1.24 [1.09, 1.40]	0.72 [0.56, 0.87]		0.65 [0.50, 0.82]	1.04 [0.88, 1.20]		0.99 [0.85, 1.14]
Intercept 4	2.36 [2.15, 2.57]		2.27 [2.09, 2.47]	2.07 [1.89, 2.26]		2.01 [1.83, 2.20]	2.09 [1.90, 2.28]		2.04 [1.87, 2.22]
Chain Position (Monotonic Trend, No AI Exposure)			0.05 [0.01, 0.09] (0.99)			0.10 [0.06, 0.13] (1.00)			0.07 [0.03, 0.10] (1.00)
Chain Position 2 (No AI Exposure)	0.18 [-0.05, 0.40] (0.94)	1.29 [0.87, 1.84] (0.90)		0.31 [0.10, 0.53] (1.00)	1.70 [1.18, 2.37] (1.00)		0.15 [-0.06, 0.36] (0.92)	1.25 [0.86, 1.74] (0.87)	
Chain Position 3 (No AI Exposure)	0.29 [0.06, 0.53] (0.99)	1.53 [1.01, 2.21] (0.98)		0.47 [0.25, 0.69] (1.00)	2.23 [1.53, 3.15] (1.00)		0.25 [0.03, 0.47] (0.99)	1.44 [0.98, 2.03] (0.97)	
Chain Position 4 (No AI Exposure)	0.27 [0.02, 0.51] (0.98)	1.56 [1.03, 2.26] (0.98)		0.59 [0.36, 0.82] (1.00)	2.63 [1.75, 3.82] (1.00)		0.33 [0.10, 0.56] (1.00)	1.72 [1.16, 2.45] (1.00)	
Chain Position 5 (No AI Exposure)	0.35 [0.08, 0.60] (1.00)	1.76 [1.12, 2.62] (0.99)		0.45 [0.21, 0.69] (1.00)	2.03 [1.35, 2.98] (1.00)		0.29 [0.05, 0.53] (0.99)	1.55 [1.03, 2.25] (0.98)	
Chain Position 6 (No AI Exposure)	0.01 [-0.26, 0.28] (0.53)	1.06 [0.66, 1.60] (0.56)		0.41 [0.15, 0.67] (1.00)	2.09 [1.32, 3.13] (1.00)		0.39 [0.14, 0.65] (1.00)	1.77 [1.14, 2.66] (0.99)	
Chain Position 7 (No AI Exposure)	0.45 [0.15, 0.74] (1.00)	2.04 [1.22, 3.21] (1.00)		0.60 [0.33, 0.88] (1.00)	2.67 [1.61, 4.18] (1.00)		0.41 [0.12, 0.68] (1.00)	1.84 [1.14, 2.87] (0.99)	
Chain Position (Monotonic Trend) × Post-AI Exposure			-0.06 [-0.14, 0.01] (0.06)			-0.03 [-0.09, 0.03] (0.15)			-0.02 [-0.08, 0.04] (0.30)

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Chain Position 2 × Post-AI Exposure	-0.25 [-0.82, 0.33] (0.20)	0.94 [0.36, 1.98] (0.37)		0.05 [-0.50, 0.62] (0.57)	1.30 [0.52, 2.71] (0.65)		-0.14 [-0.69, 0.40] (0.30)	1.03 [0.42, 2.19] (0.44)	
Chain Position 3 × Post-AI Exposure	-0.31 [-0.85, 0.23] (0.13)	0.84 [0.35, 1.71] (0.26)		-0.14 [-0.66, 0.37] (0.30)	0.95 [0.41, 1.85] (0.37)		-0.16 [-0.68, 0.36] (0.26)	1.01 [0.45, 1.99] (0.43)	
Chain Position 4 × Post-AI Exposure	-0.46 [-0.98, 0.07] (0.04)	0.65 [0.28, 1.30] (0.09)		-0.11 [-0.61, 0.40] (0.33)	0.95 [0.43, 1.84] (0.38)		-0.11 [-0.62, 0.40] (0.33)	1.05 [0.47, 2.05] (0.48)	
Chain Position 5 × Post-AI Exposure	-0.37 [-0.89, 0.16] (0.08)	0.75 [0.33, 1.46] (0.17)		-0.06 [-0.56, 0.44] (0.41)	1.20 [0.55, 2.32] (0.62)		-0.17 [-0.69, 0.33] (0.25)	0.98 [0.45, 1.88] (0.40)	
Chain Position 6 × Post-AI Exposure	-0.09 [-0.62, 0.43] (0.36)	1.12 [0.51, 2.19] (0.55)		0.15 [-0.36, 0.67] (0.71)	1.47 [0.66, 2.87] (0.80)		-0.02 [-0.53, 0.49] (0.46)	1.24 [0.57, 2.37] (0.65)	
Chain Position 7 × Post-AI Exposure	-0.51 [-1.04, 0.02] (0.03)	0.59 [0.26, 1.15] (0.06)		-0.14 [-0.65, 0.37] (0.30)	1.00 [0.45, 1.95] (0.42)		-0.15 [-0.67, 0.35] (0.28)	1.03 [0.46, 2.01] (0.46)	
Post-AI Exposure (at Chain Position 1)	0.32 [-0.13, 0.75] (0.92)	1.39 [0.73, 2.42] (0.82)	0.22 [-0.08, 0.60] (0.91)	0.12 [-0.31, 0.54] (0.71)	1.17 [0.62, 2.01] (0.64)	0.17 [-0.08, 0.45] (0.91)	0.09 [-0.34, 0.53] (0.66)	1.05 [0.56, 1.80] (0.50)	0.04 [-0.22, 0.32] (0.60)
Age (z)	-0.30 [-0.37, -0.24] (0.00)	0.60 [0.54, 0.67] (0.00)	-0.30 [-0.36, -0.24] (0.00)	-0.01 [-0.07, 0.04] (0.36)	0.99 [0.90, 1.08] (0.37)	-0.00 [-0.06, 0.05] (0.47)	-0.08 [-0.14, -0.03] (0.00)	0.87 [0.79, 0.96] (0.00)	-0.08 [-0.14, -0.03] (0.00)
Clinical History (z)	0.14 [0.08, 0.19] (1.00)	1.27 [1.15, 1.40] (1.00)	0.14 [0.09, 0.20] (1.00)	-0.05 [-0.11, -0.00] (0.02)	0.91 [0.83, 0.99] (0.02)	-0.05 [-0.11, -0.00] (0.02)	-0.04 [-0.09, 0.02] (0.08)	0.95 [0.87, 1.04] (0.13)	-0.04 [-0.09, 0.01] (0.07)
Gender: Male	0.04 [-0.07, 0.16] (0.76)	1.11 [0.91, 1.35] (0.85)	0.05 [-0.07, 0.16] (0.78)	0.03 [-0.08, 0.14] (0.71)	1.07 [0.88, 1.28] (0.73)	0.03 [-0.08, 0.14] (0.71)	-0.05 [-0.16, 0.06] (0.16)	0.92 [0.76, 1.11] (0.18)	-0.05 [-0.16, 0.06] (0.17)
Gender: NonMbinaryDthir dgender	0.27 [-0.41, 0.95] (0.78)	1.21 [0.34, 3.07] (0.52)	0.29 [-0.39, 0.97] (0.80)	0.06 [-0.61, 0.71] (0.57)	1.35 [0.45, 3.20] (0.64)	0.08 [-0.59, 0.75] (0.59)	0.26 [-0.37, 0.91] (0.78)	1.46 [0.47, 3.55] (0.68)	0.26 [-0.40, 0.91] (0.78)
Gender: Prefernottosay	0.12 [-0.92, 1.14] (0.59)	1.56 [0.27, 4.88] (0.60)	0.01 [-1.03, 1.04] (0.51)	0.25 [-0.80, 1.30] (0.68)	2.04 [0.27, 7.41] (0.67)	0.19 [-0.87, 1.25] (0.64)	-0.31 [-1.32, 0.71] (0.28)	1.01 [0.18, 3.15] (0.37)	-0.34 [-1.35, 0.67] (0.26)
Prior Knowledge (z)	0.24 [0.18, 0.29] (1.00)	1.53 [1.39, 1.68] (1.00)	0.24 [0.18, 0.29] (1.00)	0.07 [0.01, 0.12] (0.99)	1.12 [1.02, 1.24] (0.99)	0.07 [0.02, 0.12] (0.99)	0.10 [0.05, 0.15] (1.00)	1.20 [1.10, 1.31] (1.00)	0.11 [0.05, 0.16] (1.00)
Simplex (× Post-AI): Step 1 → 2	0.21 [0.01, 0.58]			0.18 [0.01, 0.52]			0.19 [0.01, 0.56]		

Simplex (× Post-AI): Step 2 → 3	0.18 [0.01, 0.53]		0.17 [0.01, 0.51]		0.17 [0.00, 0.52]
Simplex (× Post-AI): Step 3 → 4	0.18 [0.01, 0.53]		0.15 [0.00, 0.49]		0.16 [0.00, 0.50]
Simplex (× Post-AI): Step 4 → 5	0.13 [0.00, 0.44]		0.16 [0.01, 0.49]		0.16 [0.00, 0.50]
Simplex (× Post-AI): Step 5 → 6	0.15 [0.00, 0.46]		0.15 [0.00, 0.47]		0.15 [0.00, 0.48]
Simplex (× Post-AI): Step 6 → 7	0.15 [0.00, 0.46]		0.19 [0.01, 0.55]		0.18 [0.01, 0.52]
Simplex: Step 1 → 2	0.19 [0.03, 0.44]		0.36 [0.12, 0.59]		0.20 [0.03, 0.43]
Simplex: Step 2 → 3	0.18 [0.03, 0.42]		0.20 [0.04, 0.43]		0.19 [0.03, 0.42]
Simplex: Step 3 → 4	0.14 [0.02, 0.35]		0.13 [0.02, 0.31]		0.16 [0.03, 0.38]
Simplex: Step 4 → 5	0.15 [0.02, 0.37]		0.08 [0.01, 0.20]		0.13 [0.02, 0.33]
Simplex: Step 5 → 6	0.13 [0.02, 0.33]		0.10 [0.02, 0.25]		0.18 [0.03, 0.41]
Simplex: Step 6 → 7	0.21 [0.03, 0.47]		0.12 [0.02, 0.28]		0.14 [0.02, 0.32]
SD: Between-Chain Baseline Variation	0.07 [0.00, 0.19]	0.07 [0.00, 0.18]	0.06 [0.00, 0.15]	0.05 [0.00, 0.14]	0.05 [0.00, 0.14]
SD: Between-Chain Effect Variation		0.03 [0.00, 0.07]		0.01 [0.00, 0.04]	0.02 [0.00, 0.04]

***Supplementary Table 27:** Adjusted Bayesian Multilevel Ordinal Regression Results of Study 3 by Outcome. The reference category was female for the Gender predictor. No AI exposure refers to regression estimates obtained from a sample that either didn’t use AI directly or weren’t exposed to any AI generated explanations. These are the main estimands of interest for the sensitivity analyses. (z) indicates that variables were standardised. $\Pr(\beta > 0)$ denotes probability of a positive association and is only provided for the main predictors. Likert scale cutpoint and varying intercept terms (i.e. SD) for the logit models aren’t presented here owing to difference in scale units of such models (i.e. log-odds).*

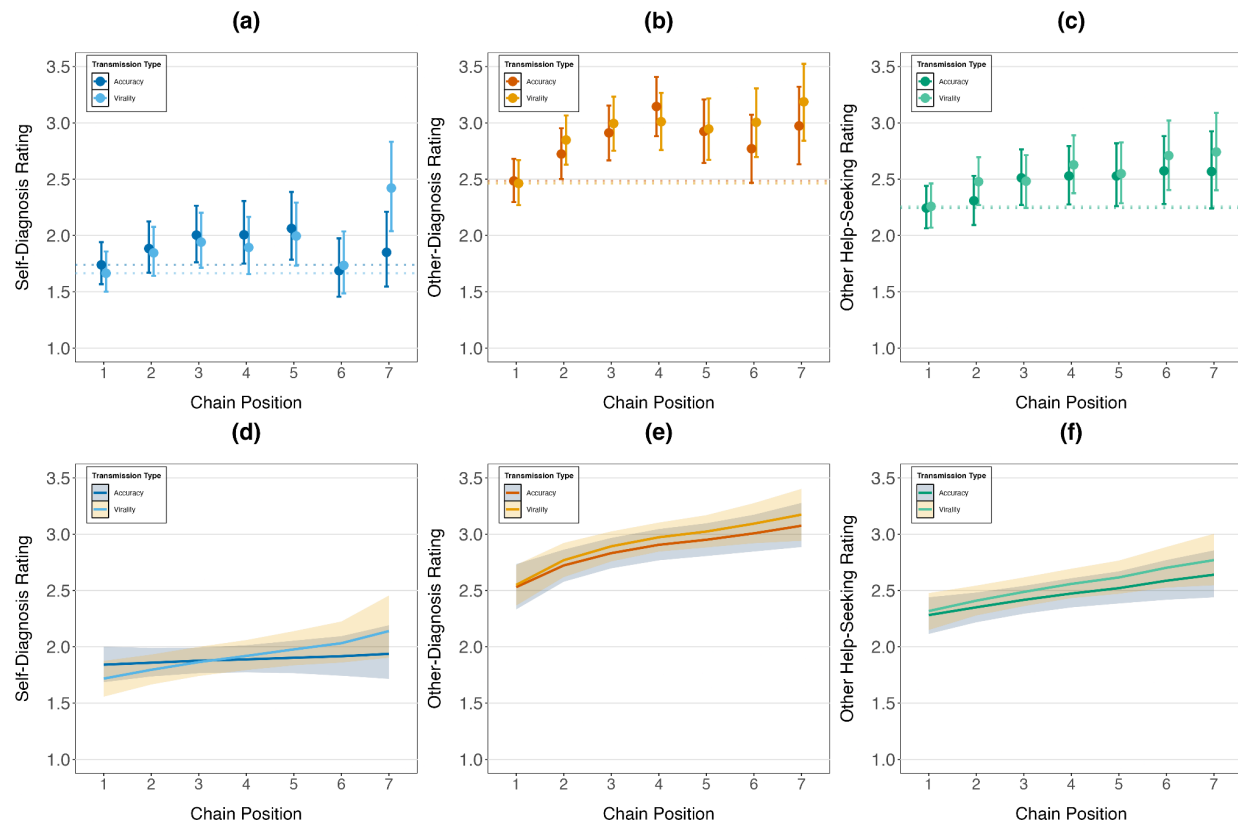


Supplementary Figure 27: Full Posterior Distributions of Study 3 Effects of Transmission on Self-Diagnosis, Other-Diagnosis and Other Help-Seeking across probit, logit and monotonic model variants. Colored horizontal lines on the x -axis denote 95% credible intervals, with the central dot representing the mean.

6.3.1 Transmission Motivation Moderation Results

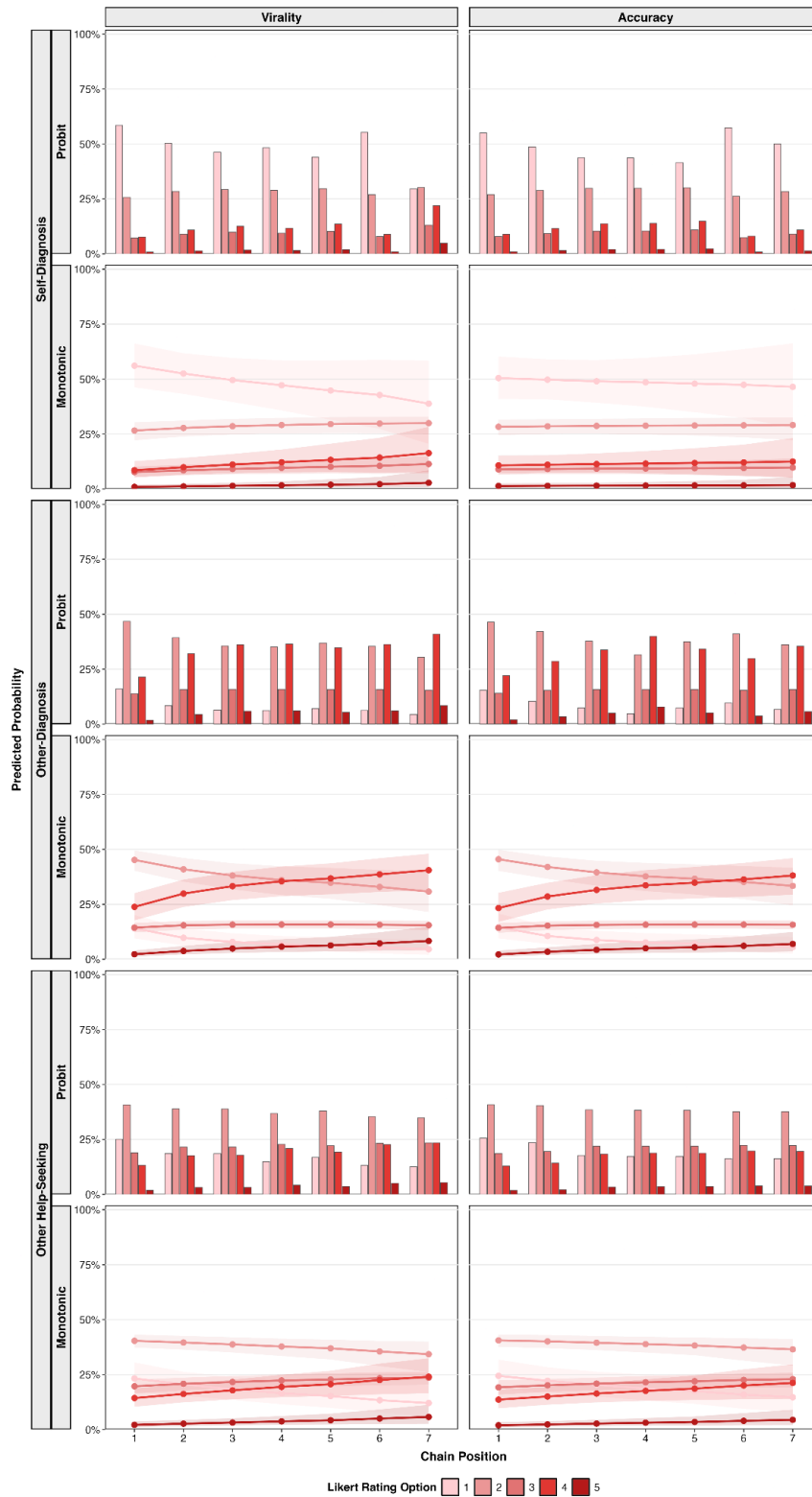
The following key moderation results became more certain during sensitivity analyses:

1. Relative to initial participants exposed to the ADHD DSM-5-TR criteria, virality-motivated chains did show stronger increases in self-diagnosis than accuracy-motivated chains after seven rounds of transmission (see **SFig 28a**; *Self-diagnosis interaction* - CP7: 0.63 latent SD, 95% CrI [0.08, 1.18], $P_{>0} = 99\%$).
2. There was moderate evidence for this moderation effect when investigating trends over the course of transmission (See **SFig 28d**; *Self-diagnosis monotonic interaction* - 0.06 latent SD, 95% CrI [-0.02, 0.14] $P_{>0} = 93\%$).



Supplementary Figure 28: Study 3 Interaction Model Predicted Effects. Predictions are shown for chains not exposed to AI where age, clinical history and prior ADHD knowledge are held at their sample means and gender is at its reference category (female). Panels (a) - (c) show predicted interaction effects of transmission motivation and peer-to-peer transmission (i.e. chain position) on Self-Diagnosis (a), Other-Diagnosis (b) and Other Help-Seeking (c) when chain position is modelled as a factor/categorical predictor. Panels (d) - (f) display the same effects when chain position is modelled as a monotonic predictor to

test the interaction effect over the course of transmission. All dependent variables here were Likert scale items ranging from 1 = Strongly Disagree to 5 = Strongly Agree. NB: Y axes for Panels (a) - (f) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.



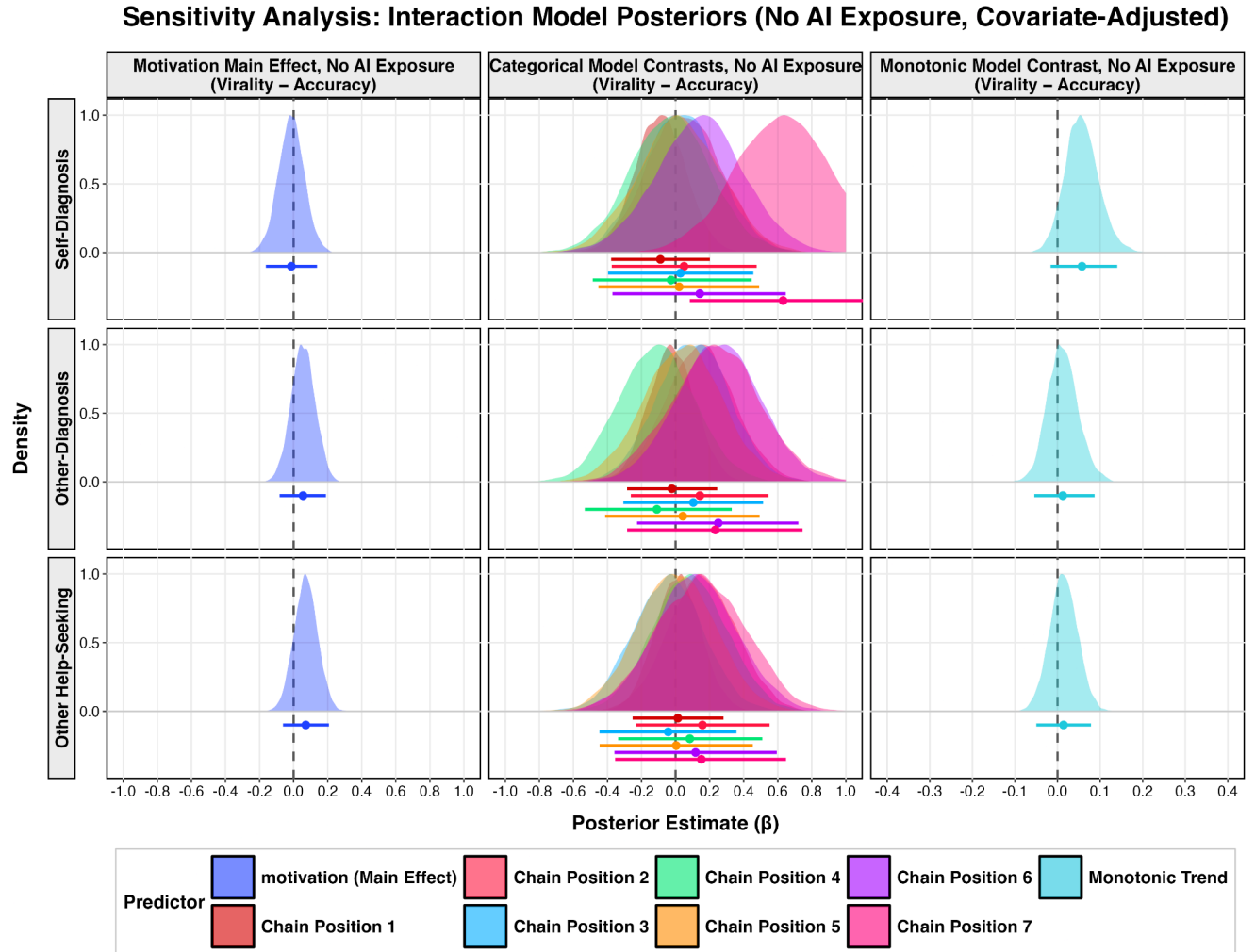
Supplementary Figure 29: Study 3 Model Predicted Effects of Peer-to-Peer Transmission by Transmission Motivation in the female sample (reference gender category) with no AI exposure and mean levels of age, clinical history and ADHD prior knowledge. All panels show the predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree)

Agree) for a particular outcome, either as bar plots (Categorical Probit models) or continuous lines for choosing each option over the course of transmission (Monotonic Trend Models).

Predictor	Mean [95% Credible Interval] (Pr($\beta > 0$))					
	Self-Diagnosis	Self-Diagnosis (Monotonic)	Other-Diagnosis	Other-Diagnosis (Monotonic)	Other Help-Seeking	Other Help-Seeking (Monotonic)
Transmission Type (Main Effect: Virality - Accuracy, No AI Exposure)	-0.01 [-0.16, 0.14] (0.43)	-0.00 [-0.15, 0.15] (0.48)	0.06 [-0.08, 0.19] (0.79)	0.06 [-0.08, 0.20] (0.79)	0.07 [-0.06, 0.21] (0.85)	0.07 [-0.06, 0.21] (0.85)
Chain Position 2 × motivation	0.05 [-0.38, 0.48] (0.58)		0.14 [-0.26, 0.55] (0.76)		0.16 [-0.23, 0.55] (0.79)	
Chain Position 3 × motivation	0.03 [-0.40, 0.46] (0.55)		0.10 [-0.31, 0.51] (0.69)		-0.04 [-0.45, 0.36] (0.42)	
Chain Position 4 × motivation	-0.03 [-0.49, 0.45] (0.46)		-0.11 [-0.53, 0.33] (0.30)		0.08 [-0.34, 0.51] (0.65)	
Chain Position 5 × motivation	0.02 [-0.45, 0.49] (0.53)		0.04 [-0.41, 0.49] (0.57)		0.00 [-0.45, 0.45] (0.50)	
Chain Position 6 × motivation	0.14 [-0.37, 0.65] (0.71)		0.25 [-0.23, 0.72] (0.85)		0.12 [-0.36, 0.59] (0.69)	
Chain Position 7 × motivation	0.63 [0.08, 1.18] (0.99)		0.23 [-0.28, 0.75] (0.81)		0.15 [-0.35, 0.65] (0.71)	
Chain Position 1 × motivation	-0.09 [-0.38, 0.20] (0.27)		-0.02 [-0.29, 0.24] (0.43)		0.01 [-0.25, 0.28] (0.54)	
Chain Position 2 × motivation × Post-AI Exposure	-0.18 [-1.08, 0.73] (0.35)		0.08 [-0.79, 0.94] (0.57)		0.11 [-0.77, 0.97] (0.59)	
Chain Position 3 × motivation × Post-AI Exposure	-0.12 [-0.94, 0.69] (0.39)		-0.16 [-0.92, 0.62] (0.35)		-0.11 [-0.92, 0.68] (0.40)	
Chain Position 4 × motivation × Post-AI Exposure	0.50 [-0.31, 1.28] (0.90)		0.06 [-0.69, 0.82] (0.56)		0.09 [-0.67, 0.84] (0.59)	

Chain Position 5 × motivation × Post-AI Exposure	0.10 [-0.69, 0.88] (0.59)		-0.11 [-0.87, 0.65] (0.38)		-0.06 [-0.81, 0.69] (0.44)	
Chain Position 6 × motivation × Post-AI Exposure	0.08 [-0.72, 0.85] (0.58)		0.11 [-0.65, 0.86] (0.61)		0.03 [-0.73, 0.77] (0.53)	
Chain Position 7 × motivation × Post-AI Exposure	-0.68 [-1.49, 0.11] (0.05)		-0.19 [-0.95, 0.59] (0.31)		0.05 [-0.73, 0.81] (0.55)	
Chain Position 1 × motivation × Post-AI Exposure	0.01 [-0.61, 0.62] (0.52)		-0.11 [-0.69, 0.47] (0.36)		-0.02 [-0.62, 0.59] (0.47)	
Chain Position (Monotonic Trend) × motivation		0.06 [-0.02, 0.14] (0.93)		0.01 [-0.05, 0.09] (0.63)		0.01 [-0.05, 0.08] (0.67)
Chain Position (Monotonic Trend) × motivation × Post-AI Exposure		-0.05 [-0.18, 0.07] (0.20)		0.01 [-0.11, 0.13] (0.56)		0.01 [-0.10, 0.12] (0.60)
SD: Between-Chain Baseline Variation	0.08 [0.00, 0.19]	0.07 [0.00, 0.18]	0.05 [0.00, 0.15]	0.05 [0.00, 0.14]	0.05 [0.00, 0.14]	0.05 [0.00, 0.13]
SD: Between-Chain Effect Variation		0.03 [0.00, 0.07]		0.01 [0.00, 0.04]		0.02 [0.00, 0.04]

Supplementary Table 28: Adjusted Bayesian Multilevel Ordinal Moderation Regression Results of Study 3 by Outcome. Chain position was treated as a categorical and monotonic variable respectively. For categorical models, Chain Position $N \times$ Motivation represents the difference-in-differences between accuracy and virality motivations at a specific chain position, relative to chain position 1 in the study sample where there was no AI usage or exposure (Post-AI Exposure = 0). For monotonic models, Chain Position (Monotonic Trend) $\times$ Transmission Motivation represents the difference in trend increases in the pertinent outcome between virality and accuracy motivations. The motivation main effect was calculated from the interaction model using the `marginaleffects` package. NB: Table display is limited to selected estimands of interest to prevent overcrowding of core results. For further information on other estimands (e.g. simplex terms), please refer to the R scripts.



Supplementary Figure 30: Full Posterior Distributions of Motivation Moderation Effects in Study 3 across probit and monotonic model variants. For probit categorical models, each Chain Position Posterior represents the difference-in-differences in the relevant outcome between accuracy and virality motivations, relative to Chain Position 1 when there was no AI usage or exposure ($\text{Post-AI Exposure} = 0$). Positive values on the x-axis mean that virality has a stronger impact. For monotonic models, the posterior represents the difference-in-differences in trend increases in the pertinent outcome between accuracy and virality motivations. Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean. For some probit and logit models, some posterior distributions aren't fully displayed due to limiting the x-axis scale. This limit was imposed to facilitate comparison of effects across equivalent model classes run across different studies.

7. Study 4 Results

7.1 Descriptive Statistics

NB: This study was run on a sample who subjectively rated themselves to be in good health.

Chain position	Observations	Age (Mean)	Female (%)	Clinical History (%)				ADHD Prior Knowledge (%)		
				MH Medication	MH Treatment	NHS MH Support	Past MH Diagnosis	Friend Support	Waitlist Support	Social Media
1	106	44.58	71.70	0	0	2.83	5.66	17.92	5.66	16.98
7	109	43.88	72.48	0	0	2.75	3.67	18.35	11.01	15.60

Supplementary Table 29: Sample characteristics, Clinical history and ADHD Prior Knowledge by Chain Position (Study 4). Percentages reflect % of participants answering “Yes” to a question.

AI Exposure Classification	AI Usage	N	% of Total Sample
Pure non-AI observations	No	172	80
Participants who used AI to generate their explanation	Yes	43	20

Supplementary Table 30: % of AI-usage in Study 4

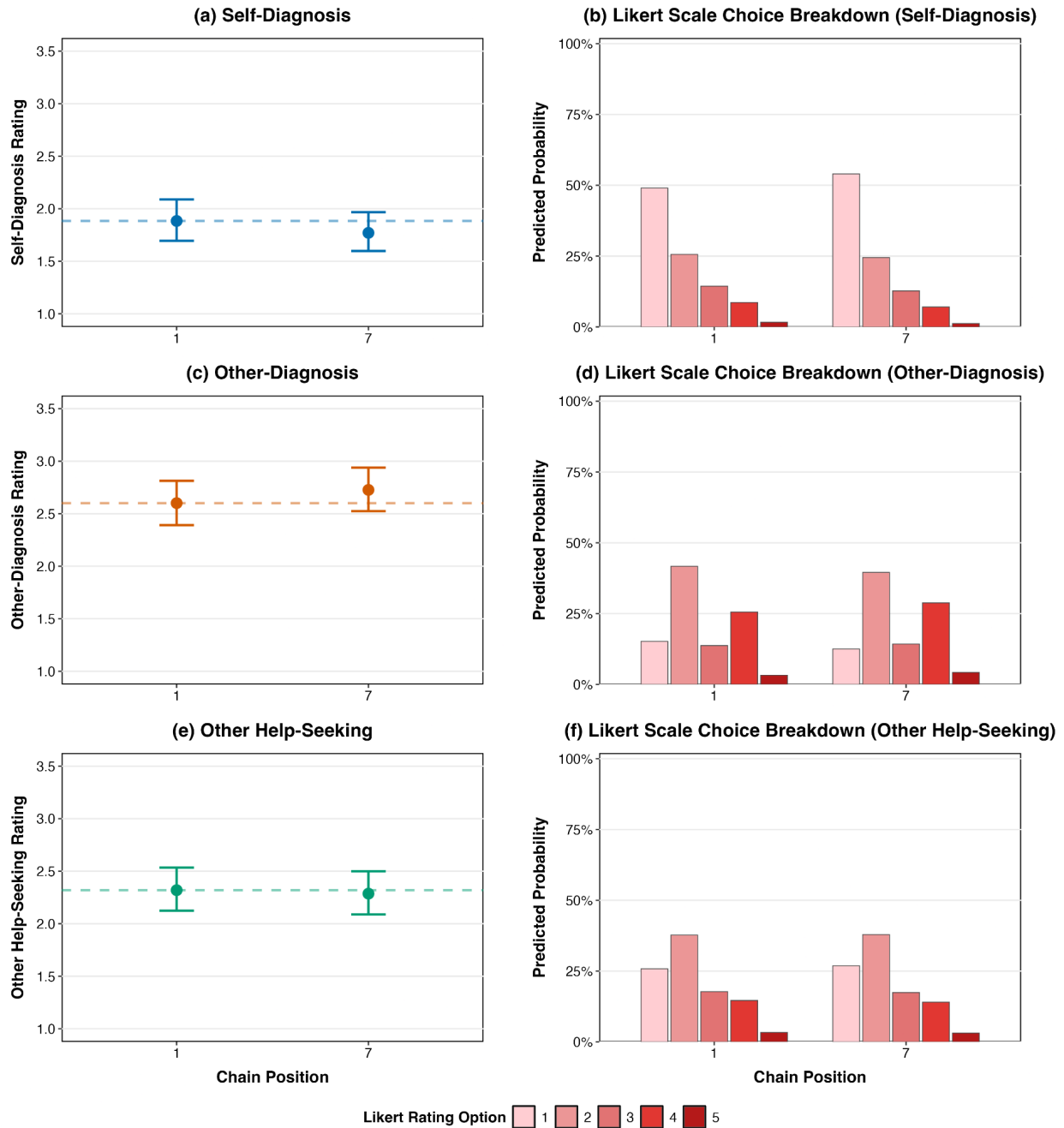
20% of participants in Study 4 were detected using AI to generate their written ADHD explanations. This is higher than Study 2 and Study 3, highlighting that AI usage is a problem in offline University panels as well.

7.2 Main Analyses

Relative to participants at Chain Position 1 exposed to the ADHD DSM-5-TR criteria, we found uncertain, but suggestive evidence that participants at Chain Position 7 had 16% lower odds of diagnosing themselves with ADHD (see **Supplementary Table 31, SFig 31 and 32 below; *Self-diagnosis* - CP7: 0.84 OR, 95% CrI [0.50, 1.31], $P_{>0} = 21\%$; -0.12 latent SD, 95% CrI [-0.42, 0.17], $P_{>0} = 20\%$) . For other-diagnosis, we found uncertain, but suggestive evidence that participants at Chain Position 7 had 27% higher odds of diagnosing another subclinical individual with ADHD (see **Supplementary Table 31, SFig 31 and 32 below; *Other-diagnosis* - CP7: 1.27 OR, 95% CrI [0.77, 1.99], $P_{>0} = 80\%$; 0.12 latent SD, 95% CrI [-0.16, 0.40], $P_{>0} = 81\%$).****

There was no difference in other help-seeking tendencies between participants at Chain Position 1 and 7 (see **Supplementary Table 31, SFig 31 and 32 below; *Other Help-Seeking* - CP7: 0.99 OR, 95% CrI [0.60, 1.54], $P_{>0} = 44\%$).**

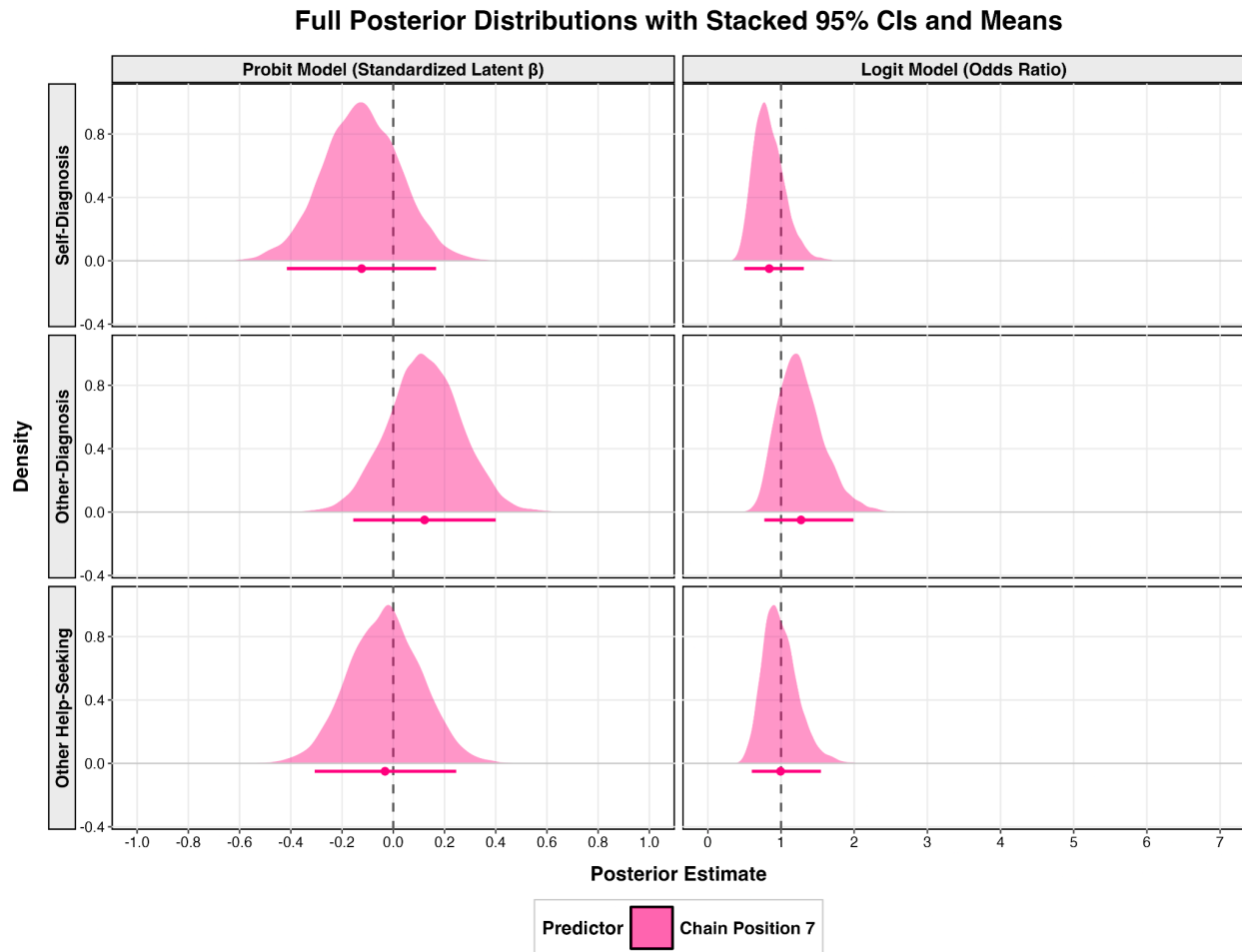
NB: No monotonic model variants were specified here owing to Study 4's simple two-group design mimicking the beginning and end of transmission from Study 2.



Supplementary Figure 31: Study 4 Model Predicted Effects of Differences between the beginning and end of transmission. Panels (a), (c), and (e) show predicted effects of transmission (i.e. chain position) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.

Mean [95% Credible Interval] (Pr($\beta > 0$))						
Predictor	Self-Diagnosis	Self-Diagnosis (Odds Ratio)	Other-Diagnosis	Other-Diagnosis (Odds Ratio)	Other Help-Seeking	Other Help-Seeking (Odds Ratio)
Intercept 1	-0.02 [-0.24, 0.20]		-1.03 [-1.28, -0.78]		-0.65 [-0.87, -0.43]	
Intercept 2	0.67 [0.45, 0.91]		0.18 [-0.04, 0.40]		0.35 [0.13, 0.57]	
Intercept 3	1.26 [0.99, 1.53]		0.56 [0.33, 0.79]		0.91 [0.67, 1.15]	
Intercept 4	2.14 [1.73, 2.62]		1.85 [1.53, 2.20]		1.83 [1.50, 2.20]	
Chain Position 7	-0.12 [-0.42, 0.17] (0.20)	0.84 [0.50, 1.31] (0.21)	0.12 [-0.16, 0.40] (0.81)	1.27 [0.77, 1.99] (0.80)	-0.03 [-0.31, 0.25] (0.41)	0.99 [0.60, 1.54] (0.44)

***Supplementary Table 31:** Bayesian Ordinal Regression Results of Study 4 by Outcome. Chain position was treated as a categorical variable. Pr($\beta > 0$) denotes probability of a positive association and is only provided for the main predictors.*



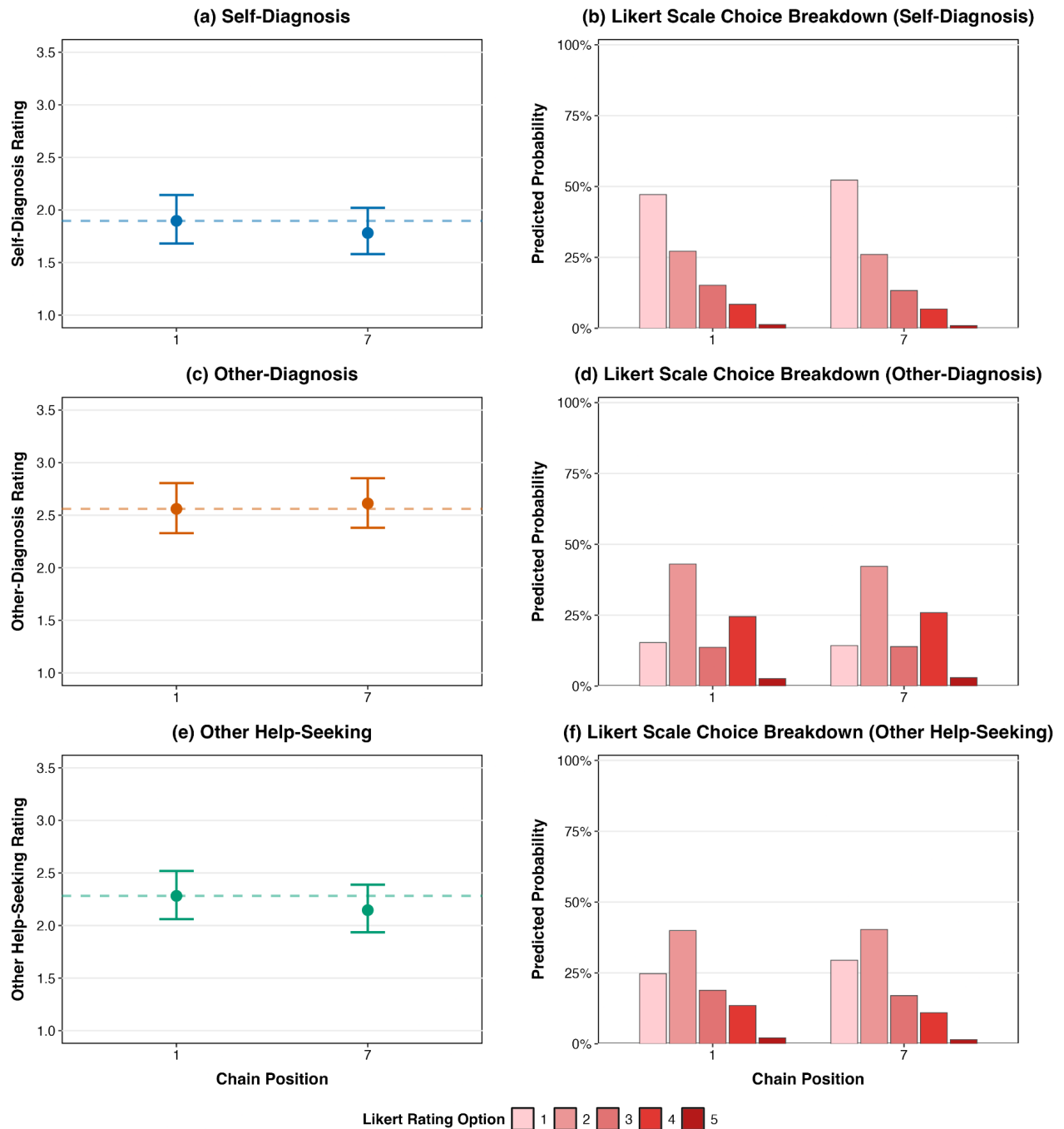
***Supplementary Figure 32:** Full Posterior Distributions of Study 4 Effects of Transmission on Self-Diagnosis, Other-Diagnosis and Other Help-Seeking across probit and logit model variants. For probit and logit categorical models, the chain position term is a contrast from Chain Position 1. Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean.*

7.3 Sensitivity Analyses

This section presents Study 4 results in the study sample where participants either didn't use AI or weren't exposed to an AI-generated explanation (see Methods section on how this was accounted for via interaction models). These results were obtained after adjusting for age, clinical history, ADHD prior knowledge and gender.

The following result changed, largely driven by the AI-usage moderation effect:

1. Relative to participants at Chain Position 1 exposed to the ADHD DSM-5-TR criteria, participants at Chain Position 7 weren't more likely to diagnose another subclinical individual with ADHD (see **Supplementary Table 32**).



Supplementary Figure 33: Study 4 Model Predicted Effects of Differences between the beginning and end of transmission.

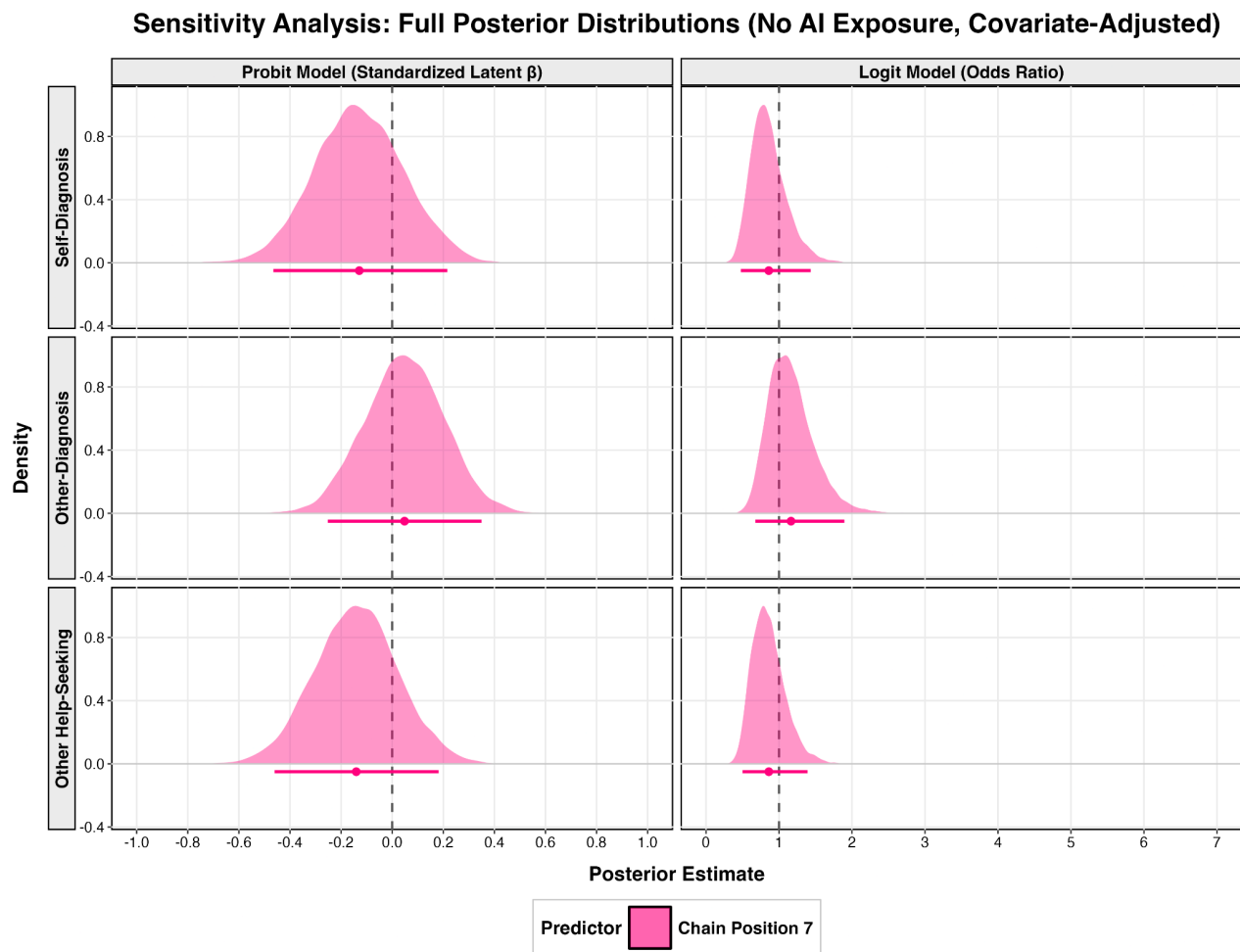
Predictions are shown for chains not exposed to AI where age, clinical history and prior ADHD knowledge are held at their sample means and gender is at its reference category (female). Panels (a), (c), and (e) show predicted effects of transmission (i.e.

chain position) on a continuous scale. Panels (b), (d) and (f) show these same effects as predicted probabilities of choosing a Likert scale option (1 = Strongly Disagree to 5 = Strongly Agree). NB: Y axes for Panels (a), (c) and (e) are limited to 3.5, rather than 5, to more clearly visualise uncertainty around plotted effects.

Predictor	Mean [95% Credible Interval] (Pr(β > 0))					
	Self-Diagnosis	Self-Diagnosis (Odds Ratio)	Other-Diagnosis	Other-Diagnosis (Odds Ratio)	Other Help-Seeking	Other Help-Seeking (Odds Ratio)
Intercept 1	-0.07 [-0.33, 0.19]		-1.02 [-1.30, -0.75]		-0.68 [-0.94, -0.42]	
Intercept 2	0.66 [0.39, 0.93]		0.22 [-0.03, 0.47]		0.39 [0.14, 0.64]	
Intercept 3	1.29 [0.99, 1.59]		0.60 [0.34, 0.86]		1.00 [0.73, 1.28]	
Intercept 4	2.23 [1.78, 2.74]		1.94 [1.57, 2.32]		2.04 [1.65, 2.46]	
Chain Position 7 (No AI Exposure)	-0.13 [-0.47, 0.22] (0.23)	0.86 [0.48, 1.43] (0.25)	0.05 [-0.25, 0.35] (0.62)	1.16 [0.68, 1.90] (0.67)	-0.14 [-0.46, 0.18] (0.19)	0.86 [0.50, 1.39] (0.24)
Age (z)	-0.24 [-0.41, -0.07] (0.00)	0.70 [0.52, 0.91] (0.00)	0.07 [-0.08, 0.23] (0.82)	1.12 [0.86, 1.44] (0.79)	-0.18 [-0.34, -0.02] (0.01)	0.77 [0.58, 0.99] (0.02)
Clinical History (z)	0.04 [-0.11, 0.19] (0.73)	1.08 [0.83, 1.38] (0.70)	-0.02 [-0.16, 0.12] (0.38)	1.00 [0.76, 1.27] (0.47)	-0.05 [-0.20, 0.09] (0.24)	0.93 [0.72, 1.18] (0.27)
Gender: Male	-0.15 [-0.51, 0.21] (0.20)	0.84 [0.43, 1.43] (0.23)	0.13 [-0.20, 0.46] (0.77)	1.26 [0.69, 2.14] (0.75)	-0.04 [-0.38, 0.30] (0.42)	0.89 [0.49, 1.49] (0.30)
Gender: Prefer not to say	0.41 [-1.00, 1.83] (0.72)	2.16 [0.26, 7.93] (0.68)	-0.86 [-2.46, 0.65] (0.14)	0.82 [0.08, 3.37] (0.25)	-0.83 [-2.43, 0.70] (0.15)	0.83 [0.09, 3.46] (0.25)
Prior Knowledge (z)	0.22 [0.06, 0.37] (1.00)	1.46 [1.11, 1.91] (1.00)	0.17 [0.02, 0.33] (0.99)	1.33 [1.01, 1.72] (0.98)	0.19 [0.04, 0.34] (0.99)	1.42 [1.09, 1.84] (0.99)
Chain Position 7 x AI Exposure Interaction	-0.04 [-0.75, 0.64] (0.45)	0.99 [0.29, 2.44] (0.39)	0.26 [-0.41, 0.92] (0.78)	1.66 [0.51, 4.08] (0.76)	0.27 [-0.40, 0.92] (0.79)	1.74 [0.57, 4.08] (0.80)

AI Exposure	-0.04 [-0.62, 0.53] (0.44)	1.03 [0.39, 2.24] (0.44)	0.04 [-0.53, 0.60] (0.56)	1.26 [0.46, 2.83] (0.59)	0.30 [-0.25, 0.87] (0.85)	2.01 [0.78, 4.29] (0.92)
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Supplementary Table 32: Adjusted Bayesian Ordinal Regression Results of Study 4 by Outcome. The reference category was female for the Gender predictor. No AI exposure refers to regression estimates obtained from a sample that either didn't use AI directly or weren't exposed to any AI generated explanations. These are the main estimands of interest for the sensitivity analyses. (z) indicates that variables were standardised. $\Pr(\beta > 0)$ denotes probability of a positive association and is only provided for the main predictors. Likert scale cutpoint for the logit models aren't presented here owing to difference in scale units of such models (i.e. log-odds).



Supplementary Figure 34: Full Posterior Distributions of Study 4 Effects of Transmission on Self-Diagnosis, Other-Diagnosis and Other Help-Seeking across probit and logit model variants. For probit and logit categorical models, the

chain position term is a contrast from Chain Position 1. Colored horizontal lines on the x-axis denote 95% credible intervals, with the central dot representing the mean.